A PHASE 3, RANDOMIZED, OPEN-LABEL, MULTICENTER STUDY OF VEPDEGESTRANT (ARV-471/PF-07850327) PLUS PALBOCICLIB VERSUS LETROZOLE PLUS PALBOCICLIB FOR THE TREATMENT OF PARTICIPANTS WITH ESTROGEN RECEPTOR-POSITIVE, HER2-NEGATIVE BREAST CANCER WHO HAVE NOT RECEIVED ANY PRIOR SYSTEMIC ANTI-CANCER TREATMENT FOR ADVANCED DISEASE (VERITAC-3)

Study Intervention Number:	PF-07850327
Study Intervention Name:	Vepdegestrant (ARV-471)
US IND Number:	140264
EU CT Number:	2022-500545-24-00
ClinicalTrials.gov ID:	NCT05909397
Pediatric Investigational Plan Number	Not Applicable
Protocol Number:	C4891002
Phase:	3
Sponsor Legal Address (Outside US):	Pfizer, Inc 66 Hudson Boulevard East New York, NY 10001
Sponsor Legal Address (US)	Arvinas Estrogen Receptor, Inc. 5 Science Park, 395 Winchester Ave. New Haven, CT 06511

Brief Title: Phase 3, Randomized, Open-Label Study of Vepdegestrant (ARV-471) Plus Palbociclib vs Letrozole Plus Palbociclib in Participants With ER(+)/HER2(-) Advanced Breast Cancer (VERITAC-3)

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Document History

Document	Version Date
Amendment 3	17 November 2025
Amendment 2	28 September 2023
Amendment 1	07 March 2023
Original protocol	22 June 2022

This amendment incorporates all revisions to date, including amendments made at the request of country health authorities and IRBs/ECs and any protocol administrative change letter(s).

Protocol Amendment Summary of Changes Table

Amendment 3 (17 November 2025)

Overall Rationale for the Amendment: The main purpose of the amendment is to reduce the Schedule of Activities with clinical assessments per standard of care or clinical judgment of the investigator. This amendment also reflects the previous decision that the Phase 3 portion of the study will not be conducted, due to a Sponsor decision and not for safety or efficacy reasons or requests from Regulatory Agencies; SLI Phase enrollment was completed for the planned participants. In addition, this amendment is to update the end of study definition and the section regarding continued access to study intervention after the end of study.

Description of Change	Brief Rationale	Section # and Name
Substantial Modification(s)		
Added rationale for Protocol Amendment 3	To describe rationale (indicated above) for this protocol amendment	Section 1.1 Synopsis, Section 4.2 Scientific Rationale for Study Design
Updated SoA and added a statement that it will be reduced after implementation of Amendment 3: concomitant medications collection in eCRF no longer required; PE / vital signs / blood samples collection/ ECG/disease assessments performed as per clinical standard of care at the discretion of the investigator	To reduce current requirements and allow several procedures to be performed as per local practice	Section 1.3 Schedule of Activities, Section 6.9 Prior and Concomitant Therapy, Section 6.9.10. Surgery and Radiotherapy, Section 8 Study Assessments and

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Description of Change	Brief Rationale	Section # and Name
		Procedures,Section 8.2 Efficacy Assessments, Section 8.3.1 Physical Examinations, Section 8.3.2 Vital Signs, Section 8.3.3 Electrocardiograms, Section 8.3.4 Clinical Safety Laboratory Assessments, Section 10.2 Clinical Laboratory Tests
Removal of Phase 3 portion of the study	This is a Sponsor decision and is not a result of a safety or efficacy concern or related to a regulatory agency request	Section 1.1 Synopsis, Section 1.2 Schema, Section 1.3 Schedule of Activities, Section 2.3.1 Risk Assessment Section 3 Objectives, Endpoints and Estimands, Section 4.1 Overall Design, Section 4.2 Scientific Rationale for Study Design, Section 4.3 Justification for Dose, Section 6 Study Intervention(s) and Concomitant Therapy, Section 7.1.1.3 Survival Follow-up, Section 8 Study Assessments and Procedures, Section 9 Statistical Considerations, Section 10.1.5.1 Data Monitoring Committee,

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Description of Change	Brief Rationale	Section # and Name
		Section 10.4.1 Male Participant Reproductive Inclusion Criteria Section 10.13.1 Patient-Reported Outcomes Assessments
Updated text to indicate that participants benefiting from the treatment (defined as complete/partial response or stable disease by RECIST v1.1) at the time of implementing Amendment 3 will continue treatment with study intervention per the SoA as part of the study until treatment discontinuation.	To fulfill the obligation to provide study treatment to participants currently on treatment still receiving a benefit from the study intervention	Section 1.1 Synopsis Section 4.1 Overall Design, Section 4.4 End of Study Definition Section 6.7 Continued Access to Study Intervention After the End of the Study
"Developmental and reproductive toxicities" has been added as an additional potential risk of clinical significance	Risk entered in alignment with 2023 IB and subsequent IB versions	Section 2.3.1 Risk Assessment
"QT Prolongation" updated to "QTc Prolongation", and Summary of Data/Rationale for Risk has been updated.	Summary of Data/Rationale for Risk have been updated in alignment with 2025 IB	Section 2.3.1 Risk Assessment
An option to continue treatment beyond progression has been removed	There are no current data to support continuing study treatment beyond progression.	Section 8.2.2 Treatment Continuation Beyond Disease Progression by Imaging
Non-Substantial Modification(s)		
Reduced content in synopsis.	To comply with new EU Clinical Trials Regulation	Section 1.1 Synopsis
Updated text to specify that contraception is required for IOCBP up to 21 days after the last dose of study intervention.	To be consistent with new data reported into the vepdegestrant IB 2025 regarding the timing after the last dose of study intervention for required use of highly	Section 10.4.2. Female Participant Reproductive Inclusion Criteria

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Description of Change	Brief Rationale	Section # and Name
	effective contraceptives. Being a combination, the most conservative timing has been used.	
Updated (AESI for vepdegestrant) venous embolism to venous thromboembolism (VTE).	To consistently use the term VTE	Section 2.3.1 Risk Assessment Section 8.4.8 Adverse Events of Special Interest
Delete text 'or their legally authorized representative's'	To comply with sponsor protocol template guidelines. The inclusion of incapacitated adults, individuals for whom a legally authorized representative [LAR] may provide consent is not an option in this study.	Section 1.1 Synopsis Section 8.4 Adverse Events, Serious Adverse Events, and Other Safety Reporting Section 9.2 Analysis Sets Section 10.1.3 Informed Consent Process
Details added for the evaluation of cases of acute kidney injury	To comply with sponsor protocol template guidelines	Section 10.7 Appendix 7: Kidney Safety Monitoring Guidelines
Revised language for the Reporting of AEs and SAEs to Pfizer Safety	To comply with sponsor protocol template guidelines and align with Safety reporting process.	Section 8.4.1.1 Reporting Nonserious AEs and SAEs to Pfizer Safety Section 8.4.3. Follow-Up of AEs and SAEs Section 10.3.3 Recording/Reporting and Follow Up of AEs and/or SAEs During the Active Collection Period Section 10.3.4 Reporting of SAEs
Clarifying statement added regarding list of prohibited drugs with the potential for DDI concerns.	To accommodate new/emerging information.	Section 10.9.1. CYP3A Inhibitors/Inducer
Added text to indicate that dispensing will cover 2 cycles of treatment	To be consistent with reduced SoA	Section 6.1.1 Administration

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Description of Change	Brief Rationale	Section # and Name
Added text to indicate that drug accountability will be performed on every dispensation visit (every 2 cycles)	To be consistent with timing of dispensing	Section 1.3 Schedule of Activities Section 6.5 Study Intervention Compliance
Revised language about telehealth visits	To align with sponsor protocol template guidelines	Section 8.1.2 Telehealth Visit
Revised language on AESI reporting	To comply with sponsor protocol template guidelines and align with Safety reporting process.	Section 8.4.8 Adverse Events of Special Interest
Revised language about reporting of medication errors	To comply with sponsor protocol template guidelines and align with Safety reporting process.	Section 8.4.10 Medication Errors
Language on informed consent, dissemination of clinical study data, medical records, publication policy, and medically qualified individual added.	To comply with sponsor protocol template guidelines	Section 10.1.3 Informed Consent Process Section 10.1.6 Dissemination of Clinical Study Data Section 10.1.9 Use of Medical Records Section 10.1.11 Publication Policy Section 10.1.12 Sponsor's Medically Qualified Individual
To specify in the Appendix that clinical laboratory tests may be performed as clinically indicated and documented in site source but not in the eCRF upon implementation of Amendment	In alignment with the updated SoA	Section 10.2 Appendix 2: Clinical Laboratory Tests
Language for Country Specific Requirements for Europe has been amended	To comply with sponsor protocol template guidelines. The single global version of the	Section 10.10.1 European Union

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Description of Change	Brief Rationale	Section # and Name
	protocol details specific local requirements into Appendix	
Previous SoA tables (from Amendment 2) moved to Section 10.14 as no longer applicable upon implementation of Protocol Amendment 3.	Previous SoA table moved to Appendix for reference purposes. To prevent sites from incorrectly treating per past SoA tables vs. current SoA tables.	Section 1.3 Schedule of Activities Section 10.14 Protocol Amendment History
The term “WOCBP ” has been updated to “ IOCBP”	To comply with sponsor protocol template guidelines	Section 1.3 Schedule of Activities Section 4.2.3. Choice of Contraception/Barrier Requirements Section 5.1 Inclusion Criteria 8.1.2 Telehealth Visits 8.1.3 Home Health Visits Section 8.3.5 Pregnancy Testing Section 10.4. Contraceptive and Barrier Guidance Section 10.15 Abbreviations
Typographical errors were corrected, and minor edits were made	To correct grammatical errors, to maintain consistency, and/or to increase clarity.	Throughout protocol
This change incorporates the PACL dated 01-Feb-2024. Updated the process for contacting a medically qualified individual.	The process for contacting a medically qualified individual has changed from a medical escalation process via a Pfizer	Section 10.1.12 Sponsor's Medically Qualified Individual Section 10.15 Abbreviations

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Description of Change	Brief Rationale	Section # and Name
	Call Center to direct clinical team contact using a Study Team Contact List. The Emergency Contact Card is being replaced by a study information card and will no longer be referenced.	
Protocol updated per PACL dated 12 June 2025. • Assessment of causality: clarified that for relatedness, "Yes" is related, and "No" is unrelated. • Outcome assessment: each outcome option was further clarified.	Changes implemented through a PACL were incorporated.	Section 10.3.3 Recording/Reporting and Follow-Up of AEs and/or SAEs During the Active Collection Period
Protocol updated per PACL dated 06 October 2025. Any pharmacokinetic assessments are no longer required.	All scheduled pharmacokinetic (PK) sample collections for this study have been completed. No additional PK assessments, including unscheduled samples, are required.	Section 8.5 Pharmacokinetics

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1. PROTOCOL SUMMARY

1.1. Synopsis

Protocol Title: A Phase 3, Randomized, Open-Label, Multicenter Study Of Vepdegestrant (ARV-471/PF-07850327) Plus Palbociclib Versus Letrozole Plus Palbociclib for the Treatment of Participants With Estrogen Receptor Positive, HER2-Negative Breast Cancer Who Have Not Received Any Prior Systemic Anti-Cancer Treatment for Advanced Disease (VERITAC-3)

Brief Title: Phase 3, Randomized, Open-Label Study of Vepdegestrant (ARV-471) Plus Palbociclib vs Letrozole Plus Palbociclib in Participants With ER(+)/HER2(-) Advanced Breast Cancer (VERITAC-3)

Regulatory Agency Identification Number(s):

US IND Number:	140264
EU CT Number:	2022-500545-24-00
ClinicalTrials.gov ID:	NCT05909397
Pediatric Investigational Plan Number:	Not Applicable
Protocol Number:	C4891002
Phase:	3

Rationale:

The purpose of this study is to demonstrate that Vepdegestrant (ARV-471) in combination with palbociclib provides superior clinical benefit compared to letrozole in combination with palbociclib in with ER(+)/HER2(-) aBC who have not received any prior systemic anti-cancer therapies for their locoregionally advanced or metastatic disease.

Rationale for Protocol Amendment 3:

The main purpose of the amendment is to reduce the Schedule of Activities with clinical assessments per standard of care or clinical judgment of the investigator. This amendment also reflects the previous decision that the Phase 3 portion of the study will not be conducted, due to a Sponsor decision and not for safety, efficacy reasons or requests from Regulatory Agencies; SLI Phase enrollment was completed for the planned participants.

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Objectives, Endpoints, and Estimands:

Study Lead-in		
Objectives	**Endpoints**	**Estimands**
Primary		
• To determine the RP3D of palbociclib when given in combination with vepdegestrant.	• Incidence of Grade 4 neutropenia AE (as graded by NCI CTCAE v5.0) with onset within the first 4 cycles. • Incidence of dose reduction or drug discontinuation within the first 4 cycles.	• See Section 9.1.1
Secondary		
• To evaluate safety and tolerability of both dose levels.	• Incidence of AEs and SAEs • Incidence of laboratory abnormalities. • Incidence of ECG abnormalities.	• Not Applicable
• To evaluate measures of tumor control and duration of response in both dose levels.	• Objective Response (OR: confirmed CR or PR) by investigator assessment per RECIST v1.1. • Clinical Benefit Response (CBR: confirmed CR or PR at any time or SD or non-CR/non-PD ≥24 weeks) by investigator assessment per RECIST v 1.1. • Duration of Response, defined as the time from the first documentation of CR or PR to the date of the first documentation of PD, as determined by investigator assessment per RECIST v1.1, or death due to any cause, whichever occurs first.	• Not Applicable
• To evaluate the concentrations of vepdegestrant and ARV-473	• Plasma concentrations of vepdegestrant and ARV-473.	• Not Applicable
• To evaluate the concentrations of palbociclib.	• Plasma concentrations of palbociclib.	• Not Applicable

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No participants will be enrolled in the Phase 3 portion of the study.

Phase 3		
Objectives	**Endpoints**	**Estimands**
Primary		
• To demonstrate that vepdegestrant in combination with palbociclib is superior to letrozole plus palbociclib in prolonging progression free survival (PFS) in participants with ER(+)/HER2(-) aBC who have not received any prior systemic anticancer therapy for their advanced disease.	• Progression-free survival (PFS), defined as the time from the date of randomization to the date of first documented disease progression, as determined by BICR assessment per RECIST v1.1, or death due to any cause, whichever occurs first.	• See Section 9.1.2.1
Key Secondary		
• To demonstrate that vepdegestrant in combination with palbociclib is superior to letrozole in combination with palbociclib, in prolonging overall survival (OS).	• Overall Survival (OS), defined as the time from the date of randomization to the date of death due to any cause.	• See Section 9.1.2.2
Secondary		
• To compare measures of tumor control between treatment arms and evaluate duration of response within each treatment arm.	• Objective Response (OR: confirmed CR or PR) by BICR assessment per RECIST v1.1. • Clinical Benefit Response (CBR: confirmed CR or PR at any time or SD or non-CR/non-PD ≥24 weeks) by BICR assessment per RECIST v 1.1. • Duration of Response, defined as the time from the first documentation of CR or PR to the date of the first documentation of PD, as determined by BICR assessment per RECIST v1.1, or death due to any cause, whichever occurs first.	• Not Applicable
• To evaluate safety and tolerability of both treatment arms.	• Incidence of AEs and SAEs • Incidence of laboratory abnormalities. • Incidence of ECG abnormalities.	• Not Applicable
• To evaluate the concentrations of vepdegestrant and ARV-473	• Plasma concentrations of vepdegestrant and its epimer ARV-473.	• Not Applicable
• To evaluate the concentrations of palbociclib	• Plasma concentrations of palbociclib.	• Not Applicable
• To evaluate patient reported outcomes between 2 treatment arms	• EuroQol EQ-5D-5L • EORTC QLQ-C30 • EORTC QLQ-BR23	• Not Applicable
• To assess changes from baseline levels in plasma ctDNA	• ctDNA plasma quantitative changes from pre-treatment to evaluate potential predictability of their associations with clinical outcomes.	• Not Applicable

Overall Design

The SLI Phase enrollment has been completed for the planned participants; enrollment in the Phase 3 portion of the study will not proceed.

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This is an international Phase 3 randomized, open-label, parallel-group, multicenter clinical trial aimed to demonstrate that vepdegestrant in combination with palbociclib is superior to letrozole in combination with palbociclib in prolonging the progression-free survival (PFS by BICR assessment) in female and male participants with ER(+)/HER2(-) aBC who have not received any prior systemic anti-cancer treatment for their advanced disease.

This study will have a Study Lead-in (SLI) aimed to evaluate safety and tolerability in participants who have not received any prior systemic anti-cancer treatment for aBC, to select the Recommended Phase 3 Dose (RP3D) of palbociclib to be used in combination with vepdegestrant in Phase 3.

In the SLI, 2 dose levels will be explored in parallel. Approximately 50 participants will be randomly assigned on a 1:1 basis to:

- Dose Level 1 (n ≈ 25). Participants will receive:
    - vepdegestrant 200 mg orally, once daily, continuously, in a 28-day cycle, *plus*
    - Palbociclib 100 mg orally, once daily for 21 consecutive days followed by 7 days off treatment in a 28-day cycle.
- Dose Level 2 (n ≈ 25). Participants will receive:
    - vepdegestrant 200 mg orally, once daily, continuously, in a 28-day cycle, *plus*
    - Palbociclib 75 mg orally, once daily for 21 consecutive days followed by 7 days off treatment, in a 28-day cycle.

When the RP3D in the SLI has been selected, the randomized, Phase 3 will start.

In Phase 3, approximately 1130 participants will be randomly assigned on a 1:1 basis to:

- Arm A (Investigational Arm; n ≈ 565): Participants will receive:
    - vepdegestrant 200 mg orally, once daily, continuously, in a 28-day cycle, *plus*
    - Palbociclib per RP3D defined in the SLI orally, once daily for 21 consecutive days followed by 7 days off treatment in a 28-day cycle.
- Arm B (Comparator Arm; n ≈ 565): Participants will receive:
    - Letrozole 2.5 mg orally, once daily, continuously, in a 28-day cycle, *plus*
    - Palbociclib 125 mg orally, once daily for 21 consecutive days followed by 7 days off treatment, in a 28-day cycle.

All the study interventions must be taken with food.

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In Phase 3, participants will be stratified by menopausal status at study entry: (pre-/peri-[including men] vs post-menopausal), visceral disease: (yes/no), and de novo metastatic disease: (yes/no). Visceral disease refers to lung, liver, brain, pleural and peritoneal involvement.

Pre-/peri-menopausal female participants and male participants must receive therapy with an LHRH agonist starting at Cycle 1 Day 1 (if not already ongoing) and continue throughout the study as outlined in the SoA.

Participants in the SLI will not be included in the Phase 3 study. Crossover between treatment arms is NOT allowed.

An independent external data monitoring committee (E-DMC) will be established to review safety data to monitor safety and tolerability of vepdegestrant in combination with palbociclib, as well as interim analysis data of the Phase 3. Data collected in the SLI will be provided to the E-DMC. The roles and responsibilities of the E-DMC will be described in a separate charter.

A Global Steering Committee will be established at the vepdegestrant program level. It will be responsible for providing objective strategic scientific advice and medical input on the ongoing and future clinical development plans for vepdegestrant in ER(+) breast cancer including for this study.

Participants will undergo efficacy assessments as outlined in the SoA. In SLI, efficacy analyses will be performed using the local radiologist's/investigator's tumor assessments as the primary data source. In Phase 3, efficacy analyses will be performed using a BICR laboratory as the primary data source. In Phase 3, all radiographic images, as well as other information collected on-study will be verified by BICR to determine the protocol defined endpoints of disease response and progression.

SLI will be considered as completed at the date of the last scheduled procedure of the last SLI participant. In Phase 3, it is anticipated that the final PFS analysis will occur at approximately 575 PFS events, and the final OS analysis will occur at approximately 473 OS events.

Number of Participants

Approximately 50 and 1130 participants will be randomized in SLI and Phase 3, respectively.

Note: "Enrolled" means a participant's agreement to participate in a clinical study following completion of the informed consent process and randomization to study intervention.

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The SLI Phase enrollment has been completed for the planned participants; the Phase 3 portion of the study will not be conducted. This is due to a Sponsor decision, and is not for safety, efficacy reasons or requests from Agencies.

Study Population

SLI and Phase 3 will have the same Inclusion/Exclusion criteria, except for inclusion criterion on blood and tumor biopsy provision which applies to participants for Phase 3 only.

Key inclusion and exclusion criteria are listed below:

Key Inclusion Criteria

Participants must be adults with ER+/HER2- A/MBC, who have not been previously treated with prior therapies for advanced/metastatic disease. Participants must have at least 1 measurable lesion as per RECIST v1.1, or for those with bone only disease, those with only non-measurable lesions are eligible.

Key Exclusion Criteria

Participants with any of the following characteristics/conditions will be excluded:

- Prior treatments:
    - Primary endocrine resistance: relapse while on the first 2 years of standard adjuvant ET.
    - Secondary endocrine resistance:
        - relapse while on standard adjuvant ET but after the first 2 years
        - relapse within 12 months of completing standard adjuvant ET
    - CDK4/6i, vepdegestrant, fulvestrant, elacestrant and other investigational agents (including novel ET, any SERDs, SERCAs, CERANs) in any setting.
- Lack of adequate bone marrow, liver and kidney function
- Impaired cardiovascular function or clinically significant cardiovascular diseases.
- Refractory nausea and vomiting, chronic GI disease, GI ulcer, GI bleeding, inability to swallow the formulated product or previous significant gastric (total or partial), bowel resection that would preclude adequate absorption of study interventions.

Study Arms and Duration:

In both SLI and Phase 3, the screening period will occur within 28 days prior to randomization. Study intervention will be administered in 28-day cycles. Participants will continue to receive assigned study intervention until objective disease progression by RECIST v1.1, unacceptable toxicity, death, participant refused further treatment, or one of the other reasons for treatment discontinuation as specified in the protocol is met.

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Information on vepdegestrant and palbociclib doses and administration is provided in the tables below. Dose modifications (ie, dose reduction, interruption and cycle delay) for toxicity of vepdegestrant and/or palbociclib are allowed.

Information on letrozole dose and administration is provided in the Phase 3 tables below. No dose reduction of letrozole is permitted.

Study Lead-in Study Arms		
Arm Title	Dose Level 1	Dose Level 2
Arm Type	Investigational	Investigational
Arm Description	Participants will receive: ○ Vepdegestrant as two 100 mg tablets (ie, 200 mg QD), orally, once daily, continuously, in a 28-day cycle, plus ○ Palbociclib as a 100 mg capsule, orally, once daily for 21 consecutive days followed by 7 days off treatment, in a 28-day cycle.	Participants will receive: • Vepdegestrant as two 100 mg tablets (ie, 200 mg QD), orally, once daily, continuously, in a 28-day cycle, plus • Palbociclib as 75 mg capsule orally, once daily for 21 consecutive days followed by 7 days off treatment, in a 28-day cycle.

Phase 3 Study Arms		
Arm Title	A	B
Arm Type	Investigational	Comparator
Arm Description	Participants will receive: • Vepdegestrant as two 100 mg tablets (ie, 200 mg QD), orally, once daily, continuously, in a 28-day cycle, plus • Palbociclib per RP3D defined in the SLI, as a capsule, orally, once daily for 21 consecutive days followed by 7 days off treatment, in a 28-day cycle.	Participants will receive: • Letrozole as 2.5 mg tablet, orally, once daily, continuously, in a 28-day cycle, plus • Palbociclib as 125 mg capsule orally, once daily for 21 consecutive days followed by 7 days off treatment, in a 28-day cycle.

Ethical Considerations:

Efficacy data as single agent and in combination with palbociclib from nonclinical studies harboring clinically relevant ESR1 mutations associated with resistance to ET, as well as preliminary data from the ongoing FIH vepdegestrant-mBC-101 study, suggest that vepdegestrant co-administered with palbociclib may represent an important therapeutic option improving upon standard ET to benefit in participants with ER(+)/HER2(-) aBC who have not received any prior systemic anti-cancer therapies for their advanced disease.

Initial safety data reported from participants treated with vepdegestrant monotherapy across multiple doses in the vepdegestrant-mBC-101 FIH study (Part A dose escalation of vepdegestrant of total daily doses of 30 to 700 mg QD and Part B dose expansion of vepdegestrant at 200 mg QD and 500 mg QD, N=149) showed vepdegestrant was well tolerated, without any dose limiting toxicities. Most treatment-related adverse events were Grade 1 or Grade 2 in severity. Data from the monotherapy dose escalation and dose expansion at vepdegestrant 200 mg QD and 500 mg QD support the monotherapy dose

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selection of vepdegestrant 200 mg QD as RP3D, based on the lack of clear differences in efficacy (by CBR) and no meaningful differences in the overall safety between vepdegestrant 200 mg QD and 500 mg QD, except for an apparent higher risk of potential QTc prolongation and dose reductions observed at vepdegestrant 500 mg QD monotherapy dose. In addition, based on preliminary safety data from participants treated with vepdegestrant in combination with palbociclib 125 mg in the vepdegestrant-mBC-101 FIH study (Part C, N=46), the combination appears tolerable. The safety profile of vepdegestrant in combination with palbociclib approved dose is generally consistent with that observed after treatment with vepdegestrant monotherapy (Parts A and B) except for Grade 4 neutropenia, that is higher than expected based on historical data of palbociclib in combination with endocrine therapy. In heavily pre-treated participants (median of 3 prior lines in the metastatic setting), all neutropenia episodes occurred shortly after initiation of the treatment (first 2 cycles) and resolved within the cycle of occurrence. Neutropenia was medically manageable with/or without supportive care and/or dose adjustments as per the palbociclib prescribing information. Promising preliminary efficacy by CBR and ORR was observed in Part C for vepdegestrant 200 mg QD in combination with palbociclib as well as for all vepdegestrant doses.
Preliminary PK data from Part C suggests a lack of effect of palbociclib on vepdegestrant exposures, while palbociclib exposure in combination with vepdegestrant (C_{max} and AUC_{tau}) was higher than in historical studies (ie, PALOMA 1 and PALOMA 2) with similar vepdegestrant effect on palbociclib exposure regardless of vepdegestrant doses within the tested range. These data suggest there would be benefit in collecting additional safety data of the combination vepdegestrant plus palbociclib in 1L participants testing lower doses of palbociclib (ie, 100 mg and 75 mg QD). This will be done in a SLI before Phase 3 activation with a goal of optimizing the combination vepdegestrant plus palbociclib dose to be used in the Phase 3 study.

Based on the known safety profiles of vepdegestrant as a single agent or in combination with palbociclib, the risk of having clinically relevant overlapping toxicities with this experimental combination is assessed as low, making the combination desirable in the target participants population.

Considering the measures taken to minimize risks to study participants, the potential risks identified in this study for vepdegestrant administered in combination with palbociclib are justified by the anticipated benefits that may be afforded to participants with ER(+)/HER2(-) aBC.

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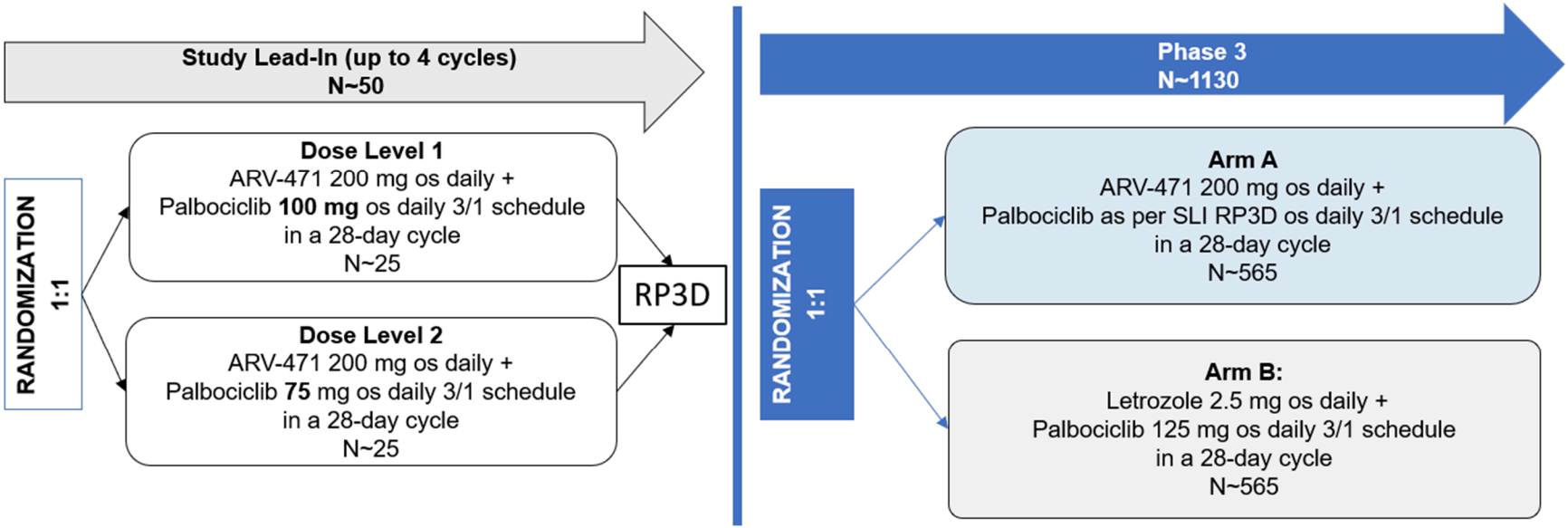
1.2. Schema



The Phase 3 portion of the study will not be conducted.

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1.3. Schedule of Activities

The SoA table provides an overview of the protocol visits and procedures. After implementation of Protocol Amendment 3, Table 1 supersedes the prior SoA tables (available in Section 10.14). Refer to the STUDY ASSESSMENTS AND PROCEDURES section of the protocol for detailed information on each assessment required for compliance with the protocol.

The investigator may schedule visits (unplanned visits) in addition to those listed in the SoA table, in order to conduct evaluations or assessments required to protect the well-being of the participant.

1.3.1. Schedule of Activities: Study Lead-in

Table 1. Study Schedule of Assessments: Study Lead-in (after Implementation of Amendment 3)

Protocol Activity	**Combination Treatment Phase (1 Cycle = 28 days)**	**EOT / Withdrawal[a]**	**Post-Treatment FU**		**Notes:**
	Visits on Day 1 of every 2 cycles (1 cycle = 28 days)		**Safety FU[b]**	**Long Term FU[c]**	**All assessments should be performed prior to study intervention administration, unless otherwise specified.**
Visit Window (days)	**±4d**		**+7d**	**±14d**	**There is a maximum of 60 days between visits during the Active Treatment Phase.**
Clinical Assessments					
Physical Examination & Vital signs	As per clinical standard of care at the discretion of the investigator.				To be performed as clinically indicated per PI/site standard of care and documented in site source only. Any observed AE must be reported in the eCRF. See Sections 8.3.1 and 8.3.2.
12-Lead ECG	As per clinical standard of care at the discretion of the investigator.				To be performed as clinically indicated per PI/site standard of care and documented in the site source only. Any AE resulting from an abnormal ECG must be reported in the eCRF. See Section 8.3.3.
Contraception Check for IOCBP and Males	X (every cycle)	X	X	X *(Men only)*	See Section 5.3.1.
Clinical Laboratory Assessments					
Hematology	As per clinical standard of care at the discretion of the investigator.				To be performed as clinically indicated per PI/site standard of care and documented in the site source only. Any AE resulting from an abnormal test result must be reported in the eCRF. See Sections 8.3.4 and Appendix 2.
Blood Chemistry	As per clinical standard of care at the discretion of the investigator.				

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Table 1. Study Schedule of Assessments: Study Lead-in (after Implementation of Amendment 3)

Protocol Activity	**Combination Treatment Phase (1 Cycle = 28 days)**	**EOT / Withdrawal[a]**	**Post-Treatment FU**		**Notes:**
	Visits on Day 1 of every 2 cycles (1 cycle = 28 days)		**Safety FU[b]**	**Long Term FU[c]**	**All assessments should be performed prior to study intervention administration, unless otherwise specified.**
Visit Window (days)	**±4d**		**+7d**	**±14d**	**There is a maximum of 60 days between visits during the Active Treatment Phase.**
Pregnancy Test (as applicable)	X (to be performed every cycle)	X	X		To be documented in site source. See Section 8.3.5 and Appendix 2.
Disease Assessments					
Tumor Assessments (other than Bone scan)	As per clinical standard of care at the discretion of the investigator.				Treatment beyond progression is no longer part of the study after Amendment 3 implementation. See Section 8.2.2.After implementation of Amendment 3, participants who discontinue study treatment for reasons other than radiographically and/or clinically (ie, for photographed lesions) documented PD as per RECIST v1.1 will not be required to have tumor assessments performed after permanent treatment discontinuation. See Section 8.2
Bone Scan, Whole Body	As per clinical standard of care at the discretion of the investigator.				
Study Intervention and LHRH Agonist					
vepdegestrant	Once Daily				See Section 6.1
Palbociclib	Once Daily on Days 1-21 followed by 7 days off				See Section 6.1
LHRH agonist (for Male and Pre-/Perimenopausal female participants)	As per local package insert			X (*at PI discretion*)	Treatment should start on Cycle 1 Day 1 and continue until 28 days after the last dose of study intervention. Long Term FU: LHRH agonist should be continued at PI discretion. See Section 6.9.1.
Other Clinical Assessments					
Drug Compliance	X				Drug accountability will be performed prior to dispensing study interventions for the following cycles. See Sections 6.2 and 6.5.
Adverse Event Reporting	X	X	X		Any observed AEs must be reported in the eCRF. See Section 8.4.1 and Appendix 3.

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Table 1. Study Schedule of Assessments: Study Lead-in (after Implementation of Amendment 3)

Protocol Activity	**Combination Treatment Phase (1 Cycle = 28 days)**			**EOT / Withdrawal[a]**	**Post-Treatment FU**		**Notes:**
	Visits on Day 1 of every 2 cycles (1 cycle = 28 days)				**Safety FU[b]**	**Long Term FU[c]**	**All assessments should be performed prior to study intervention administration, unless otherwise specified.**
Visit Window (days)	**±4d**				**+7d**	**±14d**	**There is a maximum of 60 days between visits during the Active Treatment Phase.**
Concomitant Medications/ Treatments	X			X	X		To be recorded in site source only (and on SAE form if applicable) but not in the eCRF. See Section 6.9.
FU anticancer therapy					X		First subsequent anticancer therapy data will be collected. See Section 7.1.1.2.

a. End of Treatment/Withdrawal: As soon as study intervention is permanently discontinued and no more than 28 calendar days after last dose of oral study intervention.
b. Safety FU is a minimum of 28 days from last dose of study intervention OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1).
c. Long Term FU is 3 months after EOT (See section 7.1.1.2)

The Phase 3 portion of the study will not be conducted.

2. INTRODUCTION

Vepdegestrant (also known as PF-07850327 or ARV-471) is a potent, selective, orally bioavailable PROteolysis TArgeting Chimeric (PROTAC®) small molecule that induces degradation of the estrogen receptor (ER). In this study (VERITAC-3), vepdegestrant plus palbociclib (IBRANCE®) is being investigated in participants with ER(+), human epidermal growth factor receptor negative (HER2[-]) advanced/metastatic breast cancer (BC) who have not received any prior systemic anticancer therapy for advanced disease. The investigational drugs vepdegestrant (PF-07850327 or ARV-471) and palbociclib (IBRANCE®) are identified throughout the protocol as vepdegestrant and palbociclib.

The study population, ie, participants with unresectable locoregionally advanced or metastatic BC will be reported as participants with advanced breast cancer (aBC) throughout the protocol.

Unless otherwise specified, all text is applicable to both SLI and Phase 3.

2.1. Study Rationale

The purpose of this study is to demonstrate that vepdegestrant in combination with palbociclib provides superior clinical benefit compared to letrozole in combination with palbociclib in participants with ER(+)/HER2(-) aBC who have not received any prior systemic anti-cancer therapies for their advanced disease.

2.2. Background

2.2.1. Breast Cancer

Female BC has surpassed lung cancer as the leading cause of global cancer incidence in 2020, with an estimated 2.3 million new cases, representing 11.7% of all cancer cases. BC is the fifth leading cause of cancer mortality worldwide, with 685,000 deaths (Sung et al, 2021) and the second leading cause of cancer death in women, with over 43,250 women expected to die from the disease in 2022 in the US (Siegel et al, 2022). While BC is less common in men, men account for approximately 1% of all newly diagnosed cases, and almost 530 men are projected to die from their disease in 2022 (Siegel et al, 2022).

Globally, the number of people living with mBC is increasing, primarily because of improvements in available treatment options. In US, the estimated number of women living with mBC increased by 17% from 2000 to 2010 and was projected to increase by 31% from 2010 to 2020 (Mariotto et al, 2017). In this country, the vast majority (approximately 73%) have HR(+) and HER2(-) disease (Howlader et al, 2014).

Although age adjusted mortality from BC has been decreasing during the last decade, the median survival for patients with metastatic disease is still approximately 18-24 months, and there remains an unmet medical need for more active agents in the advanced setting (Zheng et al, 2020).

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2.2.2. Treatment Options for ER-positive Breast Cancer

Treatment options for patients with aBC depend on many different factors, including whether the tumors express hormone receptors (ER and/or PR) or HER2. The standards of care for people with ER(+) aBC are represented by endocrine therapy (ET, ie, tamoxifen, letrozole, anastrozole, exemestane, and fulvestrant) and targeted therapies (CDK4/6i, PI3Ki, and mTORi, PARPi), administered as a single agent or co-administered and/or chemotherapy.

The most important advances in ET involve targeting of ER signaling, which has evolved over time from the use of SERMs (tamoxifen) to AIs (letrozole, anastrozole, exemestane), and SERDs (fulvestrant). Combination of ET with targeted agents has further improved the outcome of the patients. Fulvestrant is the first SERD developed and approved for ER(+) BC and is administered via intramuscular injection in the clinical setting. Fulvestrant inhibits estrogen signaling through the ER primarily in 2 ways: 1) competitive inhibition of estradiol binding to the ER, blocking nuclear localization, and 2) accelerated degradation of the ER protein due to the instability of the fulvestrant-ER complex. Fulvestrant's destabilizing mechanism has validated the importance of ER degradation as a therapeutic approach. After fulvestrant treatment, ER declined significantly compared to pre-treatment reaching its lowest levels at 6 months (Month 6 median ER Hscore of 48 [2-135] versus pretreatment median ER Hscore of 130 [60-190]). This ER level was generally maintained at time of fulvestrant progression (median ER Hscore of 50 [8-130]) showing that no patient had lost all ER at relapse even in the longest interval between treatment and progression of 60.7 months (Agrawal et al, 2016), indicating a need for more effective ER-degrading molecules.

AIs are oral agents that are approved worldwide for the 1L treatment alone or in combination with CDK4/6i in patients with ER(+) aBC. The paradigm in the treatment of aBC changed after the PALOMA-2 study confirmed that the addition of palbociclib to letrozole resulted in a statistically significant, and clinically meaningful 10-month improvement in mPFS (42% risk reduction) as compared with placebo plus letrozole (Finn et al, 2016).

The PALOMA-2 study was conducted in post-menopausal women with ER(+)/HER2(-) aBC who had not received any prior systemic anti-cancer therapies for their advanced/metastatic disease; mPFS was 24.8 months (95% CI: 22.1 to not estimable) in the palbociclib–letrozole group, as compared with 14.5 months (95% CI: 12.9-17.1) in the placebo–letrozole group (HR for disease progression or death=0.58; 95% CI: 0.46-0.72; p<0.001). In PALOMA-2, the most common treatment emergent AEs (≥10% of patients) in the palbociclib-letrozole group were neutropenia, leukopenia, fatigue, nausea, arthralgia, and alopecia. With the exclusion of neutropenia and leukopenia, 57.0% of the patients reported an AE with a maximum Grade of 1 or 2 and 39.2% of patients reported events of Grade 3 or higher. Febrile neutropenia was reported in 1.8% of patients in the palbociclib–letrozole group and in no patients in the placebo-letrozole group. There was no substantial difference between-group with respect to infections of Grade 3 or higher. The most common nonhematologic AEs were fatigue (occurring in 37.4% of the patients in the palbociclib-letrozole group vs 27.5% in the placebo–letrozole group), nausea (35.1% vs 26.1%), and arthralgia (33.3% vs 33.8%). Other AEs for which the incidence was higher in the palbociclib–letrozole group

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than in the placebo–letrozole group were diarrhea (26.1% vs 19.4%), cough (25.0% vs 18.9%), and stomatitis (15.3% vs 5.9%) (Finn et al, 2016).

Recent advances in novel compounds, such as CDK4/6i or PI3Ki (such as alpelisib) and mTORi (such as everolimus), have improved PFS and OS in these patients and are used with a backbone of ET (McAndrew & Finn, 2021). CDK4/6i combined with ET are the standard of care for the 1L treatment of ER(+)/HER2(-) aBC with improved efficacy in term of PFS and OS and a good toxicity profile seen in several trials. CDK4/6 inhibition plus ET yields similar or better efficacy versus chemotherapy and is associated with less toxicity, making it the preferred treatment option unless a patient has imminent organ failure. Available treatments for ER(+)/HER2(-) aBC are summarized in Table 2.

Palbociclib is indicated for the treatment of ER(+) aBC and is given on a backbone of ET (AIs or fulvestrant) and is the chosen coadministered agent for many novel ETs in 1L randomized studies versus palbociclib/AIs.

However, despite the significant clinical benefit of ET alone or in combination with targeted agents, some patients still develop endocrine resistance after or during ET. Endocrine resistance is a clinical challenge and can present at diagnosis (de novo/intrinsic resistance) or after treatment with endocrine therapy (acquired resistance). Intrinsic resistance is thought to be present in up to 30% of patients and due to lack of expression of ER (Belachew & Sewasew, 2021). Approximately 30%-50% of patients will develop SERD/SERM/ AI-resistant ER(+) metastatic lesions after an average of 5 years (Rozeboom, Dey & De, 2019);(Fanning & Greene, 2019). The development of ET-resistant phenotypes involves multiple mechanisms. These include: a) ERα mutations (where ESR1 gene mutations are one of the most studied) that leads to ligand-independent constitutive activation of ER, b) in the absence of estradiol, the activation of ERα because of signaling pathway activation, and c) variation in the expression of ERα cofactors responsible for the ERα modulator activity. In this scenario, SERDs are one of the treatments under research and development against AI- or tamoxifen-resistance. Fulvestrant has demonstrated activity in patients ESR1 wild-type and ESR1 mutated tumors in the second-line treatment setting with 4.1 months (95% CI: 3.6-5.5) mPFS in patients without ESR1 mutation and 3.9 months (95% CI: 3.0-6.0) mPFS in patients with ESR1 mutation (mESR1) (Turner et al, 2020).

Recent investigational advances have allowed the development of new oral bioavailable SERDs. Elacestrant, an investigational oral SERD, has demonstrated a statistically significant improvement in PFS over standard of care (ie, fulvestrant or AIs) in patients with ER(+)/HER2(-) mBC in second- and third-line settings (Bardia et al, 2022). Approximately half of the patients enrolled had tumors with ESR1 mutations. Study results showed a statistically significant improvement of mPFS in the elacestrant arm compared to SOC in all participants with a median PFS of 2.79 vs 1.91 months (HR=0.697 [95% CI: 0.552, 0.880]; P=0.0018) and in participants with ESR1 mutant BC with 3.78 vs 1.87 months mPFS (HR=0.546 [95% CI: 0.387, 0.768]; P=0.0005). Preliminary OS data, although not yet mature, demonstrated a trend in favor of elacestrant in all participants (HR=0.751 [95% CI: 0.542, 1.038]; P=0.0821) and in participants with ESR1 mutant BC (HR=0.592 [95% CI: 0.361, 0.958]; P=0.0325).

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An unmet need exists to improve upon standard ET to benefit patients with ER(+) BC by preventing, delaying, or overcoming therapeutic resistance, thereby prolonging PFS in the metastatic setting. Additional benefits with improved ET could be prolongation of the time to deterioration of quality of life (eg, worsening of pain) or delay in the time to use of cytotoxic chemotherapy with all the associated toxicity and QoL impacts.

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Table 2. Targeted Therapies (CDK4/6, mTOR, PI3K inhibitors) and Endocrine Combinations Registered for ER(+)/HER2(-) aBC.

	PALOMA-2[1, 14]	MONALEESA-2[2]	MONARCH-3[3]	MONALEESA-7[4, 5]	MONALEESA-3[6,7]	PALOMA-3[8,9]	MONARCH-2[10,11]	BOLERO-2[12]	SOLAR-1[13]	
Experimental Arm	Palbociclib Letrozole	Ribociclib Letrozole	Abemaciclib Letrozole or Anastrozole	Ribociclib + Tamoxifen or Letrozole or Anastrozole	Ribociclib Fulvestrant	Palbociclib Fulvestrant	Abemaciclib Fulvestrant	Everolimus Exemestane	Alpelisib Fulvestrant	
Control Arm	Placebo Letrozole	Placebo Letrozole	Placebo Letrozole or anastrozole	Placebo Tamoxifen or letrozole or anastrozole	Placebo Fulvestrant	Placebo Fulvestrant	Placebo Fulvestrant	Placebo Exemestane	Placebo Fulvestrant	
ER mut/ other mut	NA	NA	NA	NA	NA	NA	NA	NA	PI3K mut	PI3K wt
Size (Number)	666	668	493	472	726	521	669	724	341	231
Randomization	2:1	1:1	2:1	1:1	2:1	2:1	2:1	2:1	1:1	1:1
Menopausal Status	Post	Post	Post	Pre*	Post	Pre*/peri*/post	Pre*/peri*/post	Post	Post**	Post
Study Population	1L	1L	1L	1L	1L/2L	≥2L	-	≥2L	≥2L	≥2L
Prior ET for aBC	No	No	No	No	Yes	Yes	Yes	Yes	Yes	Yes
Prior CT in aBC	No	No	No	No	No	Yes (1 line)	No	Yes	No	No
ORR (%) measurable	55.3 vs 44.4	54.5 vs 38.8	61.0 vs 45.5	51 vs 36	40.9 vs 28.7	25.0 vs 11.0	48.1 vs 21.3	NA	35.7 vs 16.2	NA
mPFS (mos) HR 95% CI P value	24.8 vs 14.5 0.58 0.46 -0.72 <0.0001	25.3 vs 16.0 0.568 0.457-0.704 $=9.63 \times 10^{-8}$	28.2 vs 14.8 0.540 0.418-0.698 =0.000002	23.8 vs 13.3 0.55 0.44-0.69 <0.0001	20.5 vs 12.8 0.593 0.480-0.732 <0.001	9.5 vs 4.6 0.46 0.36 -0.59 <0.0001	16.4 vs 9.3 0.553 0.449 -0.681 <0.001	6.9 vs 2.8 0.43 0.35-0.54 <0.001	11.0 vs 5.7 0.65 0.50 -0.85 <0.001	7.4 vs 5.6 0.85; 0.58 -1.25 NA
mOS (mos) HR 95% CI P value	53.9 vs 51.2 0.956 0.777-1.177 0.3378	NR vs 33.0 0.746 0.517-1.078 NA	NA	NE v 40.9 0.71 0.54 -0.95 0.00973	NR vs 40.0 0.72 0.57 -0.92 0.00455	34.9 vs 28.0 0.81 0.64 -1.03 0.09	46.7 vs 37.3 0.757 0.606 -0.945 0.01	NA	NA	NA

*plus LHRH agonist; **men participants also included

aBC= advanced breast cancer; CI=confidence interval; CT=chemotherapy; ET=endocrine therapy; HR=hazard ratio; m=median; mos= months; mut=mutated; NA=not available; NE=not evaluable; NR=not reached; mPFS= median progression-free survival; ORR=objective response rate; mOS=median overall survival; wt=wild type.

Source: 1 (Finn et al, 2016); 2 (Hortobagyi et al, 2018); 3 (Johnston et al, 2019); 4 (Tripathy et al, 2018); 5 (Im et al, 2019); 6 (Slamon et al, 2018); 7 (Slamon et al, 2020); 8 (Cristofanilli et al, 2016); 9 (Turner et al, 2018); 10 (Sledge, 2017); 11 (Sledge et al, 2020); 12 (Baselga et al, 2012); 13 (Andre et al, 2019) 14 (Finn et al, 2022).

2.2.3. vepdegestrant

vepdegestrant is a hetero-bifunctional PROTAC® molecule that simultaneously binds the ER and the cereblon E3 ligase complex, enabling protein-protein interactions between ER and the ligase complex. As a result, the ER becomes poly-ubiquitinated on accessible lysine residues and subsequently undergoes targeted degradation by the proteasome leading to its elimination from cells (vepdegestrant [PF-07850327, ARV-471] Investigator's Brochure).

In nonclinical studies, vepdegestrant demonstrated in vitro potent (<1 nM) and robust (>90%) ER degradation even against clinically relevant, mutated (Y537S and D538G) forms of ER. Based on nonclinical data, vepdegestrant may have advantages over fulvestrant and other orally available SERDs in clinical development such as elacestrant (RAD1901) (Flanagan et al, 2018). While SERDs destabilize ER and indirectly lead to ER degradation, vepdegestrant actively degrades ER, which may lead to less drug resistance. In vivo, vepdegestrant inhibits tumor growth in multiple mouse xenograft models. Notably, vepdegestrant demonstrates superior TGI compared to fulvestrant in a Y537S estrogen receptor (ESR1) mutant patient-derived xenograft model). In the MCF7-xenograft mouse model, 3 to 30 mg/kg vepdegestrant, orally administered to mice once daily for 28 days, displayed dose-dependent efficacy with doses of 3 and 10 mg/kg/day inhibiting tumor growth by 85% and 98%, respectively and 30 mg/kg/day leading to tumor shrinkage (124% TGI). At study termination, the tumor ER levels were reduced by ≥94%, suggesting that higher doses are required for maximal efficacy than for maximal ER degradation (vepdegestrant [PF-07850327, ARV-471] Investigator's Brochure).

Moreover, the combination of vepdegestrant with palbociclib resulted in significant tumor regression and anti-tumor activity when compared to palbociclib as a single agent or fulvestrant in combination with palbociclib. Additional nonclinical efficacy studies conducted in BC tumor models combining vepdegestrant with CDK4/6i abemaciclib and ribociclib confirmed the greater anti-tumor effect of the combination vepdegestrant plus CDK4/6i compared to the combination of fulvestrant with either of these CDK4/6i (vepdegestrant [PF-07850327, ARV-471] Investigator's Brochure).

These nonclinical data suggest that vepdegestrant has the potential to offer improved ER degradation and a more convenient route of administration as compared to fulvestrant (oral vs intramuscularly administered SERD) either as monotherapy or administered in combination with palbociclib as well as with other CDK4/6i; coadministration could potentially address a need in 1L therapy for ER(+)/HER2(-) aBC.

2.2.3.1. Nonclinical Pharmacokinetics and Metabolism

vepdegestrant is a BCS Class IV compound (low solubility/low permeability). The PK profile of vepdegestrant in the nonclinical species (mouse, rat, dog, and monkey) was characterized by a low to moderate clearance (13.8% to 33.8% of hepatic blood flow), extensive tissue distribution (2.1 to 6.0 L/kg), short to moderate half-life of elimination ($t_{1/2}$) (2.1 to 8.2 hours), and moderate to good oral bioavailability (27% to 65%). vepdegestrant can interconvert to its epimer, ARV-473. Preclinical data demonstrates that the exposure of

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ARV-473 is limited compared to vepdegestrant. (vepdegestrant [PF-07850327, ARV-471] Investigator's Brochure).

A dose-dependent increase in vepdegestrant exposure was observed when vepdegestrant was administered as an oral solution in the mouse (10, 30, 100 mg/kg), rat (30, 100, 300 mg/kg), dog (15, 45, 90, 200 and 400 mg/kg), and monkey (1 and 3 mg/kg) (vepdegestrant [PF-07850327, ARV-471] Investigator's Brochure).

A 3-fold increase in AUC and reduced inter-animal variability was observed in fed dogs; as such, the data indicated that the tablets should be administered with food in clinical trials.

CYP3A4 is the main isoform contributing to vepdegestrant CYP metabolism, metabolism (accounting for 85% of CYP metabolism).

2.2.3.2. Potential Drug- Drug Interaction

Drugs that are inhibitors of CYP3A4 may increase the exposure of vepdegestrant, whereas drugs that are inducers of CYP3A4 may reduce the exposure of vepdegestrant.

In vitro, vepdegestrant is an inhibitor of CYP2B6 with an IC_{50} of 16.0 µM. However, subsequent mechanistic static modeling, using bupropion as the probe CYP2B6 substrate drug, indicated a low potential for DDI due to vepdegestrant-mediated reversible inhibition of CYP2B6 at human exposures associated with the 500 mg QD dose. Since bupropion is the only known sensitive CYP2B6 substrate, participants enrolled in clinical trials are allowed to receive CYP2B6 substrates or CYP2B6 substrates with narrow therapeutic indices.

vepdegestrant showed weak inhibition of human BCRP and is considered an in vitro inhibitor of P-gp. Based on the lack of significant safety issues when a sensitive BCRP substrate, rosuvastatin, was administered together with vepdegestrant in study ARV-471-mBC-101, participants enrolled in clinical trials will be allowed to receive sensitive BCRP substrates or BCRP substrates with narrow therapeutic indices (vepdegestrant [PF-07850327, ARV-471] Investigator's Brochure). Clinical DDI studies with P-gp substrates have not yet been conducted. Studies conducted in fasted dogs and rats showed a pH effect on absorption when administered with famotidine (lowered PK at neutral pH). Elevated gastric pH may reduce vepdegestrant absorption. Effect of PPI on vepdegestrant in healthy participants was evaluated in Study ARV-471-CPhm-103, demonstrating that vepdegestrant AUC values were similar when administered with PPI or without PPI, although PPI decreased median vepdegestrant Cmax about 18% which is not considered clinically meaningful. This indicates that there are no effects of PPI on exposure of vepdegestrant when administered with a moderate fat meal. However, these results are considered preliminary and the concomitant use of PPIs with vepdegestrant is not recommended. If PPI treatment is required, vepdegestrant intake with a moderate-fat meal (400-800 calories, approximately 35% fat) is recommended. Administer vepdegestrant ≥2 hours before or after antacids. Administer vepdegestrant ≥2 hours before or 10-12 hours after H2 receptor antagonist ARV-471-mBC-101 FIH study.

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2.2.3.3. Preliminary Safety, Efficacy and Pharmacokinetic Data in Humans

In the ongoing ARV-471-mBC-101 FIH study, vepdegestrant is being assessed as a monotherapy and coadministered with palbociclib in patients with ER(+)/HER2(-) aBC who had previously received CDK4/6i and/or AIs or SERD/ER antagonists and/or CT in the locally advanced/metastatic setting. The FIH study has 3 parts: Part A is a monotherapy dose escalation, Part B is a monotherapy dose expansion (200 mg, 500 mg, oral daily dosing), and Part C is evaluating the combination of vepdegestrant and palbociclib (vepdegestrant at 180, 200, 400 and 500 mg oral daily dosing plus palbociclib at 125 mg oral daily for 3 consecutive weeks followed by 1 week off treatment). As of 06 June 2022, 176 participants have been treated in the FIH study (Part A, n=78; Part B, n=71; and Part C, n=27) (vepdegestrant [PF-07850327, ARV-471] Investigator's Brochure). As of 06 June 2022, enrollment into Part C was still ongoing.

As of 06 June 2022, vepdegestrant monotherapy (Parts A and B) was well-tolerated across total daily doses of 30 mg to 700 mg in participants with mBC, with no DLTs observed and most TRAEs were Grade 1 or 2 (vepdegestrant [PF-07850327, ARV-471] Investigator's Brochure).

In **Part A** (monotherapy dose escalation), a total of 71 out of the 78 participants (91.0%) reported at least 1 TEAE. The most common TEAEs (regardless of attribution to study drug) observed in ≥10% of participants were: fatigue (33%), nausea (33%), constipation (31%), arthralgia (18%), vomiting (18%), back pain (17%), diarrhea (17%), headache (17%), decreased appetite (15%), AST increase (14%), hot flush (14%), hyperglycemia (13%), pain in extremity (13%), insomnia (11%), ALT increased (10%), and dizziness (10%). Most of the participants experienced Grade 1 or 2 TEAEs (74.3%), with Grade ≥3 occurring in 16.6% of participants. The Grade 3 TEAEs were hypertension (2 participants) and nausea, stomatitis, arthralgia, fatigue, aspartate aminotransferase increased, electrocardiogram QT prolonged, amylase increased, blood alkaline phosphatase increased, lipase increased, headache, presyncope, embolism, embolism venous, procedural pain, humerus fracture, acute kidney injury, and malignant pleural effusion (1 participant each). The Grade 4 TEAEs of hypoglycemia, neutrophil count decreased, and neutropenia occurred in 1 participant each. The Grade 5 TEAE of cardiac arrest occurred in 1 participant and was unrelated to vepdegestrant. A total of 54 participants (69%) had at least 1 TEAE considered potentially related to vepdegestrant. The most frequently (≥10% participants) reported TRAEs (all grades) were nausea (26%), fatigue (22%), constipation (13%), arthralgia (10%), decreased appetite (10%), hot flush (10%), and vomiting (10%). Most participants experienced Grade 1 or 2 TRAEs (64.1%). Grade 3 TRAEs (5.1%) were headache asymptomatic, amylase and lipase increased, nausea, asymptomatic electrocardiogram QT prolonged, and venous embolism. There were no Grade 4 or Grade 5 TRAEs.

The safety profile from Part B (monotherapy dose expansion) is consistent with what has been observed in Part A (monotherapy dose escalation).

In **Part B**, a total of 62 of the 71 participants (87.3%) reported at least 1 TEAE. The most commonly reported TEAEs observed in ≥10% of participants include fatigue (39%), nausea

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(21%), constipation (17%), arthralgia (17%), AST increased (13%), back pain (11%), decreased appetite (11%), and headache (11%). Most participants experienced Grade 1 or 2 TEAEs (64.8%), with Grade ≥3 TEAEs occurring in 22.5% of participants. The Grade 3 TEAEs were anemia (2 participants) and intestinal obstruction, muscular weakness, aspartate aminotransferase increased, alanine aminotransferase increased, blood alkaline phosphatase increased, electrocardiogram QT prolonged, brain oedema, seizure, syncope, thrombocytopenia, cholangitis, neutropenia, spinal cord compression, upper gastrointestinal haemorrhage, fatigue, pyrexia, back pain, and decreased appetite (1 participant each). The Grade 4 TEAEs of hyperbilirubinaemia, hypercalcaemia and hepatic haemorrhage were reported in 1 participant each. A Grade 5 TEAE of acute respiratory failure was reported in 1 participant, after that participant presented with progressive malignant pleural effusion. A total of 51 participants (71.8%) experienced at least 1 TRAE. The most common TRAEs, observed in ≥10% of participants, include fatigue (34%), nausea (17%), and arthralgia (13%). Most participants experienced Grade 1 or 2 TRAEs (64.8%). Grade 3 events (7.0%) of electrocardiogram QT prolonged, thrombocytopenia, neutropenia, decreased appetite, and fatigue were experienced by 1 participant each. There was one Grade 4 TRAE of hyperbilirubinaemia. There were no Grade 5 TRAEs reported.

Data from the monotherapy dose escalation (Part A) and dose expansion (Part B) at vepdegestrant 200 mg and 500 mg QD support the monotherapy dose selection of vepdegestrant 200 mg QD as RP3D.

Based on preliminary safety data from participants treated with vepdegestrant in combination with palbociclib (**Part C**), the combination is tolerable. The safety profile in Part C is generally consistent with that observed with vepdegestrant monotherapy (Parts A and B) and historical palbociclib studies in combination with endocrine therapy except for Grade 4 neutropenia, an AE associated with use of palbociclib that appears to be higher than historical data of palbociclib in combination with endocrine therapy (IBRANCE [palbociclib] USPI).

As of 01 November 2022, a total of 46 participants were treated with vepdegestrant in combination with palbociclib in the vepdegestrant-mBC-101 study (**Part C)**. In Part C, all participants were treated per protocol with an approved starting dose of palbociclib 125 mg, irrespective of their ability to tolerate previous full doses of CDK4/6i therapy. vepdegestrant oral daily doses tested in combination with palbociclib were as follows: 180 mg (n=2), 200 mg (n=21), 400 mg (n=3), and 500 mg (n=20). The majority of participants enrolled in Part C received prior CDK4/6i (78.3%, with 30.4% of them who received more than 1 line of prior CDK4/6i) and most received prior chemotherapy (69.6%) as well as prior hormonal therapy (91.3%). Overall, participants received a median of 4.0 prior regimens of treatment respectively. In the metastatic setting the participants received a median of 3 prior regimens of treatment.

The safety profile of the combination of vepdegestrant 200 mg and palbociclib 125 mg is generally consistent with that observed after treatment with vepdegestrant monotherapy (Part A dose escalation and Part B dose expansion) and historical palbociclib studies with the exception of increased hematologic toxicity as compared with that reported in previous

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palbociclib combination studies (mostly treatment-naïve or only prior endocrine treatment in metastatic setting). Moreover, the safety profile of vepdegestrant in combination with palbociclib was similar across vepdegestrant doses tested. Because vepdegestrant 200 mg QD is the RP3D selected for monotherapy as well as in combination with palbociclib, the data reported herein focus on the safety data from participants treated with vepdegestrant 200 mg QD in combination with palbociclib.

The median (min-max) durations of exposure of vepdegestrant 200 mg QD in combination with palbociclib was 14.14 (4.1 - 72.1) weeks.

TEAEs of vepdegestrant 200 mg in Combination with Palbociclib 125 mg (n=21)

A total of 20 of the 21 participants (95.2%) treated with vepdegestrant 200 mg + palbociclib, reported at least 1 TEAE. The most commonly reported TEAEs (regardless of attribution to study drug) observed in ≥15% of participants included neutrophil count decreased/neutropenia (61.9% and 23.8% respectively), fatigue (66.7%), platelet count decreased (47.6%), constipation (28.6%), anaemia, white blood cell count decreased and arthralgia (23.8% each) and electrocardiogram QT prolonged (19.0%). The TEAEs Grade ≥3 severity was experienced by 85.7% of participants.

Incidence of dose reduction and interruption of vepdegestrant or palbociclib were numerically lower for participants treated at the vepdegestrant 200 mg QD compared to the higher vepdegestrant doses tested. There were no discontinuations of vepdegestrant due to vepdegestrant-related TEAEs at the vepdegestrant 200 mg QD in combination with palbociclib. At the vepdegestrant 200 mg QD, one participant (4.8%) discontinued both drugs due to Grade 1 dizziness deemed not related to either vepdegestrant or palbociclib.

A lower incidence of SAEs was reported in participants treated with vepdegestrant 200 mg dose (9.5%) compared with higher vepdegestrant doses tested. All SAEs were assessed by the investigator as not related to vepdegestrant. No SAE related to palbociclib was reported in participants treated with vepdegestrant 200 mg QD.

As expected, neutrophil count decrease/neutropenia were the most frequently reported TEAEs with a higher rate of Grade 4 neutropenia compared to historical palbociclib combination studies. Consistent with historical data, neutropenia observed in Part C was deemed medically manageable through dose interruption/dose reduction and/or supportive care and resolved within the cycle of occurrence. At vepdegestrant 200 mg QD the median time to first neutropenia of any Grade was similar to historical palbociclib studies: they occurred in the first 2 treatment cycles, with a median time to onset of 14.5 days (range: 2 to 29 days). No discontinuations due to neutropenic events were reported at the vepdegestrant 200 mg dose. Dose reductions and interruptions were also numerically lower at vepdegestrant 200 mg dose compared to the higher vepdegestrant doses tested supporting the selection of vepdegestrant 200 mg dose to be used in combination with palbociclib in clinical trials. There were no episodes of febrile neutropenia or neutropenic sepsis, and no Grade 5 events were observed at vepdegestrant 200 mg QD (Arvinas, 2023).

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Based on the categorical analysis of ECG data from 149 participants in ARV-471-mBC-101 study Part A and Part B (data cutoff date of 06 June 2022), 3 participants experienced a QTcF >500 msec (1 of each at 180 mg, 200 mg, and 500 mg daily dose) and 4 participants had QTcF change from baseline >60 msec (1 at 200 mg, 2 at 500 mg and 1 at 700 mg daily dose). From the ongoing ARV-471-mBC-101 study, as of 06 June 2022, 13 participants reported TEAEs of electrocardiogram QT prolonged (12 participants had TRAEs and in 1 participant the AE was considered as not related to vepdegestrant per investigator). Most participants experienced grade 1 or 2 QT prolonged events (11 out of 13), 2 participants reported Grade 3 events and no Grade 4/5 have been reported. No QT prolonged events were reported as serious. An external cardiology consultant review that included evaluation of the participant study treatment, laboratory and medical history as well as study ECGs, concluded that of the 13 participants who reported QTc prolonged events, only 2 participants had a true diagnosis of QTc prolongation. Of these low potassium and concomitant medications were confounding factors.

As of 01 November 2022, 10 out of 46 participants (21.7%) in Part C reported non-serious events of electrocardiogram QT prolonged. There was a lower percentage of electrocardiogram QT prolonged events in the vepdegestrant 200 mg cohort compared with the vepdegestrant higher doses. Four out of 21 (19.0%) participants reported electrocardiogram QT prolonged at vepdegestrant 200 mg dose in combination with palbociclib. All events were Grade 1 and did not require any dose reduction, interruption or treatment discontinuation of either vepdegestrant or palbociclib due to this AE and all participants remain on study treatments. An external cardiology consultant review, including evaluation of the participant study treatment, laboratory, concomitant medications, medical history as well as study ECGs, concluded that all the 10 participants had events that were either invalid diagnoses based on a recalculation of QTcF, not treatment-emergent, or likely due to concomitant medications. None of the 4 patients in ARV-200 mg cohort who reported Grade 1 electrocardiogram QT prolonged met criteria for QT prolongation as per external cardiology review. In summary, the totality of updated clinical safety data and external cardiology review conclude that the combinational treatment with palbociclib does not change the current risk assessment of electrocardiogram QT prolongation with vepdegestrant (Arvinas, 2023).

There have been 17 deaths reported in the study: 11 in Part A and 6 in Part B. None of these deaths were attributable to vepdegestrant. As of 01 November 2022, 1 death has been reported in Part C and was considered related to disease under study or complications of the disease under study.

There was evidence of preliminary clinical activity in the Part A monotherapy dose escalation of ARV 471 mBC-101 study, with several participants achieving clinical benefit as defined by CBR and ORR as of 06 June 2022. In response-evaluable participants, CBR (ie, rate of confirmed CR or confirmed PR or SD≥24 weeks) was 36.2% (95% CI: 25.0-48.7) and ORR was 8.6% (95% CI: 2.9-19.0), (vepdegestrant [PF-07850327, ARV-471] Investigator's Brochure). No clear dose-dependent or exposure-response relationship for CBR was observed, with exception of participants with ESR1 mutations showing a trend (not

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statistically significant) toward higher CBR with higher vepdegestrant exposure (Arvinas, 2022).

In Part B dose expansion, the CBR in the response evaluable participants of the overall population was 37.1% (95% CI: 21.5-55.1) and 38.9% (95% CI: 23.1-56.5) at 200 mg and 500 mg daily doses, respectively. The CBR in response-evaluable participants with ESR1 mutations, was similar between the 2 doses: 47.4% (95% CI: 24.4-71.1) at 200 mg QD and 54.5% (95% CI: 32.2-75.6) at 500 mg QD. No exposure-response relationship for CBR was seen in the overall population or the ESR1 mutant population (Arvinas, 2022).

Clinical benefit (by CBR) was observed in CDK4/6i-pretreated participants with ER(+)/HER2(-) BC in all Part A and B dose cohorts.

Based on a 01 November 2022 data cutoff date, in Part C evidence of preliminary clinical activity was observed in participants treated with vepdegestrant in combination with palbociclib across all the vepdegestrant doses. In participants treated with vepdegestrant 200 mg QD in combination with palbociclib, the CBR was 66.7% (95% CI: 9.4 - 99.2) in 2 of 3 evaluable participants and ORR was 14.3% (95% CI: 1.8 - 42.8) in 2 of 14 response-evaluable participants.

Clinical benefit (by CBR and ORR) was observed in CDK4/6i-pretreated participants with ER(+)/HER2(-) aBC. Efficacy data collection is still ongoing for Part C with 54.3% of participants still on treatment with the combination (Arvinas, 2023).

Preliminary PK data following single and multiple dosing from Part A monotherapy dose escalation of Study ARV-471-mBC-101 were available in mBC participants receiving vepdegestrant at total daily dose levels ranging from 30 mg to 700 mg (QD or BID) under fed conditions. The median T_{max} ranged from 4 to 7 hours across the dose levels. The mean effective $t_{1/2}$ at steady state ranged from 23 to 33 hours. Clinical exposure of vepdegestrant on Day 15 at 60 mg QD (geometric mean AUC_{tau} 7324 ng•h/mL) has exceeded the nonclinical efficacious exposure associated with TGI (AUC_{inf}=5717 ng•h/mL) at 30 mg/kg single dose in mice. Following 200 mg QD dosing (N=8), a geometric mean accumulation ratio for AUC_{tau} of 1.7 was observed between Day 1 and Day 15. vepdegestrant can interconvert to its epimer, ARV-473. Following 200 mg QD dosing (N=8), the ratio based on AUC_{tau} of ARV-473/vepdegestrant on Cycle 1 Day 15 is 32%. Nonclinical data showed vepdegestrant has both antagonist and degradation activities against ER, whereas ARV-473 displays only antagonist activity (vepdegestrant [PF-07850327, ARV-471] Investigator's Brochure).

Safety and PK data in healthy volunteers is based on the Phase 1 clinical pharmacology study, Study ARV-471-CPhm-103, which is evaluating the effect of food or a PPI (esomeprazole) and the relative bioavailability of different tablet formulations on the single-dose PK and safety of vepdegestrant in healthy postmenopausal female participants. As of data cut-off date of 16 June 2022, a total of 47 healthy participants have been treated in Study ARV-471-CPhm-103 (14 participants in FE, 17 participants in PPI, and 16 participants in rBA cohorts). Each participant received 2 doses of 200 mg vepdegestrant, 1 in each of the 2

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periods, and the vepdegestrant treatments were separated by a washout period of at least 14 days. Preliminary analysis results showed the median T_{max} ranged from 6.0 to 8.0 hours across the cohorts. The geometric mean $t_{1/2}$ following a single 200 mg dose was about 40 hours under fed condition. Food intake significantly increased ARV 471 C_{max} and AUC_{inf} 3 to 2 fold, respectively, as compared with fasted conditions. Thus, patients should be instructed to take vepdegestrant with food. vepdegestrant AUC values were similar when administered with PPI or without PPI when given with a moderate-fat meal, although PPI decreased median vepdegestrant C_{max} about 18% which is not considered clinical meaningful.

Preliminary results from Part C indicated that vepdegestrant exposure (C_{max} and AUC_{tau}) on Cycle 1 Day 1 and Cycle 1 Day 15 increased in a dose-dependent manner from vepdegestrant 200 mg QD to vepdegestrant higher doses in combination with palbociclib (data cutoff date as of 01 November 2022), consistent with results in Part A monotherapy (data cutoff date as of 06 June 2022). vepdegestrant exposure was also comparable when given in combination with palbociclib and as monotherapy in Part A. The differences in steady state C_{max} and AUC_{tau} between vepdegestrant monotherapy (Part A) and vepdegestrant in combination with palbociclib (Part C) were less than 20%.

Palbociclib exposure of C_{max} and AUC_{tau} was observed to be approximately 45% and 65% higher in Part C when IBRANCE commercial capsules were given in combination with vepdegestrant 200 mg under fed conditions, compared with exposure reported from PALOMA-1 (isethionate capsule formulation) and PALOMA-2 (free base capsule formulation) when obtained under fasted or "minimally fasted" conditions. Geometric mean C_{trough} of steady state palbociclib in Part C was higher than the geometric mean steady-state C_{trough} observed but individual steady-state C_{trough} values were within the distribution range of palbociclib steady state C_{trough} in historical palbociclib studies observed in PALOMA 1, PALOMA 2 and 2 early phase palbociclib studies (Arvinas, 2023).

Analysis of 14 paired biopsies from patients treated in Part A (monotherapy dose escalation) at various doses, with vepdegestrant monotherapy suggest robust ER degradation (up to 89% decrease, with a median ER decrease of 67% [range: 21% - 89%]) across all doses up to 500 mg QD, regardless of ESR1 mutation status with no apparent relationship between dose or exposure and ER degradation in patients with wild-type or mutant ER (Hamilton et al, 2022).

The totality of data from these nonclinical pharmacology, PK and metabolism, and toxicology studies and available safety and efficacy clinical data in humans support continued clinical development of vepdegestrant in ER(+)/HER2(-) aBC as monotherapy and in combination with palbociclib. Data from the Part C of the ARV-471-mBC-101 FIH study support the selection of vepdegestrant 200 mg QD as RP3D in combination with palbociclib. Despite increased palbociclib exposure and some increase in the incidence of high grade neutropenia relative to historical palbociclib combination study data, the increase of neutropenia was transient, did not result in an increase in clinical complications, and was manageable when using dose modification approaches per palbociclib prescribing information.

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As of the 06 June 2023 data cutoff date, the safety profile of vepdegestrant as monotherapy and in combination with palbociclib has remained unchanged compared to the previous 2022 cutoff dates with no new significant findings (vepdegestrant [PF-07850327, ARV-471] Investigator's Brochure). The benefit/risk of vepdegestrant as single agent or in combination with palbociclib remains unchanged.

2.3. Benefit/Risk Assessment

More detailed information about the known and expected benefits and risks and reasonably expected AEs of vepdegestrant may be found in the IB (vepdegestrant [PF-07850327, ARV-471] Investigator's Brochure), which is the SRSD for this study.

More detailed information about the known and expected benefits and risks and reasonably expected AEs of palbociclib and letrozole, are provided in the palbociclib IB and letrozole UK SmPC, which are the SRSDs for this study (see Palbociclib Investigator's Brochure and (Femara [letrozole] UK SmPC).

Potential risks of clinical significance known from nonclinical and toxicological studies, preliminary clinical findings of vepdegestrant, palbociclib and letrozole as well as risks related to study procedures are summarized in Section 2.3.1.

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2.3.1. Risk Assessment

Potential Risk of Clinical Significance	Summary of Data/Rationale for Risk	Mitigation Strategy
Study Intervention: vepdegestrant		
QTc prolongation	Potential risk based on nonclinical findings. As of data cutoff date of 06 Jun 2025, 32 (10.3%) participants were reported with the AE of QT interval prolongation in the ongoing Phase 3 pivotal study (C4891001). This AE was considered related in 25 (8.0%) participants. Six participants reported Grade 3 AE, while the others were reported as Grade 1 and 2. A dedicated QTc substudy was conducted in a subgroup of participants (N=88) in the vepdegestrant arm to further characterize the effect of vepdegestrant on the corrected QT (QTc) interval. On the basis of a population modeling analysis that evaluated the relationship between vepdegestrant plasma concentrations and QTc, the mean QTcF change from baseline was 11.1 msec (upper limit of the 90% confidence interval, 13.7 msec) with vepdegestrant at a dose of 200 mg once daily. The upper limit of the 90% confidence interval of the model-predicted increase in QTcF was less than 20 msec for all tested scenarios, at the recommended vepdegestrant dose of 200 mg once daily.	Participants will be monitored for cardiac function through ECG monitoring at screening. Participants with clinically significant cardiac abnormalities as per EC #5 are excluded from study participation. During the study intervention period, ECG monitoring will also be performed as outlined in the SoA, in all participants of the SLI and in a subset of 100 participants in each arm in the Phase 3, for a better understanding of the safety profile of vepdegestrant in combination with palbociclib. In both SLI and Phase 3, all study participants will have additional ECG measurements as clinically indicated. Participants at high risk of QT prolongation are excluded (as per EC #5) from participation in the study. Concomitant drugs predisposing to Torsade de Pointes or QT interval prolongation are to be used with caution as outlined in Section 6.9.4 and Appendix 9. Study interventions will be interrupted, reduced, or permanently discontinued based on severity of the events occurred.

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Potential Risk of Clinical Significance	Summary of Data/Rationale for Risk	Mitigation Strategy
Venous Thromboembolism (VTE)	Potential risk based on study in metastatic cancer setting and known class effect with ET (SERM, SERD, AIs). One Grade 3 related serious VTE case with confounding factors of obesity, diabetes and immobility due to recent biopsy procedure, and a Grade 3 serious VE case assessed as unlikely related to vepdegestrant by the investigator, were reported in the FIH study ARV-471-mBC-101.	Participants with a history of symptomatic pulmonary embolism or other clinically significant episode of thromboembolic disease are excluded (as per EC #5) from participation into the study. Suspicion of VTE should be investigated per local standard practice (eg, D-Dimer, fibrinogen, imaging). Study interventions will be interrupted, reduced, or permanently discontinued based on severity of the events occurred.
Potential risk of drug-drug interactions (DDI)	Coadministration of vepdegestrant with palbociclib (weak time-dependent CYP3A inhibitor) may increase exposure of ARV 471, which potentially increase the risk of adverse reactions of vepdegestrant.	Blood samples for PK of vepdegestrant coadministered with palbociclib or alone will be collected as per SoA to evaluate if vepdegestrant exposure changes when administered with palbociclib. Participants will be monitored for safety throughout the study intervention period (see SoA).
Developmental and reproductive toxicities	Based on mechanism of action, the loss of estrogen signaling leads to alterations in the hormonal input and feedback loops that are necessary for the menstrual cycle (humans or estrus in animals) and for the maintenance of pregnancy. The summary of the non-clinical animal findings included: Developmental (Embryo-Fetal) Toxicities: In 1-month rat study, loss of ability to maintain pregnancy, fetal death observed. Reproductive Toxicities in animal studies: Adverse microscopic changes in female tissues that impact organ function; Leydig cell hyperplasia and prostate changes in males; Clear impact on female fertility, potential risk for male fertility.	Protocol inclusion criteria specify allowable menopausal status and mitigations. Pregnant participants and participants <18 years are excluded in the clinical study protocols. Risk minimization measures have been implemented, including 1) the requirement for LHRH agonist use for pre/perimenopausal women prior to and during treatment and 2) the requirement for ensuring highly effective contraceptive measures for IOCBP and for male participants with IOCBP partners.

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Potential Risk of Clinical Significance	Summary of Data/Rationale for Risk	Mitigation Strategy
	Vepdegestrant may cause fetal loss or reproductive abnormalities when administered to the pregnant woman or IOCBP. Clinical findings showed that no developmental or reproductive toxicities have been reported to date.	
Study Intervention: Palbociclib		
Bone marrow suppression	Bone marrow is the main target organ for palbociclib. Reversible and manageable neutropenia, leukopenia, thrombocytopenia, and anemia are identified risks. Since palbociclib has myelosuppressive properties, it may predispose to infections.	Adequate bone marrow function is required for study inclusion and participants will be monitored throughout the study intervention period (see SoA). Unplanned hematological assessment as clinically indicated. Study interventions will be interrupted, reduced, or permanently discontinued based on severity of the toxicity as outlined in Section 6.6. Participants with active infections are excluded (as per EC #13) from participation into the study. Moreover, participants will be monitored for signs and symptoms of infection and treated as medically appropriate (see Section 6.9.5).
Pneumonitis/Interstitial lung disease (ILD)	Pneumonitis/ILD has been reported in patients receiving palbociclib even if the causal association between palbociclib treatment and pneumonitis/ILD/could not be established.	Participants with active infections are excluded (as per see EC #13) from participation into the study. Participants will be monitored for signs and symptoms of pneumonitis/ILD. Suspected cases of pneumonitis/ILD will be identified with radiographic imaging and if appropriate participants will receive corticosteroids, supportive care, and antibiotics as applicable. Study interventions will be permanently discontinued in case of severe/life-threatening pneumonitis or ILD as outlined in Section 6.6.
Potential risk of drug-drug interactions (DDI)	Participants treated with palbociclib 125 mg QD in combination with vepdegestrant in the ongoing ARV-471-mBC-101 (Part C) had higher palbociclib exposure compared to	Blood samples for PK of palbociclib coadministered with vepdegestrant will be collected as per SoA. Participants will be monitored for safety throughout the study intervention period (see SoA).

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Potential Risk of Clinical Significance	Summary of Data/Rationale for Risk	Mitigation Strategy
	historical data of palbociclib in combination with endocrine therapy, as well as the finding of an increase in frequency of Grade 4 neutropenia. Coadministration of vepdegestrant with palbociclib may increase exposure of palbociclib, which potentially increase the risk of toxicities of palbociclib. The mechanism in increased exposure of palbociclib is currently unknown.	Lower doses of palbociclib (ie, 100 and 75 mg QD) in combination with vepdegestrant will be explored in this study.
Study Interventions: Letrozole (Phase 3 will not be conducted, therefore this risk is N/A)		
Bone density loss **Tendonitis/tendon rupture (rare)** **Cardiovascular events**	These risks are based on letrozole SRSD.	Adverse events and clinical laboratory results will be monitored on an ongoing basis and appropriate medication/measures will be administered based on the severity of the AE. Calcium level will be assessed at screening and during the study and supportive care (eg, bisphosphonate or RANKL inhibitors) will be administered as needed as outlined in Section 6.9.5.
Study Procedures		
Tumor biopsy requested for inclusion and EOT	Risks associated with any tumor biopsy include the following: • Pain from local anesthetic. • Pain/discomfort from biopsy procedure. • Bleeding, swelling, scarring, pain or bruising at the biopsy site. • Infection of wound. Biopsies of certain sites may increase chances of life-threatening complications, eg, pneumothorax during biopsy of lung.	All procedures will be completed by qualified and trained medical practitioners. Local anesthetic will be administered. Sterile technique will be used. Participants will not be subjected to a significant risk procedure to obtain biopsies. EOT biopsy (optional) will be collected only in participants who progressed and if clinically feasible.
Blood sample collections for safety assessments, pharmacokinetics, and biomarker	A blood draw may cause inflammation of the vein, pain, bruising, discomfort, redness, burning, or bleeding at the site where the	All blood draws will be completed by qualified and trained medical personnel.

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Potential Risk of Clinical Significance	Summary of Data/Rationale for Risk	Mitigation Strategy
	needle is placed to draw the blood. May cause participant to feel dizzy or faint. There is a slight chance of infection.	Sterile technique will be used. Blood sample collections will be conducted at facilities prepared for adverse reactions related to the collection.
Participants will have CT/MRI scans and bone scans during study participation	CT scans expose participants to a small dose of radiation. Contrast dye used for CT scans may cause pain or burning upon injection, may worsen kidney function in participants with kidney disease and may cause allergic reactions that could be severe and life-threatening. Radioactive material for bone scan may cause discomfort or burning upon injection.	Radiation exposure during the study is not expected to exceed what a participant would receive under standard of care (SoC) and should not create a significant risk to health. Participants with renal function impairment are excluded (as per EC #11) from participation into the study. CT scans will be conducted at facilities prepared for adverse reactions to the contrast dye. Hydration and/or premedication prior to contrast media administration or steroids administration in case of allergic reactions may to be instituted as per local standard practice. A bone scan exposes participants to a small dose of radiation that should not create a significant risk to health.
Other		
Decentralized/telehealth interactions study assessments may impact outcome assessments.	Decentralized study/telehealth study assessments may increase the chance of missing data. Vital signs and a physical examination will not be performed for telehealth interactions, which may increase the risk of not capturing clinically relevant information.	Training will be provided to all parties involved in study conduct on data acquisition tools, including those used in home health visits, and their role/responsibilities to ensure complete and correct data flow. Home health visits will be performed by a registered nurse/other qualified health care provider and the investigator or designee must be available for consultation. If required by local regulations, home health visits will be performed by the investigator or by a physician designee for specific assessments (eg, fulvestrant intramuscular administration, physical examinations, and collection of adverse events).

2.3.2. Benefit Assessment

In nonclinical studies, vepdegestrant demonstrated a potent (<1 nM) and robust (>90%) ER degradation in vitro, even in the presence of clinically relevant ER mutations (vepdegestrant [PF-07850327, ARV-471] Investigator's Brochure). vepdegestrant has demonstrated greater ER degradation compared to other novel SERDs in clinical development such as elacestrant (RAD1901) (Flanagan et al, 2018). In vivo, vepdegestrant inhibits tumor growth in multiple mouse xenograft models including ESR1 mutant patient-derived xenograft model as single agent or in combination with palbociclib. Moreover, in nonclinical studies, the combination of vepdegestrant with palbociclib resulted in significant tumor regression and anti-tumor activity when compared to palbociclib as a single agent or fulvestrant in combination with palbociclib (vepdegestrant [PF-07850327, ARV-471] Investigator's Brochure).

Preliminary clinical data from the ongoing FIH ARV-471-mBC-101 study demonstrated that vepdegestrant has antitumor activity as monotherapy with CBR of 36.2% (95% CI: 25.0 - 48.7) and ORR 8.6% (95% CI: 2.9 - 19.0) in Part A dose escalation and CBR of 37.1% (95% CI: 21.5 - 55.1) and 38.9% (95% CI: 23.1 - 56.5) at 200 mg and 500 mg QD, respectively, in Part B dose expansion. Clinical benefit (by CBR) was observed in CDK4/6i-pretreated patients with ER(+)/HER2(-) BC in all Part A and Part B dose cohorts. In this study, vepdegestrant has shown robust ER degradation (up to 89% decrease with a median decrease of 67% [range: 21% - 89%]) observed across all doses up to 500 mg daily, regardless of ESR1 mutation status (Hamilton et al, 2022).

Safety data from the ongoing FIH ARV-471-mBC-101 study demonstrated that vepdegestrant monotherapy is safe and well tolerated at all the tested doses. Based on a 01 November 2022 data cutoff date, evidence of preliminary clinical activity was observed in participants treated with vepdegestrant in combination with palbociclib across all the vepdegestrant doses in Part C. In participants treated with vepdegestrant 200 mg QD in combination with palbociclib, the CBR was 66.7% (95% CI: 9.4 - 99.2) in 2 of 3 evaluable participants and ORR was 14.3% (95% CI: 1.8 - 42.8) in 2 of 14 response-evaluable participants.

Clinical benefit (by CBR and ORR) was observed in CDK4/6i-pretreated participants with ER(+)/HER2(-) BC. Efficacy data collection is still ongoing for Part C with 54.3% of participants still on treatment. The safety profile in Part C is generally consistent with that observed with vepdegestrant monotherapy (Parts A and B) except for the incidence of Grade 4 neutropenia, that appear to be higher than historical data for palbociclib in combination with endocrine therapy. In heavily pre-treated participants (median of 3 prior lines in the metastatic setting), all neutropenic episodes occurred shortly after initiation of the treatment (first 2 cycles), were transient, did not result in an increase in clinical complications, and were manageable when using dose modification approaches and/or supportive care as per palbociclib prescribed information (Arvinas, 2023).

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2.3.3. Overall Benefit/Risk Conclusion

As of 06 June 2022, preliminary data from the ongoing FIH ARV-471-mBC-101 study suggested that vepdegestrant in combination with palbociclib has the potential to provide clinical benefit to patients with ER(+)/HER2(-) aBC. Based on the known safety profiles of vepdegestrant monotherapy and in combination with palbociclib and preliminary data from nonclinical studies and the ongoing ARV-471-mBC-101 study, the risk of clinically significant overlapping toxicities with this experimental combination is assessed as low. Preliminary clinical data from the ongoing FIH ARV-471-mBC-101 study (Parts A and B) demonstrated that vepdegestrant has antitumor activity: CBR of 36.2% (95% CI: 25.0 - 48.7) and ORR of 8.6% (95% CI: 2.9 - 19.0) in Part A dose escalation and CBR of 37.1% (95% CI: 21.5 - 55.1) and 38.9% (95% CI: 23.1 - 56.5) at 200 mg and 500 mg QD in Part B dose expansion were reported, respectively. In addition, clinical benefit (by CBR) was observed in participants with ER(+)/HER2(-) BC that had been previously exposed to CDK4/6i in all Part A and B dose cohorts (Arvinas, 2022).

As of the 01 November 2022 data cutoff, evidence of clinical activity was observed in heavily pre-treated participants (median of 3 lines of prior therapy in the metastatic setting) treated with vepdegestrant in combination with palbociclib (Part C). Efficacy in term of CBR and ORR, was similar across all the vepdegestrant doses. At vepdegestrant 200 mg QD, the CBR was 66.7% (95% CI: 9.4 - 99.2) in 2 of 3 evaluable participants and ORR was 14.3% (95% CI: 1.8 - 42.8) in 2 of 14 response-evaluable participants. Clinical benefit (by CBR and ORR) was observed in CDK4/6i-pretreated participants with ER(+)/HER2(-) BC.

The safety profile in Part C is generally consistent with that observed with vepdegestrant monotherapy (Parts A and B) except for Grade 4 neutropenia, an AE associated with use of palbociclib that appears to be higher than the historical data for palbociclib in combination with endocrine therapy. Neutropenia was medically manageable with or without supportive care and/or dose adjustments as per the palbociclib prescribing information. All neutropenia episodes were reversible and non-cumulative. The safety profiles at vepdegestrant 200 mg in combination with palbociclib was similar to that reported for the other vepdegestrant doses. However, vepdegestrant and palbociclib dose interruptions and dose reductions were numerically lower for participants treated at the vepdegestrant 200 mg dose compared to the vepdegestrant higher tested doses. There were no episodes of febrile neutropenia or neutropenic sepsis and no Grade 5 events observed due to neutropenia or complicated neutropenia at vepdegestrant 200 mg QD. No discontinuations of vepdegestrant due to vepdegestrant-related TEAEs were reported at vepdegestrant 200 mg QD (Arvinas, 2023).

In addition, vepdegestrant was active in nonclinical models harboring clinically relevant ESR1 mutations associated with resistance to ET, including superior TGI compared to fulvestrant in a Y537S estrogen receptor (ESR1) mutant patient-derived xenograft model. Moreover, in nonclinical studies, the combination of vepdegestrant with palbociclib resulted in significant tumor regression and anti-tumor activity when compared to palbociclib as a single agent or fulvestrant in combination with palbociclib.

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Thus, the combination of vepdegestrant with palbociclib may represent an important therapeutic option improving upon standard ET to benefit in participants with ER(+)/HER2(-) aBC by preventing, delaying, or overcoming therapeutic resistance, thereby prolonging PFS in the metastatic setting.

Considering the measures taken to minimize risks to study participants, the potential risks identified in this study for vepdegestrant coadministered with palbociclib are justified by the anticipated benefits that may be afforded to participants with ER(+)/HER2(-) aBC.

3. OBJECTIVES, ENDPOINTS, AND ESTIMANDS

Study Lead-in		
Objectives	**Endpoints**	**Estimands**
Primary		
• To determine the RP3D of palbociclib when given in combination with vepdegestrant	• Incidence of Grade 4 neutropenia AE (as graded by NCI CTCAE v5.0), with onset within the first 4 cycles. • Incidence of dose reduction or drug discontinuation within the first 4 cycles.	• See Section 9.1.1.
Secondary		
• To evaluate safety and tolerability of both dose levels.	• Incidence of AEs and SAEs • Incidence of laboratory abnormalities. • Incidence of ECG abnormalities.	• Not Applicable
• To evaluate measures of tumor control and duration of response in both dose levels.	• Objective Response (OR: confirmed CR or PR) by investigator assessment per RECIST v1.1. • Clinical Benefit Response (CBR: confirmed CR or PR at any time or SD or non-CR/non-PD ≥24 weeks) by investigator assessment per RECIST v1.1. • Duration of Response, defined as the time from the first documentation of CR or PR to the date of the first documentation of PD, as determined by investigator assessment per RECIST	• Not Applicable

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Study Lead-in		
Objectives	**Endpoints**	**Estimands**
	v1.1, or death due to any cause, whichever occurs first.	
• To evaluate the concentrations of vepdegestrant and ARV-473	• Plasma concentrations of vepdegestrant and ARV-473.	• Not Applicable
• To evaluate the concentrations of palbociclib.	• Plasma concentrations of palbociclib.	• Not Applicable
Exploratory		
• To evaluate correlation of biomarker with exposure of vepdegestrant in combination with palbociclib and other clinical outcomes	• Measurements of surrogate pharmacodynamic markers involved in cell proliferation (eg, TK1).	• Not Applicable

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No participants will be enrolled in the Phase 3 portion of the study.

Phase 3		
Objectives	**Endpoints**	**Estimands**
Primary		
• To demonstrate that vepdegestrant in combination with palbociclib is superior to letrozole plus palbociclib in prolonging progression free survival (PFS) in participants with ER(+)/HER2(-) aBC who have not received any prior systemic anticancer therapy for their advanced disease.	• Progression-free survival (PFS), defined as the time from the date of randomization to the date of first documented disease progression, as determined by BICR assessment per RECIST v1.1, or death due to any cause, whichever occurs first.	• See Section 9.1.2.1.
Key Secondary		
• To demonstrate that vepdegestrant in combination with palbociclib is superior to letrozole in combination with palbociclib, in prolonging overall survival (OS).	• Overall Survival (OS), defined as the time from the date of randomization to the date of death due to any cause.	• See Section 9.1.2.2.
Secondary		
• To compare measures of tumor control between treatment arms and evaluate duration of response within each treatment arm.	• Objective Response (OR: confirmed CR or PR) by BICR assessment per RECIST v1.1. • Clinical Benefit Response (CBR: confirmed CR or PR at any time or SD or non-CR/non-PD ≥24 weeks) by BICR assessment per RECIST v 1.1. • Duration of Response, defined as the time from the first documentation of CR or PR to the date of the first documentation of PD, as determined by BICR	• Not Applicable

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Phase 3		
Objectives	**Endpoints**	**Estimands**
	assessment per RECIST v1.1, or death due to any cause, whichever occurs first.	
• To evaluate safety and tolerability of both treatment arms.	• Incidence of AEs and SAEs. • Incidence of laboratory abnormalities. • Incidence of ECG abnormalities.	• Not Applicable
• To evaluate the concentrations of vepdegestrant and ARV-473.	• Plasma concentrations of vepdegestrant and its epimer ARV-473.	• Not Applicable
• To evaluate the concentrations of palbociclib.	• Plasma concentrations of palbociclib.	• Not Applicable
• To evaluate patient reported outcomes between 2 treatment arms.	• EuroQol EQ-5D-5L • EORTC QLQ-C30 • EORTC QLQ-BR23	• Not Applicable
• To assess changes from baseline levels in plasma ctDNA.	• ctDNA plasma quantitative changes from pre-treatment to evaluate potential predictability of their associations with clinical outcomes.	• Not Applicable
Exploratory		
• To evaluate correlations of biomarkers with exposure of vepdegestrant in combination with palbociclib, efficacy, and other clinical outcomes.	• Tumor tissue and/or blood biomarkers, to determine gene mutations (eg, ESR1, PIK3CA), and cell proliferation and oncologic driven proteins (eg, Ki67) involved in the biology aBC, mechanisms of resistance of vepdegestrant in combination with palbociclib.	• Not Applicable

4. STUDY DESIGN

4.1. Overall Design

This is an international Phase 3 randomized, open-label, parallel-group, multicenter clinical trial aimed to demonstrate that vepdegestrant in combination with palbociclib is superior to letrozole in combination with palbociclib in prolonging the PFS by BICR assessment in

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female and male participants with ER(+)/HER2(-) aBC who have not received any prior systemic anti-cancer treatment for their advanced disease.

SLI Phase

This study will have a Study Lead-in (SLI) aimed to evaluate safety and tolerability in participants who have not received any prior systemic anti-cancer treatment for aBC, to select the Recommended Phase 3 Dose (RP3D) of palbociclib to be used in combination with vepdegestrant in Phase 3 study.

In the SLI, 2 dose levels will be explored in parallel. Approximately 50 participants will be randomly assigned on a 1:1 basis to:

- Dose Level 1 (n≈ 25). Participants will receive:
  - vepdegestrant 200 mg orally, once daily, continuously, in a 28-day cycle, *plus*
  - Palbociclib 100 mg orally, once daily for 21 consecutive days followed by 7 days off treatment in a 28-day cycle.
- Dose Level 2 (n≈ 25). Participants will receive:
  - vepdegestrant 200 mg orally, once daily, continuously, in a 28-day cycle, *plus*
  - Palbociclib 75 mg orally, once daily for 21 consecutive days followed by 7 days off treatment, in a 28-day cycle.

When the RP3D in the SLI has been selected the randomized Phase 3 will start.

SLI Phase enrollment has been completed for the planned participants. The SLI participants benefitting from treatment (defined as complete/partial response or stable disease as per RECIST v1.1) at the time of Amendment 3 implementation, will continue to receive Sponsor-supplied study intervention as part of the study per the 1.3.

Phase 3

The Phase 3 portion of the study will not be conducted, due to a Sponsor decision and not for safety or efficacy reasons.

In Phase 3, approximately 1130 participants will be randomly assigned on a 1:1 basis to:

- Arm A (Investigational Arm; n ≈ 565): Participants will receive:
  - vepdegestrant 200 mg orally, once daily, continuously, in a 28-day cycle, *plus*
  - Palbociclib per RP3D defined in the SLI orally, once daily for 21 consecutive days followed by 7 days off treatment in a 28-day cycle.

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- Arm B (Comparator Arm; n ≈565): Participants will receive:
    - Letrozole 2.5 mg orally, once daily, continuously, in a 28-day cycle, *plus*
    - Palbociclib 125 mg orally, once daily for 21 consecutive days followed by 7 days off treatment, in a 28-day cycle.

All the study interventions must be taken with food as outlined in Section 6.1.1.

Information on vepdegestrant and palbociclib doses and administration is provided in Table 3 and Table 5 for SLI and Phase 3, respectively and Sections 6.1.1.1 and 6.1.1.2. Details on dose modifications for toxicity for vepdegestrant and palbociclib are provided in Section 6.6.1.3.

Information on letrozole dose and administration is provided in Section 6.1.1.3. No dose reduction of letrozole is permitted.

In Phase 3, participants will be stratified by menopausal status at study entry: (pre-/peri-[including men] vs post-menopausal), visceral disease: (yes/no), and de novo metastatic disease: (yes/no). Visceral disease refers to lung, liver, brain, pleural and peritoneal involvement.

Pre-/peri-menopausal female participants and male participants must receive therapy with an LHRH agonist starting at Cycle 1 Day 1 (if not already ongoing) and continue during the study as outlined in the SoA and in Section 6.9.1.

Participants will continue to receive assigned study intervention until objective disease progression by RECIST v1.1, unacceptable toxicity, death, participant refused further treatment, or one of the other reasons for treatment discontinuation as outlined in Section 7.1 is met. However, participants may continue treatment in the arm assigned at randomization beyond the time of RECIST-defined PD at the discretion of the investigator if that is considered to be in the best interest of the participant and as long as no new anticancer treatment is initiated (see Section 8.2.2). In this case, the participant should continue with routine safety and efficacy assessments as per the SoA for the Active Treatment Phase.

Participants in the SLI will not be included into the Phase 3. Crossover between treatment arms is NOT allowed.

Participants will undergo efficacy assessments as outlined in the SoA (and detailed in Section 8.2.1). In SLI, efficacy analyses will be performed using the local radiologist's/investigator's tumor assessments as the primary data source. In Phase 3, efficacy analyses will be performed using a BICR laboratory as the primary data source. BICR will complete the review as per Section 8.2.3. In Phase 3, all radiographic images for all participants, as well as other information as per Section 8.2.3 collected on-study will be verified by BICR as described in the Study Imaging Manual.

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Safety will be monitored at regular intervals through the study by laboratory tests and clinical visits as described in the SoA.

Blood samples for pharmacokinetics analysis of vepdegestrant, its epimer ARV-473 and palbociclib will be collected in SLI and Phase 3. For additional details on the PK sample collections, refer to the SoA and Section 8.5.

PROs assessments (Phase 3 only) will be administered to evaluate the impact on quality of life, functioning, symptoms, and general health status using the EQ-5D-5L and EORTC QoL questionnaires (QLQ-C30 and QLQ-BR23 [female and male questionnaire versions]) (see Section 8.9, Appendix 13, Section 10.13.1).

A blood sample for DNA extraction (ie, Retained Research Sample for Genetics) will be collected as outline in the SoA, unless prohibited by local regulations, IRBs/ECs.

In SLI, serial blood samples for the evaluation of surrogate pharmacodynamic markers involved in cell proliferation (eg, TK1) will be collected as outlined in the SoA and Section 8.5.

In Phase 3, serial ctDNA blood and tumor samples will be collected as outline in the SoA. Molecular profiling will be used to assess the relationship between breast tumor sensitivity and resistance to vepdegestrant-related genes and proteins.

An independent E-DMC will be established to review safety data, to monitor safety and tolerability of vepdegestrant in combination with palbociclib, as well as interim analysis data of Phase 3. Data collected in the SLI will be provided to the E-DMC. The roles and responsibilities of the E-DMC will be described in a separate charter.

A Global Steering Committee will be established at the vepdegestrant program level. It will be responsible for providing objective strategic scientific advice and medical input on the ongoing and future clinical development plans for vepdegestrant in ER(+) breast cancer including for this study.

Participants discontinuing the Active Treatment Phase will enter a follow-up period (see Section 7.1.1). After the completion of the Safety FU as per Section 7.1.1.1, participants in the SLI will enter the Long-Term FU in which the first FU anticancer therapy will be collected. Then the participants should be considered as completing the study (see Sections 7.1.1.2 and 7.2). Participant in the Phase 3 will enter the Survival FU (see Section 7.1.1.3) during which survival and information on administration of any new anticancer therapy will be collected every 3 months from the end of treatment date, until one of the reasons for discontinuation from the study is met as reported in Section 7.2.

It is anticipated that approximately 50 and 1130 participants will be randomized in SLI and Phase 3, respectively. In Phase 3, the final PFS analysis will occur at approximately 575 PFS events, and the final OS analysis will occur at approximately 473 OS events.

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4.2. Scientific Rationale for Study Design

The efficacy and the safety of CDK4/6i in combination with ET have been confirmed in multiple randomized studies supporting their use as the preferred frontline regimen for most participants with ER(+)/HER2(-) aBC (see Section 2.2.2). Despite the significant clinical benefit of ET alone or in combination with targeted agents, participants may eventually acquire ET resistance over the course of treatment. ET resistance remains a substantial barrier to disease remission/quiescence in aBC. Therefore, unmet need exists to improve upon standard ET to benefit participants with ER(+)/HER2(-) aBC by preventing, delaying, or overcoming therapeutic resistance, thus improving curative rates and prolonging PFS in the metastatic setting.

In nonclinical studies, vepdegestrant has demonstrated robust ER degradation, superior activity against clinically relevant ER mutated forms (eg, Y537S and D538G), and improved TGI as single agent compared to fulvestrant in ER wild type and ESR1 Y537S mutant patient-derived xenograft models as administered as single agent or in combination with palbociclib (See Section 2.2.3). In the FIH vepdegestrant-mBC-101 study, vepdegestrant administered orally, as single agent has been demonstrated to be safe and well tolerated across total daily doses of 30 to 700 mg with preliminary evidence of efficacy in participants pretreated with CDK4/6i plus ET.

Palbociclib, the first CDK4/6i registered by FDA in combination with ET has demonstrated improved PFS in participants with ER(+)/HER2(-) aBC in the 1L setting and as such is a logical partner to be developed in combination with vepdegestrant in this participant population.

This information as a whole provides support for the investigation of vepdegestrant in combination with palbociclib in participants with ER(+)/HER2(-) in aBC who have not received any prior systemic anticancer therapies for their advanced disease.

For further details, refer to the Sections 2.1, 2.2, and 2.3 of this protocol and the vepdegestrant IB (vepdegestrant [PF-07850327, ARV-471] Investigator's Brochure).

Rationale for Protocol Amendment 3

The main purpose of the amendment is to reduce the Schedule of Activities with clinical assessments per standard of care or clinical judgment of the treating physician. This amendment also reflects the previous decision that the Phase 3 portion of the study will not be conducted, due to a Sponsor decision and not for safety, efficacy reasons or requests from Agencies; SLI Phase enrollment was completed for the planned participants. In addition, this amendment is to update the end of study definition and the section regarding continued access to study intervention after the end of the study.

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4.2.1. Patient Input Into Design

Patient Insight surveys have been conducted in several countries via interviews through a third party. The team gathered qualitative insights to develop a deeper understanding of the breast cancer patient journey, uncover their views on clinical trials and options on certain aspects of the protocol assessments. No significant findings in the study design or type, or frequency of the assessments as per SoA preventing study participation, have been identified.

4.2.2. Diversity of Study Population

The diversity strategy will include high performing sites with the potential to support the recruitment of diverse and under-represented populations in the US. The strategy to ensure diversity includes recognition that the presentation of aBC varies by gender, age, race, and geographic location; inclusion of diverse principal investigators; site-level training on clinical study diversity; support sites in recruitment plans in order to randomize a more diverse patient population; customized diverse recruitment materials, advertising, and referrals to support their efforts; accelerating an education campaign to community oncologists, patient advocacy groups, and breast cancer social media sites to increase awareness about this clinical trial option.

4.2.3. Choice of Contraception/Barrier Requirements

vepdegestrant is known to cause reversible degenerative changes in the reproductive system organs of female rats and dogs and male dogs, which may be regarded as potential safety risks for women and men, and which are considered related to the pharmacologic activity of vepdegestrant. Therefore, there is a potential risk of male and female infertility. Studies to evaluate effect of the vepdegestrant in pregnant women have not been conducted. However, based on mechanism of action, vepdegestrant may cause fetal harm when administered to the pregnant woman. Therefore, the use of a highly effective method of contraception is required, if applicable as per above. (see Appendix 4).

There are no clinical data on fertility in humans and no adequate well-controlled studies using palbociclib in pregnant women. Because of the potential for genotoxicity, male participants with female partners of childbearing potential, are required to use adequate contraception. In addition, based on animal studies, palbociclib may impair fertility in males. Thus, men may consider sperm preservation prior to beginning therapy with palbociclib. Based on findings in animals and mechanism of action, palbociclib can cause fetal harm when administered to a pregnant woman. In animal studies, palbociclib was fetotoxic at maternally-toxic doses. Therefore, the use of a highly effective method of contraception in IOCBP is required, if applicable as per below (see Appendix 4).

Based on studies in animals showing reproductive toxicity and human experience in which there have been isolated cases of birth defects (labial fusion, ambiguous genitalia), letrozole may cause congenital malformations when administered during pregnancy. Therefore, the use of a highly effective method of contraception in IOCBP participants is required, if applicable as per below (see Appendix 4).

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During the study, IOCBP participants and male participants are required to use contraception.

Details on contraception requirements and guidance are outlined in Appendix 4.

4.2.4. Collection of Retained Research Samples

Retained Research Samples for genetics will be collected and stored, as allowed per local regulations, IRBs and ECs, for further analyses which may, for example, provide greater understanding of the study intervention. Retained Research Samples may be used for research related to the study intervention(s) and disease under study (see Section 8.6.2).

4.3. Justification for Dose

In the FIH ARV-471-mBC-101 study, vepdegestrant monotherapy (Parts A and B) was safe and well tolerated across total daily doses of 30 mg to 700 mg, without DLTs observed in the dose escalation. The 200 and 500 mg daily doses were selected as RP2Ds and are currently being evaluated in the Part B monotherapy dose expansion portion of ARV-471-mBC-101. Nonclinical data in MCF7 tumor-bearing mice showed dose-dependent tumor growth inhibition at doses of 3, 10 and 30 mg/kg with ≥94% ER degradation in tumors at all 3 doses (vepdegestrant [PF-07850327, ARV-471] Investigator's Brochure). This suggests that doses/exposures required for maximal anti-tumor activity are higher than those required for maximal ER degradation. Therefore, the nonclinical exposure threshold for maximal efficacy was targeted for RP2D dose selection. Based on the desire to maintain clinical exposures of vepdegestrant above this threshold in the majority of patients and the lack of notable differences in safety across the doses evaluated in the Part A monotherapy dose escalation, 200 mg QD was selected as the lower dose for the Part B dose expansion. The dose of 500 mg QD was selected as the higher dose option in the dose expansion because exposures had been observed to plateau above 500 mg.

vepdegestrant monotherapy demonstrates a dose-dependent increase in vepdegestrant exposure up to 500 mg with no notable differences in safety, ER degradation, or efficacy (as measured by CBR) observations across this dose range. An apparent higher risk for potential QTc prolongation was observed at 500 mg QD, supported by concentration QTc modelling and observed data from Parts A and B.

Data as of 06 June 2022 for vepdegestrant monotherapy (Parts A and B) demonstrates that:

- No meaningful differences in overall safety, such as TRAEs, AEs leading to discontinuations, or dose interruptions were noted, except for an apparent higher risk of potential QTc prolongation at 500 mg QD. Dose reductions occurred only at the 500 mg QD dose in Part B.
- No clear differences are observed in efficacy (by CBR) and no significant dose-response relationship is seen between the 200 mg and 500 mg daily doses in a heavily pretreated population.

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Based on the totality of biologic evidence including clinical efficacy, safety, PK , PD, and nonclinical data, vepdegestrant 200 mg QD dose was selected as the RP3D as monotherapy (Arvinas, 2022).

The palbociclib dose used in the Part C of ARV-471-mBC-101 study (ie, 125 mg QD) is the dose already approved in combination with letrozole and fulvestrant in 1L and 2L aBC, respectively, and confirmed to be well tolerated and efficacious.

As of the 01 November 2022 data cutoff date, 46 heavily pre-treated participants with ER(+)/HER2(-) BC were treated with a range of doses of vepdegestrant in combination with palbociclib at the approved 125 mg QD dose in Part C of the Phase 1 study, ARV-471-mBC-101. Twenty-one (21) of the 46 participants were treated at the vepdegestrant 200 mg QD dose.

Clinical benefit (by CBR and ORR) was observed in CDK4/6i-pretreated participants, with no significant dose-response relationship across the vepdegestrant doses tested.

The safety profile of vepdegestrant in combination with palbociclib (Part C) is generally consistent with that observed with vepdegestrant monotherapy (Parts A and B) except for Grade 4 neutropenia reported at a greater frequency than expected based on historical data of palbociclib in combination with endocrine therapy. All neutropenic events occurred within the first 2 cycles of treatment and resolved within the cycle of occurrence. Neutropenic events were medically manageable with dose modifications and supportive care as per palbociclib prescribing information. The safety profile of vepdegestrant 200 mg QD in combination with palbociclib was similar across all vepdegestrant doses. At vepdegestrant 200 mg QD, a numerically lower number of palbociclib dose modifications was observed compared to those reported at higher doses of vepdegestrant, with no permanent discontinuations of palbociclib due to palbociclib-related TEAEs (Arvinas, 2023).

Overall, vepdegestrant exposure (C_{max} and AUC_{tau}) was comparable when given in combination with palbociclib in part C and when given as monotherapy in part A. However, higher exposures of palbociclib were observed at steady-state when combined with vepdegestrant compared to exposure data obtained from studies of palbociclib when given alone or in combination with an aromatase inhibitor (IBRANCE [palbociclib] Product Monograph; Durairaj et al, 2018). The magnitude of change in palbociclib exposure (C_{max} and AUC_{tau}) was similar when given in combination with vepdegestrant 200 and vepdegestrant higher doses, suggesting that the observed increase is not related to vepdegestrant dose within this tested dose range.

In summary despite greater palbociclib exposures and an increase in the frequency and severity of neutropenia relative to previous palbociclib combination studies, the neutropenia was transient, did not result in an increase in clinical complications and was manageable with dose modification approaches per palbociclib prescribing information. Promising preliminary anti-tumor activity was observed in heavily pretreated participants in a high unmet need BC population.

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Based on the totality of previously submitted nonclinical data as well as updated safety, efficacy and PK data presented herein for vepdegestrant in combination with palbociclib (Part C of ARV-471-mBC-101), the vepdegestrant 200 mg QD was selected as the RP3D for Study C4891002. To better characterize the safety of vepdegestrant 200 mg QD in combination with palbociclib, a SLI will be conducted at 100 mg QD and 75 mg QD of palbociclib before starting the Phase 3.

The Phase 3 portion of the study will not be conducted.

4.3.1. SLI Observation Period

During the SLI, the observation period of up to 4 cycles will begin on Cycle 1 Day 1. Safety decisions will be considered, based on Grade 4 neutropenia (by NCI CTCAE v5.0), with onset within the first 4 cycles, dose reductions and drug discontinuations (see Section 9.1.1) occurring within the first 4 cycles.

4.3.2. RP3D definition

The RP3D is the dose level chosen for further investigation in Phase 3 based on the SLI results determined by the sponsor with input from investigators, the Steering Committee and after discussion with Health Authorities, as required. The RP3D will be determined based on the Grade 4 neutropenia observed during the SLI observation period as specified above, as well as the dose reductions and discontinuations (see details in Section 9.1.1). Additional data may be taken into account from the observation period, subsequent cycles or after the end of the SLI and will include safety, PK data and clinical activity. Once determined, the proposed RP3D will be communicated to sites participating in Phase 3.

4.4. End of Study Definition

The end of study is defined as the date of the last scheduled procedure shown in the 1.3 for the last participant in the trial globally.

A participant is considered to have completed the study if they have completed all periods of the study, or if criteria for study discontinuation have been met or at sponsor discretion if the data support ending the study.

For information on continuation of the study treatment after the end of study, refer to Section 6.7.

5. STUDY POPULATION

This study can fulfill its objectives only if appropriate participants are enrolled, including participants across diverse and representative racial and ethnic backgrounds. A prescreening tool for study recruitment purposes will include collection of information that reflects the enrollment of a diverse participant population including, where permitted under local regulations, age, sex, and race, and ethnicity. The following eligibility criteria are designed to select participants for whom participation in the study is considered appropriate. All relevant

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medical and nonmedical conditions should be taken into consideration when deciding whether a particular participant is suitable for this protocol.

Prospective approval of protocol deviations to recruitment and enrollment criteria, also known as protocol waivers or exemptions, is not permitted.

5.1. Inclusion Criteria

Participants are eligible to be included in the study only if all of the following criteria apply:

Inclusion Criteria
Age and Sex:
1. Participants aged 18 years or older (or the minimum age of consent in accordance with local regulations) at screening. a. Female participants under 60 years of age, with cessation of regular menses for 12 consecutive months and with no other alternative medical cause, must have a FSH level within the post-menopausal level, as per local laboratory reference range. b. Pre-/peri-menopausal female participants and male participants must agree to initiate or continue an LHRH agonist as per SoA and Section 6.9.1 c. IOCBP participants and male participants must agree to use contraception. Refer to Appendix 4 for further details.
Type of Participant and Disease Characteristics:
2. Participants who are willing and able, to comply with all scheduled visits, treatment plans, laboratory tests, lifestyle considerations, and other study procedures. a. **Phase 3 ONLY.** Participants must provide a blood sample AND a tumor sample collected at the time of diagnosis of locally advanced/metastatic disease. If not available, a de novo biopsy is required. The sole exception is those patients with bone only disease for whom archival tumor sample at initial diagnosis is acceptable. Refer to Section 8.7 for further details.
3. Histological or cytological confirmation of BC with evidence of locoregionally advanced or metastatic disease, not amenable to surgical resection or radiation therapy with curative intent. a. Documented ER(+), defined as ER(+) ≥1% stained cells by an assay consistent with local standards, on the most recent tumor biopsy, ie, at diagnosis of recurrence or metastatic disease (Allison et al, 2020). The sole exception is those patients with bone only disease for whom ER(+) using archival tissue at initial diagnosis is acceptable.

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Inclusion Criteria
b. Documented HER2(-) tumor by either IHC or in-situ hybridization per ASCO/CAP guideline (Wolff et al, 2018) on the most recent tumor biopsy, as per above. c. Participants who have bilateral BCs which are both ER(+)/HER2(-) are eligible.
4. No prior systemic anti-cancer therapy for their locoregionally advanced or metastatic disease.
5. At least 1 measurable lesion as defined by RECIST v1.1. Bone only disease: participants with only non-measurable lesions are eligible. Refer to Section 8.2.1 and Appendix 11 for further details.
Other Inclusion Criteria:
6. Eastern Cooperative Oncology Group (ECOG) performance status (PS) ≤2 (see Appendix 13 Section 10.13.2).

5.2. Exclusion Criteria

Participants are excluded from the study if any of the following criteria apply:

Exclusion Criteria
Medical Conditions:
1. Other medical (eg, hypersensitivity) or psychiatric condition including recent (within the past year) or active suicidal ideation/behavior or laboratory abnormality that may increase the risk of study participation or, in the investigator's judgment, make the participant inappropriate for the study.
2. Any other solid tumors within the past 3 years, except for the following: 1) adequately treated basal or squamous cell carcinoma of the skin; and 2) curatively treated *in situ* carcinoma of the cervix. For all other solid tumors, they must have been curatively treated and with no evidence of disease for >3 years. Participants with inflammatory BC, prior hematopoietic stem cell or bone marrow transplantation are excluded.
3. Participants with newly diagnosed brain metastasis or symptomatic central nervous system (CNS) metastases, carcinomatous meningitis, or leptomeningeal disease as indicated by clinical symptoms, cerebral edema, and/or progressive growth. Participants with a history of CNS metastases or cord compression are eligible if they have been definitively treated (eg, radiotherapy, stereotactic surgery), are clinically stable (including participants with residual CNS symptoms/deficits) and off enzyme-inducing anticonvulsants and steroids for at least 14 days prior to randomization.

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Exclusion Criteria
4. Participants in visceral crisis at risk of immediately life-threatening complications in the short term, including participants with massive uncontrolled effusions (pleural, pericardial & peritoneal), pulmonary lymphangitis, or liver involvement >50%.
5. Impaired cardiovascular function or clinically significant cardiovascular diseases, defined as: a. Symptomatic cardiac valve disease. Participants with mitral valve prolapse that is asymptomatic or not associated with clinically significant sequalae (eg, mitral regurgitation) are eligible. b. Any of the following in the previous 6 months: myocardial infarction, severe/unstable angina, coronary/peripheral artery bypass graft, symptomatic congestive heart failure (New York Heart Association Class III or IV), cerebrovascular accident, transient ischemic attack, or symptomatic pulmonary embolism or other clinically significant episode of thromboembolic disease, congenital long QT syndrome, Torsade de Pointes, clinical important arrhythmias, left anterior hemiblock (bifascicular block), ongoing cardiac dysrhythmias of NCI CTCAE Grade ≥2, atrial fibrillation of any grade. c. Cardiac rhythm device/pacemaker. Participants with cardiac rhythm device/pacemaker must be discussed in detail with sponsor medical monitor to determine eligibility. d. QTcF >470 msec on screening ECG.
Prior/Concomitant Therapy
6. Prior treatments: a. Primary endocrine resistance: relapse while on the first 2 years of standard adjuvant ET. b. Secondary endocrine resistance: o relapse while on standard adjuvant ET but after the first 2 years o relapse within 12 months of completing standard adjuvant ET c. CDK4/6i, vepdegestrant, fulvestrant, elacestrant and other investigational agents (including novel ET, any SERDs, SERCAs, CERANs) in any setting. d. Participation in other studies involving investigational drug(s) within 28 days prior to randomization. If in the FU Phase, the participant is eligible provided at least 5 half-lives have elapsed from the last dose.
7. Major surgery, radiotherapy, or systemic anticancer therapy within 14 days from randomization (28 days or 5 half-lives, whichever is shorter, for anticancer therapy containing an antibody-based agents, approved or investigational). Participants who received prior radiotherapy to ≥25% of bone marrow are not eligible independent of when it was received (see Appendix 12: Bone Marrow Reserve in Adults).

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Exclusion Criteria
8. Any unresolved toxicities from prior surgeries or therapies Grade >1 (Grade >2 for peripheral neuropathy) by NCI CTCAE v5.0 at the time of randomization, except for alopecia.
9. Concurrent administration of medications, food or herbal supplements that are strong inhibitors or strong inducers of CYP3A. Prior use of strong CYP3A inhibitors must be stopped 7 days before randomization and strong CYP3A inducers must be stopped 14 days before randomization. Refer to Section 6.9 (Prior and Concomitant Therapy) and Appendix 9 for the list of prohibited drugs.
Diagnostic Assessments:
10. Hematologic abnormalities defined as: a. Hemoglobin <9 g/dL (1 transfusion allowed ≤14 days from randomization); b. Absolute neutrophils count <1500/mm^3 or <1.5 x 10^9/L; c. Platelets <100,000/mm^3 or <100 x 10^9/L.
11. Renal impairment defined as an eGFR <45 mL/min/1.73m^2 calculated using the 2021 CKD-EPI equations, as outlined in Appendix 7.
12. Hepatic dysfunction defined as: a. Total bilirubin >1.5 x ULN (≥3 x ULN in case of documented Gilbert's syndrome); b. AST and ALT >3 x ULN (>5.0 x ULN in case of liver involvement by the tumor); c. ALP >2.5 x ULN (>5 x ULN in case of bone metastasis); d. aPTT >1.25 x ULN and INR >1.25 unless the participant is receiving anticoagulation, then aPTT and INR should be within the therapeutic range of the intended use.
13. Known active infection including SARS-CoV-2, HBV, HCV, and HIV or AIDS-related illness (screening for chronic conditions is not required).
14. Refractory nausea and vomiting, chronic gastrointestinal (GI) disease, GI ulcer, GI bleeding, inability to swallow the formulated product or previous significant gastric (total or partial) or bowel resection that would preclude adequate absorption of study interventions.
Other Exclusions:
15. Investigator site staff directly involved in the conduct of the study and their family members, site staff otherwise supervised by the investigator, and sponsor and sponsor delegate employees directly involved in the conduct of the study and their family members.

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5.3. Lifestyle Considerations

Restrictions regarding lifestyle for study eligibility and/or participation are reported in the Sections below. Food requirement for vepdegestrant, letrozole and palbociclib intake are detailed in Section 6.1.1.

5.3.1. Contraception

As applicable per Section 4.2.3, the investigator or their designee, in consultation with the participant, will confirm that the participant is using an appropriate method of contraception for the individual participant and their partner(s) from the permitted list of contraception methods (see Appendix 4) and will confirm that the participant has been instructed in its consistent and correct use. As applicable, at time points indicated in the SoA, the investigator or designee will inform the participant of the need to use highly effective contraception consistently and correctly and document the conversation and the participant's affirmation in the participant's chart. Participants need to affirm their consistent and correct use of at least 1 of the selected methods of contraception, considering that their risk for pregnancy may have changed since the last visit.

In addition, the investigator or designee will instruct the participant to call immediately if the selected contraception method is discontinued and document the requirement to use an alternate protocol-specified method, including if the participant will no longer use abstinence as the selected contraception method, or if pregnancy is known or suspected in the participant or partner.

5.3.2. Meals and Dietary Restrictions

Participants must be instructed to take study interventions with food as outlined in Section 6.1.1.

5.4. Screen Failures

Screen failures are defined as participants who consent to participate in the clinical study but are not subsequently randomized in the study. A minimal set of screen failure information is required to ensure transparent reporting of screen failure participants to meet the CONSORT publishing requirements and to respond to queries from regulatory authorities. Minimal information includes demography, screen failure details, and any SAE.

Individuals who do not meet the criteria for participation in this study (screen failure) may be rescreened only once. Rescreened participants must be assigned a new unique participant number that is different from the participant's number used for the initial screening.

6. STUDY INTERVENTION(S) AND CONCOMITANT THERAPY

Study interventions are all prespecified investigational and non-investigational medicinal products, medical devices, and other interventions (eg, surgical and behavioral) intended to be administered to the study participants during the study conduct.

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For the purposes of this protocol, study intervention refers to vepdegestrant and palbociclib (SLI) vepdegestrant and palbociclib (Arm A of Phase 3) and letrozole and palbociclib (Arm B of Phase 3).

The Phase 3 portion of the study will not be conducted.

6.1. Study Intervention(s) Administered

General information regarding study interventions administered in the SLI and Phase 3 are summarized in Table 3-Table 4 and Table 5-Table 6, respectively.

Table 3. SLI Study Interventions

Intervention Name	**ARV-471 (PF-07850327)**	**Palbociclib**
Arm Name	Dose Levels 1 & 2 (Investigational)	Dose Levels 1 & 2 (Investigational)
Type	Drug: PROteolysis Targeting Chimera	Drug: CDK 4/6 inhibitor
Dose Formulation	Tablet	Capsule
Unit Dose Strength(s)	100 mg	100 mg 75 mg 25 mg
Dosage Level(s)	200 mg	100 mg 75 mg 50 mg
Route of Administration	Oral	Oral
Use	Experimental	Experimental
IMP or NIMP/AxMP	IMP	IMP
Sourcing	Provided centrally by the sponsor. Please refer to the IP manual.	Provided centrally by the sponsor. Please refer to the IP manual.
Packaging and Labeling	Study intervention will be provided in a high-density polyethylene bottle with child-resistant cap. Each bottle will be labeled as required per local regulations.	Study intervention will be provided in a bottle with a safety screw cap. Each bottle will be labeled as required per local regulations.
Current/Former Names or Aliases	ARV-471 (PF-07850327) Vepdegestrant	Palbociclib, PD-332991, IBRANCE

Table 4. SLI Study Arms

Arm Title	Dose Level 1	Dose level 2
Arm Type	Investigational	Investigational

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Table 4. SLI Study Arms

Arm Description	Participants will receive: • ARV-471 as two 100 mg tablets (ie, 200 mg QD), orally, once daily, continuously, in a 28-day cycle, plus • Palbociclib as a 100 mg capsule, orally, once daily for 21 consecutive days followed by 7 days off treatment, in a 28-day cycle.	Participants will receive: • ARV-471as two 100 mg tablets (ie 200 mg QD), orally, once daily, continuously, in a 28-day cycle, plus • Palbociclib as a 75 mg capsule, orally, once daily for 21 consecutive days followed by 7 days off treatment, in a 28-day cycle.
Associated Intervention Labels	• ARV-471 (PF-07850327) • Palbociclib	• ARV-471 (PF-07850327) • Palbociclib

Table 5. Phase 3 Study Intervention (Phase 3 will not be conducted)

Intervention Name	vepdegestrant (PF-07850327)	Palbociclib	Letrozole
ARM Name	A (Investigational)	A & B (Investigational & Comparator)	B (Comparator)
Type	Drug: PROteolysis Targeting Chimera	Drug: CDK 4/6 inhibitor	Drug: Nonsteroidal aromatase inhibitor
Dose Formulation	Tablet	Capsule	Tablet
Unit Dose Strength(s)	100 mg	125 mg (Arm B only) 100 mg 75 mg 25 mg (Arm A only)	2.5 mg
Dosage Level(s)	200 mg See Section 6.1.1.1	as per RP3D defined in the SLI (Arm A only) 125 mg (Arm B only) See Section 6.1.1.2	2.5 mg See Section 6.1.1.3
Route of Administration	Oral	Oral	Oral
Use	Experimental	Experimental & Active comparator	Active comparator
IMP or NIMP	IMP	IMP	IMP
Sourcing	Provided centrally by the sponsor. Please refer to the IP manual.	Provided centrally by the sponsor. Please refer to the IP manual.	Provided centrally by the sponsor. Please refer to the IP manual.
Packaging and Labeling	Study intervention will be provided in a high-density polyethylene bottle with child-resistant cap. Each bottle will be labeled as required per local regulations.	Study intervention will be provided in a bottle with a safety screw cap. Each bottle will be labeled as required per local regulations.	Study intervention will be provided in a bottle with a safety screw cap. Each bottle will be labeled as required per local regulations.

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Table 5. Phase 3 Study Intervention (Phase 3 will not be conducted)

Current/Former Names or Aliases	ARV-471 (PF-07850327) Vepdegestrant	Palbociclib, PD-332991, IBRANCE	Letrozole

Table 6. Phase 3 Study Arms (Phase 3 will not be conducted)

Arm Title	A	B
Arm Type	Investigational	Comparator
Arm Description	Participants will receive: • vepdegestrant as two 100 mg tablets (ie, 200 mg QD), orally, once daily, continuously in 28-day cycle, plus • Palbociclib per RP3D defined in the SLI as capsule, orally, once daily for 21 consecutive days followed by 7 days off treatment in a 28-day cycle.	Participants will receive: • Letrozole as 2.5 mg tablet, orally, once daily in continuously in 28-day cycle, plus • Palbociclib as 125 mg capsule orally, once daily for 21 consecutive days followed by 7 days off treatment in a 28-day cycle.
Associated Intervention Labels	• ARV-471 (PF-07850327) • Palbociclib	• Letrozole • Palbociclib

Study interventions may be shipped by courier to study participants if permitted by local regulations and in accordance with storage and transportation requirements for the study interventions. Pfizer does not permit the shipment of study interventions by mail. The tracking record of shipments, including temperature monitoring data and the chain of custody of the study interventions must be kept in the participant's source documents/medical records.

6.1.1. Administration

In the SLI, participants will receive vepdegestrant plus palbociclib. vepdegestrant will be administered in tablet form while palbociclib will be administered in capsule form.

In the Phase 3, participants in Arm A will receive vepdegestrant plus palbociclib while participants in Arm B will receive letrozole plus palbociclib. vepdegestrant and letrozole will be administered in tablet form while palbociclib will be administered in capsule form.

Participants should receive study intervention within 3 days after randomization.

All study interventions will be self-administered outside the clinic, except on days of site visits. Participants must be instructed not to take study interventions at home and to withhold their daily doses until assessment as per SoA have been completed. On these days study interventions should be taken in the clinic under the supervision of the study site personnel.

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Site personnel must ensure that participants clearly understand the directions for self-medication. Participants will be given a sufficient supply of study intervention for administration on each dosing day until their next scheduled study visit. After implementation of Amendment 3, a sufficient supply of study intervention will be provided to the participants to cover an interval of every 2 cycles. Participants should be instructed to maintain the study interventions in the bottles provided throughout the course of dosing. Unused drug and/or empty bottles should be returned to the site at the next study visit. Returned unused study intervention MUST NOT be re-dispensed to a participant.

Participants must be instructed to take their medications with food. All the study interventions (particularly vepdegestrant and palbociclib) should be taken at approximately the same time, preferably in the morning. Participants must be instructed not to take more than the pre-specified dose at any time.

Participants will be given a dosing diary and should be instructed to record daily administration of the study interventions in the participant dosing diary.

Refer to the IPM for detailed administration instructions for all the study interventions administered in this study.

6.1.1.1. vepdegestrant

vepdegestrant will be administered as 100 mg tablets (Table 4 and Table 6). vepdegestrant will be administered orally with food (as reported in Section 6.1.1), once daily on Days 1-28 of each 28-day cycle according to the SoA.

Recommended dose modifications for toxicity related to study intervention are presented in Table 10.

6.1.1.2. Palbociclib

In the SLI, palbociclib will be administered as 100 mg (starting dose for Dose Level 1) or 75 mg (starting dose for Dose Level 2) capsules (Table 4) depending on random assignment.

In the Phase 3, palbociclib will be administered as capsules according to the RP3D defined in the SLI in the experimental arm and as a 125 mg capsule in the control arm (Table 6).

Palbociclib will be administered orally with food (as reported in Section 6.1.1), once daily on Days 1-21 of each 28-day cycle according to the SoA. Administration of palbociclib without food should be avoided.

In the event the Day 1 clinic visit of the subsequent cycle is scheduled during the Day -2 allowable visit time window (ie, Days 27 and 28), participants must be instructed to complete their 7-day off-treatment period of the current cycle prior to resuming the therapy even if

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criteria for treatment resumption are met at the visit. Cycle off-treatment periods of less than 7 days are considered protocol deviations.

In the event the Day 1 clinic visit of the subsequent cycle is scheduled during the Day +2 allowable visit time window (ie, next cycle Days 1 and 2), treatment can be restarted unless treatment delay is suggested for safety reason (see Section 6.6.3).

Recommended dose modifications for toxicity related to study intervention for palbociclib in combination with vepdegestrant are presented in Table 10.

6.1.1.3. Letrozole (Phase 3 Control Arm only)

The Phase 3 portion of the study will not be conducted.

Letrozole will be administered as 2.5 mg tablets (Table 5). Letrozole will be administered orally, with food (as reported in Section 6.1.1), once daily on Days 1-28 of each 28-day cycle according to the SoA.

Dose reduction for letrozole is NOT permitted.

6.2. Preparation, Handling, Storage and Accountability

1. The investigator or designee must confirm that appropriate conditions (eg, temperature) have been maintained during transit for all study interventions received and any discrepancies are reported and resolved before use of the study intervention.

2. Only participants randomized in the study may receive study intervention and only authorized site staff may supply study intervention.

3. All study interventions must be stored in a secure, environmentally controlled, and monitored (manual or automated recording) area in accordance with the labeled storage conditions with access limited to the investigator and authorized site staff. At a minimum, daily minimum and maximum temperatures for all site storage locations must be documented and available upon request. Data for nonworking days must indicate the minimum and maximum temperatures since previously documented upon return to business.

4. Any excursions from the study intervention label storage conditions should be reported to Pfizer upon discovery along with actions taken. The site should actively pursue options for returning the study intervention to labeled storage conditions as soon as possible. Once an excursion is identified, the study intervention must be quarantined and not used until Pfizer provides permission to use the study intervention. Specific details regarding the excursion definition and information to report for each excursion will be provided to the site in the IPM.

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5. Any storage conditions stated in the SRSD will be superseded by the storage conditions stated on the label. Site staff will instruct participants on the proper storage requirements for take-home study intervention.

6. Study interventions should be stored in their original containers.

7. The investigator, institution, head of the medical institution, (where applicable) or authorized site staff is responsible for study intervention accountability, reconciliation, and record maintenance (ie, receipt, reconciliation, and final disposition records), such as the IPAL or sponsor-approved equivalent. All study interventions will be accounted for using a study intervention accountability form/record. All study intervention that is taken home by the participant, both used and unused, must be returned to the investigator by the participant. **Returned study intervention must <u>not</u> be redispensed to the participants.**

8. Further guidance and information for the final disposition of unused study interventions are provided in the IPM. All destruction must be adequately documented. If destruction is authorized to take place at the investigator site, the investigator must ensure that the materials are destroyed in compliance with applicable environmental regulations, institutional policy, and any special instructions provided by Pfizer.

Upon identification of a product complaint, notify the sponsor within 1 business day of discovery as described in the IPM.

6.2.1. Preparation and Dispensing

A qualified staff member will dispense the study intervention using the IRT system via unique container numbers on the bottles and provided in sufficient quantities according to the dosing schedule outlined in the SoA. A second staff member will verify the dispensing.

6.3. Assignment to Study Intervention

This is an open-label study.

In the SLI, eligible participants will be randomized in a 1:1 ratio to one of the 2 dose levels, as reported in Table 4.

The Phase 3 portion of the study will not be conducted.

In the Phase 3, eligible participants will be randomized in a 1:1 ratio to receive either vepdegestrant plus palbociclib (Arm A) or letrozole plus palbociclib (Arm B) and stratified by:

- menopausal status at study entry (pre-/peri- vs post-). Men will be stratified into the pre-/peri-menopausal group.

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- visceral disease: (yes/no). Visceral disease refers to lung, liver, brain, pleural and peritoneal involvement.
- de novo metastatic disease (yes/no).

Stratification factors should be verified before randomization. This stratified randomization will be centrally allocated through the use of an IRT system (IWR).

Participants randomized cannot be de-randomized.

In both SLI and Phase 3, allocation of participants to treatment groups will proceed through the use of an IRT system. The site personnel (study coordinator or specified designee) will be required to enter or select information including but not limited to the user's ID and password, the protocol number, and the participant number. The site personnel will then be provided with a randomization number corresponding to the assigned treatment group, and DU or container number(s) for all study interventions when study intervention is being supplied via the IRT system. The IRT system will provide a confirmation report containing the participant number, randomization number, and DU or container number assigned. The confirmation report must be stored in the site's files.

Study treatment must start within 3 days after participant's randomization. Study intervention will be dispensed at the study visits summarized in the SoA.

The study-specific IRT reference manual and IPM will provide the contact information and further details on the use of the IRT system.

6.4. Blinding

Not Applicable

6.5. Study Intervention Compliance

When participants are dosed at the site, they will receive study intervention directly from the investigator or designee, under medical supervision. The date and time of each dose administered in the clinic will be recorded in the source documents and recorded in the CRF. The doses of study interventions and study participant identification will be confirmed at the time of dosing by a member of the study site staff other than the person administering the study interventions.

When participants self-administer study interventions at home, compliance with study interventions will be assessed on Day 1 of each cycle, prior to the dispensing of study interventions for the next cycle. After implementation of Amendment 3, compliance will be assessed every other cycle, prior to the dispensing of study interventions for the next two cycles (see SoA - after Implementation of Amendment 3 for details). Participants will receive a dosing diary to document self-administered dosing of study intervention in each cycle to include the dose of study intervention taken, the date of dosing, if any doses were missed and the reason for the missed dose. Compliance will be assessed by a review of diary entries

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and/or direct questioning and/or counting returned tablets/capsules during the visits. Deviation(s) from the prescribed dosage regimen should be recorded in the eCRF and study staff will document their follow-up with the participant in the source records when they discover compliance discrepancies. The number of remaining capsules/tablets will be documented in the source documents. Participants must be instructed to return all bottles of study interventions, as well as the completed participant dosing diary at every site visit for drug compliance.

In absence of toxicity requiring dose modifications (ie, reduction, interruption and/or delay), a study participant is considered underdosed if less than 80% of the prescribed doses for the given cycle has been administered.

A record of the number of each study intervention dispensed to and taken by each participant must be maintained and reconciled with study interventions and compliance records. Study interventions start and stop dates, including dates for dose reduction, interruption, and/or delay, will also be recorded in the CRF.

6.6. Dose Modifications

Every effort should be made to administer study interventions on the planned dose and schedule. However, in the event of significant toxicity, administration of study interventions may need to be modified based on the toxicity observed. In the event of multiple toxicities, dose modification should be based on the worst toxicity observed. Participants are to be instructed to notify investigators at the first occurrence of any adverse symptom. In addition to dose modifications, investigators are encouraged to employ best supportive care according to local institutional clinical practices.

Depending on the nature of the toxicity observed, dosing modifications may be required for just one or all study interventions in the combination. In the event treatment interruption is deemed necessary for just one of the study interventions of the combination, treatment with the other study interventions will continue as planned.

Dose modifications may occur in 1 of 3 ways:

- Within a cycle: dosing interruption until adequate recovery and dose reduction, if required and if applicable, during a given treatment cycle;
- Between cycles: next cycle administration may be delayed due to persisting toxicity when a new cycle is due to start;
- In the next cycle: dose reduction, if applicable, may be required in a subsequent cycle based on toxicity experienced in the previous cycle.

Toxicities potentially related to vepdegestrant in combination with palbociclib should be managed according to the dose modifications described in Table 10.

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In the Phase 3, toxicities potentially related to palbociclib administered with letrozole should be managed according to the dose modifications described in Table 11.

No dose reduction is permitted for toxicities potentially related to letrozole, but dosing interruption is allowed. Refer to letrozole prescribing information for further details on letrozole dose interruption (Femara [letrozole] UK SmPC).

The Phase 3 portion of the study will not be conducted.

Treatment interruption for drug-related toxicities will be performed as per the investigator's best medical judgment.

6.6.1. Dose Reductions

Following dosing interruption or cycle delay due to toxicity, vepdegestrant and/or palbociclib dose may need to be reduced when treatment is resumed.

No specific dose modifications are recommended for Grade 1 or 2 treatment-related toxicity. However, investigators should always manage their participants according to their medical judgment based on the particular clinical circumstances.

Participants experiencing recurrent and intolerable Grade 2 toxicity may resume dosing at the next lower dose level once recovery to Grade ≤1 or baseline is achieved, if dose reduction is applicable as per Table 7 and Table 8.

All dose modifications must be clearly documented in the participant's source notes and Investigational Product administration CRF.

6.6.1.1. vepdegestrant Dose Reductions

Dose reduction of vepdegestrant by 1 dose level will be allowed depending on the type and severity of toxicity encountered. Participants requiring more than one vepdegestrant dose reduction (ie, a dose lower than 100 mg QD) will be permanently discontinued from treatment with vepdegestrant. Investigators are encouraged to consult the sponsor medical monitor before permanent discontinuation of vepdegestrant due to treatment-related AEs.

Table 7. vepdegestrant/ Available Dose Levels

Dose Level	Available Doses	Administration Dose
Starting	200 mg/day	Two 100 mg tablets once daily
-1	100 mg/day*	One 100 mg tablet once daily

* Dose de-escalation below 100 mg/day is not allowed.

Once a dose has been reduced for a given participant, all subsequent cycles should be administered at that dose level. Depending on the nature of the toxicity and the rapidity of

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recovery following dose interruption, resumption of the same dose may be considered after a Grade 3 treatment-related AE, as reported in Table 10.

However, if the participant has demonstrated significant benefit, dose re-escalation of vepdegestrant may be considered after consultation with the sponsor medical monitor, if the AE was rapidly reversible, and re-escalation is not expected to pose a significant risk to the participant.

6.6.1.2. Palbociclib Dose Reductions

6.6.1.2.1. Palbociclib Dose Reduction for SLI and Phase 3 Experimental Arm (Arm A)

The Phase 3 portion of the study will not be conducted.

Participants under treatment with palbociclib 100 mg QD may be allowed to reduce the palbociclib dose to 75 mg QD depending on the type and severity of toxicity encountered (Table 10). Once a palbociclib dose has been reduced for a given participant, all subsequent cycles should be administered at that dose level.

Participant under treatment with palbociclib 75 mg QD experiencing a palbociclib-related toxicity, may be offered at the investigator's discretion to restart treatment at the same dose (re-challenge at 75 mg QD) or at a lower dose (ie, 50 mg QD, if available), depending on the type and severity of toxicity encountered (Table 10). Dose reduction to 50 mg QD may be offered at investigator's discretion and, where required, subject to regulatory approval. Dose reduction below 50 mg is not allowed. Investigators are encouraged to employ best supportive care according to local institutional clinical practices.

Participants under treatment with palbociclib 75 mg or 50 mg QD with recurrent Grade 3-4 event without improvement >1 week from dose interruption despite maximal supportive care should permanently discontinue the treatment. In case of recurrent Grade 3-4 event as previously reported occurring to participants under treatment at 75 mg QD, dose reduction to 50 mg QD, if available, is recommended before permanent discontinuation of palbociclib. Investigators are encouraged to consult the sponsor medical monitor before permanently discontinuing the treatment with palbociclib.

Participants permanently discontinuing palbociclib, may continue on the Active Treatment Phase of the study receiving vepdegestrant- at investigator's discretion. If vepdegestrant is permanently discontinued, treatment continuation with palbociclib monotherapy is NOT allowed. Then participants should enter the Post-treatment Follow- up Phase (see Section 7.1.1).

Once a palbociclib dose has been reduced for a given participant, all subsequent cycles should be administered at that dose level, unless further dose reduction is required. After a dose reduction, palbociclib dose re-escalation is NOT allowed.

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Table 8. Palbociclib Available Dose Levels

Palbociclib Dose	Administration of Dose
100 mg/daily	One 100 mg capsule once daily
75 mg/daily*	One 75 mg capsule once daily
50 mg/daily**	Two 25 mg capsules once daily

* Rechallenge at 75 mg/d is allowed at investigator's discretion.
**Palbociclib dose de-escalation below 50 mg/d is NOT allowed.

If a palbociclib dose reduction is applied in the same cycle, the participant will need to return to the clinic to receive new drug supply.

6.6.1.2.2. Palbociclib Dose Reduction for Phase 3 Control Arm (Arm B)

The Phase 3 portion of the study will not be conducted.

Palbociclib dose reductions by 1 and, if needed, 2 dose levels (Table 9) will be allowed depending on the type and severity of toxicity encountered (Table 11). Participants requiring more than 2 dose reductions (ie, a dose lower than 75 mg QD) will be permanently discontinued from palbociclib. Investigators are encouraged to consult the sponsor medical monitor before permanent discontinuation of palbociclib due to AEs. Participants permanently discontinuing palbociclib, may continue on the Active Treatment Phase of the study receiving letrozole at investigator's discretion. If letrozole is permanently discontinued, treatment continuation with palbociclib monotherapy is NOT allowed. Then participants should enter the Post-treatment Follow- up Phase (see Section 7.1.1).

Once a palbociclib dose has been reduced for a given participant, all subsequent cycles should be administered at that dose level, unless further dose reduction is required. Palbociclib dose re-escalation is NOT allowed.

Table 9. Palbociclib Available Dose Levels for Phase 3 Control Arm (Arm B)

Dose Level	Palbociclib Dose	Administration of Dose
Starting dose	125 mg/daily	One 125 mg capsule once daily
-1	100 mg/daily	One 100 mg capsule once daily
-2	75 mg/daily*	One 75 mg capsule once daily

*Palbociclib dose de-escalation below 75 mg/daily in NOT allowed

If a palbociclib dose reduction is applied in the same cycle, the participant will need to return to the clinic to receive new drug supply.

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6.6.1.3. Recommended vepdegestrant and Palbociclib Dose Modifications

The Phase 3 portion of the study will not be conducted.

Recommended vepdegestrant and palbociclib dose modification in the SLI and experimental arm of Phase 3 are outlined in Table 10.

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Table 10. vepdegestrant and Palbociclib Recommended Dose Modifications for Investigational Product-Related Toxicity for SLI and Phase 3 Experimental Arm (Arm A)

Toxicity	NCI CTCAE Severity Grade	vepdegestrant Dose Modifications	Palbociclib Dose Modifications
Non-Hematological Toxicities	Grade 1	Continue at the same dose.	Continue at the same dose.
	Grade 2	Continue at the same dose.	Continue at the same dose.
	Grade 3	Withhold dose until recovery to Grade ≤1 (Grade ≤2, if not considered a safety risk for the participant). Resume treatment at the same dose or reduce at the next lower dose at investigator's discretion*. *Exceptions:* • Laboratory values that do not have any clinical correlate (eg, amylase or lipase abnormality that is not associated with symptoms or clinical manifestations of pancreatitis).	Withhold dose until recovery to Grade ≤1 (Grade ≤2, if not considered a safety risk for the participant). • If palbociclib 100 mg QD: resume treatment at the next lower dose. • If palbociclib 75 mg QD: resume treatment at the same dose or at the next lower dose (if 50 mg QD dose reduction available). For Grade 3 recurrent event without improvement >1 week from dose interruption despite maximal supportive care, discontinue palbociclib (consider consulting the sponsor medical monitor before permanently discontinuing the treatment). See Section 6.6.1.2.1 for further details.
	Grade 4	Withhold dose until recovery to Grade ≤1. Resume treatment at the next lower dose or discontinue at investigator's discretion. *Exceptions:* • Laboratory values that do not have any clinical correlate, as per above.	Withhold dose until recovery to Grade ≤1 (Grade ≤2, if not considered a safety risk for the participant). • If palbociclib 100 mg QD: resume treatment at the next lower dose. • If palbociclib 75 mg QD: resume treatment at the same dose or at the next lower dose (if 50 mg dose reduction available). For Grade 4 recurrent event without improvement > 1 week from dose interruption despite maximal supportive care, discontinue palbociclib (consider consulting the sponsor medical monitor before permanently discontinuing the treatment). See Section 6.6.1.2.1 for further details.

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Table 10. vepdegestrant and Palbociclib Recommended Dose Modifications for Investigational Product-Related Toxicity for SLI and Phase 3 Experimental Arm (Arm A)

Toxicity	NCI CTCAE Severity Grade	vepdegestrant Dose Modifications	Palbociclib Dose Modifications
ILD/pneumonitis	Grade 3-4	Permanently discontinue treatment.	Permanently discontinued treatment.
Hematological Toxicities[a]	Grade 1	Continue at the same dose.	Continue at the same dose.
	Grade 2	Continue at the same dose.	Continue at the same dose.
	Grade 3	Continue at the same dose. If toxicity persist without improvement 1 week after palbociclib dose interruption, withhold treatment until toxicity is Grade ≤2. Resume treatment at the same dose, once the participant meets criteria to restart as per Section 6.6.3.	Day 1 of cycle: Withhold dose, repeat complete blood count monitoring within 1 week. When recovered to Grade ≤2, start the next cycle at the same dose. Day 15 of first 2 cycles: If Grade 3 on Day 15, continue palbociclib at current dose to complete cycle and repeat complete blood count on Day 22. If Grade 4 on Day 22, see Grade 4 dose modification guidelines below. If Grade 3 neutropenia lasting > 1 week or recurrent Grade 3 neutropenia on Day 1 of subsequent cycles: - If palbociclib 100 mg QD: resume treatment at next lower dose - If palbociclib 75 mg QD: resume treatment at the same dose or at the next lower dose (if 50 mg QD dose reduction available). Consider institution of supportive care per local guidelines.
Grade 3-4 neutropenia[b] with fever ≥38.5ºC and/or infection[b]	Grade 3-4	Continue at the same dose. If toxicity persist without improvement 1 week after palbociclib dose interruption, withhold treatment until toxicity is Grade ≤2. Resume treatment at the next lower dose.	At any time: Withhold dose until recovery to Grade ≤2. • If palbociclib 100 mg QD: resume treatment at the next lower dose • If palbociclib 75 mg QD: resume treatment at the same dose or at the next lower dose (if 50 mg QD dose

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Table 10. vepdegestrant and Palbociclib Recommended Dose Modifications for Investigational Product-Related Toxicity for SLI and Phase 3 Experimental Arm (Arm A)

Toxicity	NCI CTCAE Severity Grade	vepdegestrant Dose Modifications	Palbociclib Dose Modifications
			reduction available). Grade 3-4 recurrent event without improvement >1 week from dose interruption despite maximal supportive care: discontinue palbociclib (consider consulting the sponsor medical monitor before permanently discontinuing the treatment). See Section 6.6.1.2.1 for further details.
	Grade 4	Continue at the same dose. If toxicity persist without improvement 1 week after palbociclib dose interruption, withhold treatment until toxicity is Grade ≤2. Resume treatment at the next lower dose.	At any time: Withhold dose until recovery to Grade ≤2. • If palbociclib 100 mg QD: resume treatment at the next lower dose. • If palbociclib 75 mg QD: resume treatment at the same dose or at the next lower dose (if 50 mg QD dose reduction available). Grade 4 recurrent event without improvement >1 week from dose interruption despite maximal supportive care: discontinue palbociclib (consider consulting the sponsor medical monitor before permanently discontinuing the treatment). See Section 6.6.1.2.1 for further details.

* Grade 3 fatigue lasting <7 days, Grade 3 nausea/vomiting/diarrhea lasting <72 hours in the absence of maximal medical therapy do not require dose reduction.

a. Recommendation applies to all hematologic adverse reactions except lymphopenia (unless associated with clinical events, eg, opportunistic infections).

b. Neutropenia = Grade 1: ANC below LLN and up to 1500/mm^3; Grade 2: ANC up to 1000 to less than 1500/mm^3; Grade 3: ANC up to 500 to less than 1000/mm^3; Grade 4: ANC less than 500/mm^3

Recommended palbociclib dose modification in combination with letrozole in the control arm of Phase 3 are outlined in Table 11.

The Phase 3 portion of the study will not be conducted.

Table 11. Recommended Palbociclib Dose Modifications for Investigational Product-Related Toxicities for Phase 3 Control Arm (Arm B)

Toxicity	NCI CTCAE Severity Grade	Palbociclib Dose Modifications
Non-Hematological Toxicities	Grade 1	Continue at the same dose.
	Grade 2	Continue at the same dose.
	Grade 3-4	Withhold dose until recovery to Grade ≤1 (Grade ≤2, if not considered a safety risk for the participant). Resume treatment at next lower dose
ILD/pneumonitis	Grade 3-4	Permanently discontinue treatment
Hematological Toxicities[a]	Grade 1	Continue at the same dose.
	Grade 2	Continue at the same dose.
	Grade 3	Day 1 of cycle: Withhold dose, repeat complete blood count monitoring within 1 week. When recovered to Grade ≤2, start the next cycle at the same dose. Day 15 of first 2 cycles: If Grade 3 on Day 15, continue palbociclib at current dose to complete cycle and repeat complete blood count on Day 22. If Grade 4 on Day 22, see Grade 4 dose modification guidelines below. Consider dose reductions in cases of prolonged (>1 week) recovery from Grade 3 neutropenia or recurrent Grade 3 neutropenia on Day 1 of subsequent cycles.
Grade 3 neutropenia[b] with fever ≥38.5ºC and/or infection[b]	Grade 3	At any time: • Withhold dose until recovery to Grade ≤2. • Resume at the next lower dose.
	Grade 4	At any time: • Withhold dose until recovery to Grade ≤2. • Resume at the next lower dose.

a. Recommendation applies to all hematologic adverse reactions except lymphopenia (unless associated with clinical events, eg, opportunistic infections).
b. Neutropenia = Grade 1: ANC below LLN and up to 1500/mm3; Grade 2: ANC up to 1000 to less than 1500/mm3; Grade 3: ANC up to 500 to less than 1000/mm3; Grade 4: ANC less than 500/mm3

6.6.2. Dosing Interruptions

With respect to study intervention, participants experiencing the AEs should have their treatment interrupted as reported in vepdegestrant and palbociclib dose modification tables (Table 10 as Table 11, as applicable). Refer to letrozole prescribing information for dose interruption for letrozole (Femara [letrozole] UK SmPC).

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Treatment interruption for drug-related toxicities will be performed as per the investigator's best medical judgment.

Appropriate follow-up assessments should be done until adequate recovery occurs as assessed by the investigator. Criteria required before treatment can resume are described in the Dose Delays (Section 6.6.3).

Doses may be held up to 28 days until toxicity resolution. Depending on when the AE resolved, a treatment interruption may lead to the participant missing all subsequent planned doses within that same cycle or even to delay the initiation of the subsequent cycle.

If the AE that led to the treatment interruption recovers within the same cycle, then re-dosing in that cycle is allowed. Doses omitted for toxicity should NOT be replaced within the same cycle. The need for a dose reduction at the time of treatment resumption should be based on the criteria reported in Section 6.6.3 and Table 10 or Table 11 as applicable, unless expressly agreed otherwise following discussion between the investigator and the sponsor medical monitor.

In the event of a treatment interruption for reasons other than treatment-related toxicity (eg, elective surgery) lasting >28 days, treatment resumption will be decided in consultation with the sponsor medical monitor.

6.6.3. Dose Delays

Re-treatment following treatment interruption for treatment-related toxicity or at the start of any new cycle may not occur until all of the following parameters have been met:

- ANC ≥1000/mm^3 and no fever.
- Platelet count ≥50,000/mm^3.
- Grade ≥3 nonhematologic toxicities have returned to Grade ≤1 severity (or, at the investigator's discretion, Grade ≤2 if not considered a safety risk for the participant).
- QTc <501 ms and potential reversible causes (eg, electrolyte imbalance, concomitant medications known to prolong QTc) corrected. If QTc remains above 480 ms, ECG should be monitored more frequently as per the investigator's best medical judgement until QTc ≤480 ms.

If a treatment delay results from worsening of hematologic or biochemical parameters, the frequency of relevant blood tests should be increased as clinically indicated.

If these conditions are met within 28 days of treatment interruption, study intervention may be resumed. Refer to the Table 10 for recommended dose modification for vepdegestrant and palbociclib for AEs requiring dose reduction at the time of treatment resumption. Refer to letrozole prescribing information for dose interruption for letrozole (Femara [letrozole] UK SmPC).

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If participants require interruption of study intervention for more than 28 days at any time during the study, then study treatment should be permanently discontinued, unless the investigator's benefit/risk assessment suggests otherwise after discussion with the sponsor medical monitor.

If a treatment interruption continues beyond Day 28 of the current cycle, then the day when treatment is restarted will be counted as Day 1 of the next cycle. Procedures required as per SoA on Day 1 of the given cycle will be performed when study treatment is resumed.

6.6.4. Palbociclib Dose Increase

Participants randomized to palbociclib 75 mg QD may be offered to increase their palbociclib dose to 100 mg QD, at the investigator's discretion, after completion of the observation period (up to 4 cycles) and in absence of Grade ≥3 treatment-related clinically relevant toxicities reported in the last 4 weeks. Investigators are encouraged to consult the sponsor medical monitor before increasing the palbociclib dose.

After escalation to 100 mg QD, Day 1 and Day 15 hematology assessments should be performed for 2 additional cycles. Results should be provided to the site (eg, by email/fax) and brought to the next visit.

If after dose escalation to 100 mg QD, participants need a dose de-escalation to 75 mg QD, palbociclib dose re-escalation is NOT allowed.

6.7. Continued Access to Study Intervention After the End of the Study

Participants should be discontinued from study intervention as soon as PD by imaging as determined by the investigator per RECIST v1.1 is reported. No study intervention will be provided per protocol to study participants beyond the end of the study.

If the study is terminated (see Section 10.1.10) and the study intervention is not available as an approved treatment in the local country at the time of study termination, participants still demonstrating clinical benefit as per RECIST v1.1 (defined as complete or partial response or stable disease) would be eligible to receive Sponsor-supplied study intervention through different modalities (eg: expanded access/continued use mechanism/rollover) if the investigator and participant desire to continue treatment and at the discretion of the sponsor and subject to local conditions and regulations. In case a rollover study is implemented, written notification would be provided to investigational sites, and participants from the current trial would be consented and entered into the rollover study, if eligible.

6.8. Treatment of Overdose

Overdose is defined in this study as any dose of vepdegestrant>200 mg QD, palbociclib >125 mg QD, or letrozole > 2.5 mg QD.

No specific treatment for an overdose is recommended; treatment should be supportive.

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In the event of an overdose, the investigator should:

1. Contact the study medical monitor within 24 hours.

2. Closely monitor the participant for any AEs/SAEs and laboratory abnormalities as medically appropriate and follow-up until resolution, stabilization, the event is otherwise explained, or the participant is lost to follow-up (as defined in Section 7.3) or 5 half-lives of the concerned study intervention (approximately 6 days for vepdegestrant and palbociclib and approximately 10 days for letrozole) whichever is longer.

3. Document the quantity of the excess dose as well as the duration of the overdose in the CRF.

4. Overdose is reportable to Pfizer Safety **only when associated with an SAE.**

Decisions regarding dose interruptions or modifications will be made by the investigator in consultation with the study medical monitor as needed based on the clinical evaluation of the participant.

6.9. Prior and Concomitant Therapy

Participants must be instructed not to take any additional medications (over the counter or other products) during the study without prior consultation with the investigator. Any medications including herbal supplements, vitamins, or treatment (as reported in the sections below) taken by the participant from signature of the informed consent and up to 28 days following the last dose of investigational product must be recorded on the CRF along with:

- Reason for use.
- Dates of administration including start and end dates.
- Dosage information including dose and frequency.

The sponsor medical monitor or designee should be contacted if there are any questions regarding prior or concomitant therapies.

Concomitant treatment considered necessary for the participant's well-being may be given at the discretion of the investigator. All concomitant treatments, blood products, as well as non-drug interventions (eg, paracentesis) received by participants from signature of the informed consent and up to 28 days following the last dose of investigational product must be recorded on the CRF. Upon Amendment 3 implementation, concomitant therapy is to be recorded in site source only and not in the eCRF. SAE forms still require concomitant therapy reporting.

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6.9.1. Luteinizing Hormone-Releasing Hormone Agonist

Ovarian/testicular suppression therapy with an LHRH agonist (eg, goserelin or leuprolide acetate or equivalent agents) should be administered in pre-/peri-menopausal female participants and in male participants, starting (if not already ongoing) at Cycle 1 Day 1 and then administered every 28 days or every 3 months irrespectively of cycle duration up to 28 days after the last dose of study intervention. LHRH agonist treatment continuation during the survival FU will be at the discretion of the investigator. Participants already on treatment with a 28-day or 3-month formulation may continue it for the duration of the trial. Formulations longer than 3 months are not allowed. The investigator will determine and supply the appropriate LHRH agonist locally approved for use in BC considering any potential DDI based on the product prescribing information. If required by local regulations, LHRH may be provided by the sponsor if available to source in-country.

If LHRH agonist is administered at home, a diary will be implemented. The completed diary must be returned to the site at the next visit.

Overall, no male participants have been enrolled in the study.

6.9.2. Prohibited Inhibitors/Inducers

The concurrent use of following medications, food or herbal supplements is prohibited throughout the duration of the Active Treatment Phase in this study.

- **CYP3A strong inhibitors/inducers:** CYP3A4 is involved in vepdegestrant metabolism based on in vitro assays. Coadministration with drugs that are strong CYP3A inhibitors and strong CYP3A inducers may change the plasma concentrations of vepdegestrant and palbociclib in humans. Refer to Appendix 9 for examples of drugs that are strong CYP3A inhibitors/inducers.

6.9.3. Other Prohibited and/or Limited use of Antitumor/Anticancer or Experimental Drugs, or Procedures

The following treatments are also prohibited throughout the duration of the Active Treatment Phase in this study:

- **Anticancer agents:** No additional investigational or commercial anticancer agents such as chemotherapy, immunotherapy, targeted therapy, biological response modifiers, or endocrine therapy other than the initiated treatment by the investigator will be permitted during the Active Treatment Phase. No switching will be permitted. In general, any drugs containing “for the treatment of breast cancer” on the prescribing information are not permitted.

- **Hormone replacement therapy**, topical estrogens (including any intravaginal preparations), megestrol acetate and selective estrogen receptor modulators (eg, raloxifene).

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6.9.4. Concomitant Treatments to be used with Caution or Not Recommended

The following treatments should **be used with caution** throughout the duration of the Active Treatment Phase in this study:

- **Drugs known to predispose to QT interval prolongation or Torsade de Pointes**: Coadministration of drugs known to cause QT interval prolongation is cautioned in this study. Refer to Appendix 9 for examples of drugs known to predispose to Torsade de Pointes.

- **P-gp sensitive substrates:** Based on in vitro data vepdegestrant may inhibit P-gp sensitive substrates. Drugs that are P-gp sensitive substrates should be used with caution as vepdegestrant may increase the exposure of these drugs. Consult the respective local prescribing information of the P-gp substrate before the use. Refer to Appendix 9 for examples of drugs that are sensitive substrates of P-gp.

- **H2 receptor antagonists** (eg, cimetidine, famotidine) or **antacids** (eg, aluminum hydroxide, calcium carbonate, bismuth subsalicylate) may be used, but should be staggered. Take vepdegestrant, letrozole and palbociclib at least 2 hours before or 10 to 12 hours after H2 receptor antagonist dosing. Take vepdegestrant, letrozole and palbociclib at least 2 hours before or after antacid dosing. Alternatively, H2 receptor antagonists or antacids can be administered anytime when vepdegestrant, letrozole, and palbociclib are taken with a moderate-fat meal (400-800 calories, approximately 35% fat).

- **CYP3A sensitive substrates** whose minimal concentration changes may lead to serious toxicities (formerly known as Narrow Therapeutic Window [NTI]). These drugs should be used with caution as palbociclib may increase the drug exposure. Refer to Appendix 9 for examples of drugs that are CYP3A sensitive substrates whose minimal concentration changes may lead to serious toxicities.

The following treatments are **NOT recommended** throughout the duration of the Active Treatment Phase in this study:

- **PPIs**: co-administration of gastric acid-reducing agents may reduce vepdegestrant absorption. The concomitant use of PPIs with vepdegestrant is not recommended. If PPI treatment is required, vepdegestrant intake with a moderate-fat meal (400-800 calories, approximately 35% fat) is recommended. Refer to Appendix 9 for examples of drugs that are PPIs.

- **Herbal medicine**: use of herbal medicine.

Consider, whenever possible, to replace the concomitant treatments to be used with caution or not recommended with other drugs with the same indication for use.
If one of the treatments to be used with caution or not recommended is deemed necessary,

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consultation and agreement with the sponsor medical monitor or designee is encouraged prior to treatment initiation.

6.9.5. Supportive Care

Palliative and supportive care for disease related symptoms may be administered at the investigator's discretion and according to the specific supportive care product prescribing information or the current ASCO guidelines.

Participants should receive appropriate supportive care measures throughout duration of the Active Treatment Phase as deemed necessary by the investigator including but not limited to the items outlined below:

- **Standard therapies** for preexisting medical conditions, medical and/or surgical complications, and palliation. Any medication intended solely for supportive care (eg, analgesics, antidiarrheals, antidepressants) may also be used at the investigator's discretion.

- **Bisphosphonates and receptor activator of nuclear factor kappa B ligand** (RANKL) inhibitors for the treatment of osteoporosis or management of existing bone metastases may be continued for participants who have been receiving them at a stable dose for at least 2 weeks prior to randomization. However, the need to initiate or increase the dose of these therapies during the study will be considered as indicative of disease progression leading to the discontinuation of participant from the Active Treatment Phase unless disease progression can be completely ruled out and the exact reason for the use of these therapies clearly documented in the subject's source documentation.

- **Vaccines** locally approved vaccinations and booster doses are allowed, including Influenza and SARS-CoV-2 as appropriate.

- **Transfusion**: RBC and PLT transfusions may be administered as per investigator's discretion.

- **Anti-infectives** may also be used at the investigator's discretion. Before administration verify any potential DDI with the study interventions per Appendix 9.

- **Anticoagulant's use**: Participants who need to be on anticoagulant therapy during treatment should be treated with low molecular weight heparin. If low molecular weight heparin cannot be administered, warfarin or other warfarin derivatives or other oral anticoagulants (eg, direct Xa inhibitors) may be allowed. When using oral anticoagulants, close monitoring for signs of bleeding is recommended. Consult the product prescribing information for the appropriate coagulation test to be used for monitoring, as needed. Refer to Appendix 9 for potential DDI with P-gp sensitive substrates.

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6.9.6. Hematopoietic Growth Factors

Hematopoietic growth factors (eg, GCSF, GMCSF) may be used as indicated by the current ASCO guidelines (Smith et al, 2015).

Erythropoietin may be used at the investigator's discretion for the supportive treatment of anemia. For those countries where the indication and dosage of GCSF compounds may differ from ASCO guidelines, refer to the local prescribing information or follow the country's clinical practice.

6.9.7. Antidiarrheal, Antiemetic Therapy

Participants should receive appropriate supportive care measures as deemed necessary by the investigator including but not limited to the items outlined below:

- **Antidiarrheal:** All participants who experience diarrhea should be advised to drink liberal quantities of clear fluids. If sufficient oral fluid intake is not feasible, fluid and electrolytes should be substituted via IV infusion. Anti-diarrheal agents should be administered as needed.

- **Antiemetic Therapy**: Nausea and vomiting should be treated aggressively, and consideration should be given in subsequent cycles to the administration of prophylactic antiemetic therapy according to standard institutional practice. Participants should be strongly encouraged to maintain liberal oral fluid intake.

The choice of the prophylactic drug, as well as the duration of treatment, is up to the investigator with sponsor medical monitor approval assuming there is no known or expected DDI and assuming the drug is not included in the Concomitant Therapy section (Section 6.9).

6.9.8. Corticosteroids

Chronic immune suppressive therapies, including systemic corticosteroid use (prednisone >10 mg/day or equivalents) for palliative or supportive purposes is not permitted. Steroids given as hormonal replacement (prednisone ≤10 mg/day or equivalents) is allowed.

Acute emergency administration, topical applications, inhaled sprays, eye drops, local injections of corticosteroids or as premedication prior to contrast media administration as per local guidelines is allowed.

6.9.9. Anti-Inflammatory Therapy

Anti-inflammatory or narcotic analgesic may be offered as needed assuming there is no known or expected drug-drug interaction and assuming the drug is not included in the Concomitant Therapy section (Section 6.9).

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Acetaminophen/paracetamol to a maximum total daily dose of 2 grams is permitted. Daily intake over 2 grams is prohibited.

6.9.10. Surgery and Radiotherapy

Any cancer-related surgery or concurrent radiotherapy (excluding palliative radiotherapy as per specifications below) are prohibited throughout the duration of the Active Treatment Phase of the study. Participants requiring any of these procedures will be discontinued from the Active Treatment Phase and will enter the Follow-up Phase.

Caution is advised for any surgical procedures during the Active Treatment Phase of the study. The appropriate interval of time between surgery and vepdegestrant and palbociclib administration required to minimize the risk of impaired wound healing and bleeding has not been determined. Stopping vepdegestrant and palbociclib administration is recommended at least 7 days prior to surgery. Postoperatively, the decision to reinitiate vepdegestrant and palbociclib treatment should be based on a clinical assessment of satisfactory wound healing and recovery from surgery.

Palliative radiotherapy during the Active Treatment Phase of the study is permitted for the treatment of painful bony lesions provided the lesions were known to be present at the time of study entry and the investigator clearly documents that the need for palliative radiotherapy is not indicative of disease progression.

In view of the current lack of data about the interaction of vepdegestrant/ with radiotherapy, vepdegestrant treatment should be interrupted 7 days before initiation of palliative radiotherapy and restarted after recovery of any radiotherapy associated signs and symptoms. Palbociclib treatment should be interrupted 1 day before initiation of palliative radiotherapy and restarted 7 days after provided the recovery of any radiotherapy associated signs and symptoms is achieved.

For participants with bone involvement, it is suggested to institute palliative radiotherapy before study initiation if possible and clinically appropriate (eg, lesions at risk for spontaneous micro-fractures or painful lesions). Information related to administration of palliative radiotherapy (eg, date of RT, site of RT) should be recorded on the appropriate CRF. Upon Amendment 3 implementation, concomitant procedures should be recorded in site source only and not in the eCRF. SAE forms still require concomitant therapy reporting.

No specific requirements are needed for concomitant surgery and radiotherapy in respect to the use of letrozole (Femara [letrozole] UK SmPC).

6.9.11. Rescue Medicine

There is no rescue therapy to reverse the AEs observed with any of the study intervention involved in this protocol. Standard medical supportive care should be provided to manage the AEs.

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7. DISCONTINUATION OF STUDY INTERVENTION AND PARTICIPANT DISCONTINUATION/WITHDRAWAL

7.1. Discontinuation of Study Intervention

It may be necessary for a participant to permanently discontinue study intervention. Reasons for permanent discontinuation of study intervention may include the following:

- Objective disease progression per RECIST v1.1 by investigator assessment (see Appendix 11). However, participants with disease progression by RECIST v1.1 who are continuing to derive clinical benefit from the study intervention (ie, positive benefit/risk assessment as assessed by investigator), will be eligible to continue study intervention. See Section 8.2.2 for details);
- Global deterioration of health status requiring discontinuation;
- Unacceptable toxicity;
- Pregnancy (refer to Section 8.3.5);
- Significant protocol violation;
- Lost to follow-up;
- Participant refused further treatment;
- Study terminated by sponsor;
- Death

Note that discontinuation of study intervention does not represent withdrawal from the study. If study intervention is permanently discontinued, the participant should remain in the study to be evaluated for safety and survival. See the SoA for data to be collected at the time of discontinuation of study intervention and follow-up for any further evaluations that need to be completed.

In the event of discontinuation of study intervention, it must be documented on the appropriate CRF/in the medical records whether the participant is discontinuing further receipt of study intervention or also from study procedures, posttreatment study follow-up, and/or future collection of additional information.

7.1.1. Post-Treatment Follow-up

Participants discontinuing the Active Treatment Phase will enter to the Post-Treatment Follow-up period, which comprises Safety and Long-Term/Survival Follow-up Periods (see SoA for required assessments), as applicable.

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7.1.1.1. Safety Follow-Up:

At least 28 calendar days and no more than 35 calendar days after discontinuation of study intervention or prior to initiation of any new anticancer therapy whichever comes first, participants will return to site to undergo the safety FU assessments as reported in the SoA.

Any SAEs occurring during the 28-day from last dose (active reporting period) must still be reported to Pfizer Safety irrespective of any intervening treatment. See Section 8.4.1 for details of AE/SAE reporting requirements.

7.1.1.2. Long-Term Follow-up (SLI only)

Contraception for male participants and the start date of the first FU anticancer therapy (if not already collected during the Safety FU) will be collected as per SoA. Start date of the first FU anticancer therapy may be collected by telephone or email at site, telehealth, or home health visits or via family member contact, as applicable.

Of Note: in LTFU, tumor assessment (including bone scan, if applicable) is required only for participants who discontinued the study treatment for reason other than PD. Tumor assessment (including bone scan, if applicable), with the frequency reported in the SoA, should continue to be performed and reported in the CRF until PD (see Section 8.2.1).

7.1.1.3. Survival Follow-up (Phase 3 only)

The Phase 3 portion of the study will not be conducted.

During the Survival Follow-up period, post-treatment survival status will be collected every 3 months after the EOT date. Participants will be contacted (eg, telephone, email, site visits, family member contact) for survival status and collection of subsequent anticancer therapies (eg, start and stop dates, date of PD of the therapy), until death, withdrawal of consent, lost to follow-up, or end of study, whichever occurs first. On sponsor request, participants may be contacted for survival information, eg, prior to each protocol-specified interim and final analysis. As permitted by local law, study site personnel may use public databases, perform an internet search, or review obituaries to determine date of death. Survival information should be recorded in the CRF.

7.1.2. COVID-19

While SARS-CoV-2 testing is not mandated for this study, local clinical practice standards for testing should be followed. A participant should be excluded if he/she has a positive test result for SARS-CoV-2 infection, is known to have asymptomatic infection, or is suspected of having SARS-CoV-2. When the infection resolves, the participant may be considered for re-screening.

If a participant develops symptomatic COVID-19 during the Active Treatment Phase, this should be reported as an AE or SAE (as appropriate) and appropriate medical intervention provided considering potential DDIs for any concomitant medication administered for treatment of SARS-CoV-2 infection (see Section 6.9). Study treatment should be

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interrupted/delayed to allow the symptoms to recover. Before restarting treatment, the participant should be afebrile for 72 hours, and SARS-CoV-2-related symptoms should have recovered to Grade ≤1 for a minimum of 72 hours. Notify the study team when treatment is restarted.

If the investigator is concerned about the safety of the participant, it is recommended that he/she discuss any permanent discontinuation of study intervention with the sponsor medical monitor or designee.

7.2. Participant Discontinuation/Withdrawal From the Study

A participant may withdraw from the study at any time at their own request. Reasons for discontinuation from the study may include:

- Withdrawal by participant
- Lost to follow-up
- Death
- Study terminated by sponsor
- Completed (SLI only)

Participant in the SLI will be considered as completed the study participation after the Safety FU as per Section 7.1.1.1 has been performed and collection of the start date of the first FU anticancer therapy obtained.

If a participant withdraws from the study, he/she may request destruction of any remaining samples taken and not tested, and the investigator must document any such requests in the site study records and notify the sponsor accordingly.

If the participant withdraws from the study and also withdraws consent (see Section 7.2.1) for disclosure of future information, no further evaluations will be performed and no additional data will be collected. The sponsor may retain and continue to use any data collected before such withdrawal of consent.

7.2.1. Withdrawal of Consent

Participants who request to discontinue receipt of study intervention will remain in the study and must continue to be followed for protocol-specified follow-up procedures. The only exception to this is when a participant specifically withdraws consent for any further contact with them or persons previously authorized by the participant to provide this information. Participants should notify the investigator in writing of the decision to withdraw consent from future follow-up, whenever possible. The withdrawal of consent should be explained in detail in the medical records by the investigator, as to whether the withdrawal is only from further receipt of study intervention or also from study procedures and/or posttreatment study follow-up and entered on the appropriate CRF page. In the event that vital status (whether

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the participant is alive or dead) is being measured, publicly available information should be used to determine vital status only as appropriately directed in accordance with local law.

7.3. Lost to Follow-Up

A participant will be considered lost to follow-up if the participant repeatedly fails to return for scheduled visits and is unable to be contacted by the study site.

The following actions must be taken if a participant fails to return to the clinic for a required study visit:

- The site must attempt to contact the participant and reschedule the missed visit as soon as possible. Counsel the participant on the importance of maintaining the assigned visit schedule and ascertain whether the participant wishes to and/or should continue in the study.

- Before a participant is deemed lost to follow-up, the investigator or designee must make every effort to regain contact with the participant (where possible, 3 telephone calls and, if necessary, a certified letter to the participant's last known mailing address or local equivalent methods). These contact attempts should be documented in the participant's medical record.

- Should the participant continue to be unreachable, the participant will be considered to have withdrawn from the study.

8. STUDY ASSESSMENTS AND PROCEDURES

After implementation of Amendment 3, the 1.3 will be reduced, as per section 1.3.

8.1. Administrative and Baseline Procedures

The investigator (or an appropriate delegate at the investigator site) must obtain a signed and dated ICD before performing any study-specific procedures.

Study procedures and their timing are summarized in the SoA. Protocol waivers or exemptions are not allowed.

Adherence to the study design requirements, including those specified in the SoA, is essential and required for study conduct.

All screening evaluations must be completed and reviewed to confirm that potential participants meet all eligibility criteria. The investigator will maintain a screening log to record details of all participants screened and to confirm eligibility or record reasons for screening failure, as applicable.

Safety assessments, ECGs and tumor assessment by imaging as per Section 8.2.1, conducted as part of the participant's routine clinical management (and obtained before signing of the ICD may be utilized for eligibility provided the procedures met the protocol-specified criteria and were performed within the time frame defined in the SoA.

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Every effort should be made to ensure that protocol-required tests and procedures are completed as described. However, it is anticipated that from time to time there may be circumstances outside the control of the investigator that make it unfeasible to perform the test. In these cases, the investigator must take all steps necessary to ensure the safety and well-being of the participant. When a protocol-required test cannot be performed, the investigator will document the reason for the missed test and any corrective and preventive actions that they have taken to ensure that required processes are adhered to as soon as possible. The Pfizer study team must be informed of these incidents in a timely manner.

For samples being collected and shipped, detailed collection, processing, storage, and shipment instructions and contact information will be provided to the investigator site prior to initiation of the study.

The total blood sampling volume for individual participants in the SLI of this study is approximately 15 mL to 60 mL per study visit.

The total blood sampling volume for individual participants in the Phase 3 of this study is approximately 10 mL to 45 mL per study visit.

The Phase 3 portion of the study will not be conducted.

The actual collection times of blood sampling may change. Additional blood samples may be taken for safety assessments at times specified by Pfizer, provided the total volume taken during the study does not exceed 550 mL during any period of 56 consecutive days.

8.1.1. Baseline Procedures

Medical history will be recorded at screening for each participant. Medical history includes any signs, symptoms, and history not associated to the disease under study before informed consent signature.

Oncological history will be recorded at screening for each participant and includes details regarding the disease under study, eg, tumor histologic grade, ER, PR, HER2 status as per local standards, relevant disease characteristics, TNM staging per the American Joint Committee on Cancer (AJCC; 8th edition) cancer staging manual (Giuliano, Edge & Hortobagyi, 2018) and all prior treatments including prior systemic therapies, radiation, and surgical procedures.

8.1.2. Telehealth Visits

Telehealth visits may be used to assess participant safety and collect data points, upon written confirmation from the sponsor. Telehealth includes the exchange of healthcare information and services via telecommunication technologies (eg, audio, video, video conferencing software) remotely, allowing the participant and the investigator to communicate on aspects of clinical care, including medical advice, reminders, education, and

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safety monitoring. The following assessments must be performed during a telehealth visit (see the SoA):

- Review and record study intervention(s), including compliance and missed doses.
- Review and record any AEs and SAEs since the last contact. Refer to Section 8.4.
- Review and record any new concomitant medications/treatments (eg, surgery or radiotherapy per Section 6.9.10) or changes in concomitant medications since the last contact. Information on FU anticancer therapy may be collected as well, as applicable.
- Review laboratory tests and ECGs reports performed at alternative facilities, if applicable.
- Review LHRH agonist compliance for pre-/peri-menopausal female participants and for male participants (Section 6.9.1) and contraception for IOCBP participants and male participants.
    - Review and record contraceptive method and results of pregnancy testing, as applicable (Section 8.3.5). Confirm that the participant is adhering to the contraception method(s) required in the protocol. Refer to Appendix 4.
- PRO assessment (see Appendix 13 Section 10.13.1), as applicable.

Participants should be instructed to bring original reports of assessments performed in alternative facilities and diaries at the next site visit.

Any telehealth visit may be changed to an in-person visit based on participant preference or investigator discretion.

Participants must be reminded to promptly notify site staff about any change in their health status. Participants may request an unscheduled telehealth or in-person visit with site staff.

8.1.3. Home Health Visits

A home health visit may be utilized to facilitate scheduled visits and will be allowed on a case-by-case basis. Home health visits include a registered nurse or other qualified healthcare provider conducting an in-person study visit at the participant's location, rather than an in-person study visit at the site. The investigator or designee must be available for consultation. If required by local regulations, home heath visits will be performed by the investigator or by a physician designee for specific assessments (eg, collection of adverse events, physical examination).

The following may be performed during a home health visit (see the SoA):

- Review and record study intervention(s), including compliance and missed doses.

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- Review and record any AEs and SAEs since the last contact. Refer to Section 8.4.
- Review and record any new concomitant medications/treatments (eg, surgery or radiotherapy per Section 6.9.10) or changes in concomitant medications since the last contact. Information on FU anticancer therapy may be collected as well, as applicable.
- Collect blood samples for safety assessment, PK and biomarkers, perform ECGs, as applicable, and review laboratory tests and ECGs reports performed at alternative facilities, if applicable.
- Review LHRH agonist compliance for pre-/peri-menopausal female participants and for male participants (Section 6.9.1) and contraception for IOCBP participants and male participants.
    - Review and record contraceptive method and results of pregnancy testing, as applicable. Confirm that the participant is adhering to the contraception method(s) required in the protocol. Refer to Appendix 4.
- Perform a physical examinations and vital signs collection.
- PRO assessments (see Appendix 13 Section 10.13.1), as applicable.

The investigator must review the reports from healthcare professionals conducting home health visits and follow up with the participant as appropriate.

8.1.4. Alternative Facilities

In the event that an in-person visit is not feasible at the site the following may be performed at alternative facilities, approved by the investigator upon written notification from the sponsor, if allowed by local regulations (see the SoA):

- **ECGs.** (see Section 8.3.3.1).
- **Safety laboratory tests** (see Section 8.3.4.1).
- **Tumor assessments** (ie, all tumor scans, photographs, etc) (see Section 8.2.4).

8.2. Efficacy Assessments

After implementation of Amendment 3, frequency of efficacy assessments will be performed as per clinical standard of care at the discretion of the investigator. Assessment will be performed according to RECIST v1.1.

8.2.1. Imaging Tumor Response Assessments

Timely and complete disease assessments in this study are critical. Imaging assessments are to be scheduled according to the calendar, starting from the randomization date as the

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reference date for all time-points regardless of treatment delays/interruptions and are NOT to be scheduled based on the date of the previous imaging time-point. Imaging assessment delay to conform to a treatment delay is NOT permitted.

Radiographic imaging, for all known or suspected disease sites, is required at screening and during the Active Treatment Phase using the **SAME** imaging techniques. The CT and MRI scans (including brain CT/MRI, if applicable) should be performed with contrast agents unless contraindicated for medical reasons. Premedication prior to contrast media administration as per local guidelines is allowed (see Section 6.9). If IV contrast is medically contraindicated, the imaging modality to be used to follow the disease (either CT without contrast or MRI) should be the modality which best evaluates the disease, and the choice should be determined by the investigator in conjunction with the local radiologist. MRI of the abdomen and pelvis can be substituted for a CT only if the MRI adequately depicts the disease. However, a CT of the chest should not be substituted by a chest MRI even if IV contrast is contraindicated. In such case, a CT should be performed without contrast. If MRI is used to follow-up bone lesion(s), it must be performed a few days before any treatment that may affect bone marrow cellularity (eg, GCSF).

Screening tumor assessment will be carried out within the 28-day period before randomization unless otherwise specified. Disease assessment for all participants at screening will include:

- Computerized tomography (CT) or magnetic resonance imaging (MRI) of chest abdomen and pelvis (CAP), mandatory.
- CT/MRI scan of any other sites of disease (including brain), as clinically indicated.
- Clinical assessment of superficial lesions, as clinically indicated.
- Bone scan (whole body), mandatory.

Screening CT/MRI scans obtained per the participant's standard of care prior to the signature of the informed consent do not need to be repeated and are acceptable to be used as baseline evaluations, if 1) obtained within 28 days before randomization; 2) they were performed using the method requirements outlined in RECIST v1.1; 3) the same technique/modality can be used to follow identified lesions throughout the trial for a given participant; and 4) appropriate documentation indicating that these radiographic tumor assessments were performed as standard of care is available in the participant's source notes.

Screening brain scan is only required if signs and symptoms suggest the presence of metastatic brain disease. Brain scans performed before the signing of informed consent as routine procedures (but within 6 weeks before randomization) do not need to be repeated and may be used as screening assessments as long as 1) tests were performed using the method requirements outlined in RECIST v1.1; 2) the same technique/modality can be used to follow identified lesions throughout the trial for a given participant; 3) appropriate documentation indicating that these radiographic tumor assessments were performed as standard of care is

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available in the participant's source notes. Post-baseline brain scans are required only if brain metastases are present at screening or in case new brain lesion(s) is(are) suspected.

Clinical assessment of superficial lesions will include assessment with caliper of all identified superficial metastatic lesions. For the case of skin lesions, documentation by color photography including a ruler to estimate the size of the lesion is suggested.

Screening bone scan is mandatory to detect bony sites of disease. Bone scans performed before the signing of informed consent as routine procedures (but within 12 weeks before randomization) do not need to be repeated and may be used as screening assessments as long as 1) tests were performed using the method requirements outlined in RECIST v1.1; 2) the same technique/modality can be used to follow identified lesions throughout the trial for a given participant; 3) appropriate documentation indicating that these radiographic tumor assessments were performed as standard of care is available in the participant's source notes. Any suspicious abnormalities (ie, hotspots) identified on the screening bone scans and on subsequent bone scans MUST be confirmed by X-ray, CT scan with bone windows or MRI. Bone lesion(s) identified at screening by bone scan can be further assessed at screening by CT with bone windows or MRI as per local practice and subsequently re-assessed on treatment by CT with bone windows or MRI (consistently with the technique used at screening) as per tumor assessment schedule (ie, on week 8, 16 and 24 then every 12 weeks (in SLI) and every 12 weeks (in Phase 3) from randomization date. Areas that have received palliative radiotherapy cannot be used to assess response to study treatment.

All lesions identified at screening as measurable and non-measurable must be recorded in the CRF and evaluated during the Active Treatment Phase.

Of note: Tumor lesions previously irradiated or subjected to other locoregional therapy will only be deemed measurable if progression at the treated site as per RECIST v1.1 is clearly documented, after completion of therapy. A target lesion biopsied during the screening period will NOT be considered as a measurable lesion.

Active Treatment Phase Tumor Assessments will include

- CT/MRI scan of CAP mandatory.
- CT/MRI scan of any other sites of disease (including brain), if lesions identified at screening assessment.
- Clinical assessment of superficial lesions, if lesions identified at screening
- Bone scan, mandatory only if it is the preferred technique to follow bone lesions identified at screening.

CT/MRI scans must be repeated on treatment on weeks 8, 16 and 24 then every 12 weeks in the SLI and every 12 weeks (in Phase 3) and bone scans (if bone scan is the preferred technique to follow bone lesions on treatment) every 24 weeks from randomization date,

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until radiographically and/or clinically (ie, for photographed or palpable lesions) documented PD as per RECIST v1.1, study treatment discontinuation (for participants continuing treatment beyond RECIST-defined disease progression), initiation of new anticancer therapy or discontinuation of participant from overall study participation (eg, death, participant's request, lost to follow up), whichever occurs first. After implementation of Amendment 3, participants who discontinue study treatment for reasons other than radiographically and/or clinically (ie, for photographed lesions) documented PD as per RECIST v1.1 will not be required to have tumor assessments performed after permanent treatment discontinuation.

After implementation of Amendment 3, treatment beyond progression will no longer be initiated as part of the study.

CR and PR must be confirmed with repeated imaging performed at least 4 weeks after initial documentation of response.

Brain scan or scans of any other anatomical site identified will be repeated on the Active Treatment Phase only if lesion/s present at screening or new lesions are suspected.

Clinical assessment of superficial disease should be carried out preferably on the same date as the imaging studies.

If no bone lesions were identified at screening, bone scans will only be repeated during the Active Treatment Phase when clinically indicated (ie, participant describes new or worsening bone pain, or has increasing alkaline phosphatase level, or other signs and symptoms of new/progressing bone metastases) or at the time of confirmation of CR. Any abnormality identified by bone scan on treatment, must also be confirmed by X-ray, CT scan with bone windows or MRI.

Further imaging assessments may be performed at any time if clinically indicated (eg, suspect PD, symptomatic deterioration).

Participants who have already demonstrated objective disease progression as determined by investigator per RECIST v1.1 are expected to discontinue study therapy and begin the follow-up phase of the trial. See Section 8.2.2 for treatment continuation beyond progression.

For participants who do NOT have documented objective disease progression at time of study treatment discontinuation, tumor assessment will continue to be performed as per SoA, until radiographically and/or clinically (ie, for photographed or palpable lesions) confirmed objective disease progression, initiation of new anticancer therapy, or discontinuation of participant from overall study participation (eg, death, participant's request, lost to follow-up).

For participants having effusions or ascites, cases having cytological proof of malignancy should be recorded as non-target lesions on the tumor assessment CRFs. Effusions that have not been evaluated using cytology or were found to be non-malignant should NOT be

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recorded on the non-target lesion CRF. Upon Amendment 3 implementation, tumor assessments should be recorded in the site source only and not in the eCRF.

Disease response assessments will be based on the Response Evaluation Criteria in Solid Tumors (RECIST v1.1) (also refer to Appendix 11).

All participants' files and radiologic images and pathology samples must be available for source verification.

Management of Incidental Findings

An incidental finding is one unknown to the participant that has potential health or reproductive importance, which is discovered unexpectedly in the course of a research study but is unrelated to the purpose and beyond the aims of the study.

Tumor assessment images will be reviewed by a central review facility. The purpose of this review is to evaluate images for disease assessment. Central image review is not a complete medical review of the participant, and no incidental findings will be shared with the investigator, site staff, or the participant. All safety reviews will be the sole responsibility of site staff.

The Phase 3 portion of the study will not be conducted.

8.2.2. Treatment Continuation Beyond Disease Progression By Imaging

After implementation of Amendment 3, treatment beyond progression will no longer be initiated as part of the study. For participants who have already progressed before implementation of Amendment 3 and are on active treatment at the time of Amendment 3 implementation, their discontinuation of study intervention will follow instruction in Section 7.1.

Participants should be discontinued from study intervention as soon as PD by imaging as determined by the investigator per RECIST v1.1 is reported.

8.2.3. Independent Review of Disease Assessments (Phase 3 only)

The Phase 3 portion of the study will not be conducted.

A BICR laboratory will perform a review of radiographic images for all participants as well as all the other information reported below, as applicable, collected on study to determine the protocol defined endpoints of disease response and progression.

It is important to the integrity of the study that the following materials are provided to BICR as each participant enrolls and progresses through the study:

- imaging studies for all participants;
- clinical assessment of superficial lesion as per Section 8.2.1;

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- clinical information, as applicable.

Screening, Active Treatment Phase, Follow-up images and clinical information should be sent for central reading on an ongoing basis (as soon as they are available) until PD by BICR as per RECIST v1.1, irrespectively of initiation of a new anticancer therapy.

8.2.3.1. Expedited Blinded Independent Central Review for Disease Progression

To mitigate the potential for bias in determining disease progression, expedited BICR review will be performed for investigator assessed disease progression. Upon investigator assessed disease progression, the radiographic image as well as all the other information reported as per Section 8.2.3 will be submitted to the BICR for expedited review. See the Study Imaging Manual for process details. At investigator discretion, the participant may remain on study intervention until the BICR has completed the review.

8.2.4. Alternative Facilities for Tumor Response Assessments

Tumor assessments (eg, by tumor scans, photographs, etc) may be performed upon sponsor approval at an alternative facility as long the equipment is adequate (eg, qualification, calibration, maintenance) and the images are available at the investigative site for overall tumor response assessments and for submission to BICR, if applicable (see Section 8.2.1).

8.2.5. Survival Follow-Up Analysis (Phase 3 only)

The Phase 3 portion of the study will not be conducted.

The study Survival Follow-up period will conclude at the time of the final OS analysis (see Section 4.4).

8.3. Safety Assessments

Planned time points for all safety assessments are provided in SoA. Unscheduled safety measurements may be obtained at any time during the study to assess any perceived safety issues.

8.3.1. Physical Examinations

At screening a complete PE will include assessments of all the major body systems. The complete PE will include the examination of general appearance (head, eyes, ears, nose, mouth, throat, skin, heart, lung, lymph nodes, gastrointestinal, genitourinary, neurologic, and muscle-skeletal). Height (at screening only) and weight will also be measured and recorded.

Sites of disease identified by PE will be recorded on the tumor assessment pages and recorded as target or non-target lesions. Investigators should pay special attention to clinical signs related to previous serious illnesses.

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For measuring weight, a scale with appropriate range and resolution must be used and placed on a stable, flat surface. Participants must remove shoes, bulky layers of clothing, and jackets so that only light clothing remains. They must also remove the contents of their pockets and remain still during measurement of weight.

PEs may be conducted by a physician, trained physician's assistant, or nurse practitioner as acceptable according to local regulation. On dosing days PE must be performed prior to study intervention administration.

Symptom-directed physical examinations and body weight may be performed at subsequent visits as clinically indicated per Amendment 3.

PE findings collected during the study will be considered source data and will not be required to be reported, unless otherwise noted. Any untoward physical examination findings that are identified during the active collection period and meet the definition of an AE or SAE (Appendix 3) must be reported according to the processes in Sections 8.4.1 to 8.4.3.

8.3.2. Vital Signs

Vital signs should be measured per institutional standards and will include systolic and diastolic blood pressure, and pulse rate.

Per Amendment 3, vital signs will be collected as clinically indicated.

Vital signs should be assessed while the participant is in a sitting position or semi-recumbent position (recommended). It is recommended that the same position be used consistently throughout the duration unless circumstances, eg, hospitalization, dictate otherwise.

Vital signs should be preceded by at least 5 minutes of rest for the participant in a quiet setting without distractions (eg, television, cell phones).

Abnormal readings should be repeated and confirmed.

Any untoward vital sign findings that are identified during the active collection period and meet the definition of an AE or SAE (Appendix 3) must be reported according to the processes in Sections 8.4.1 to 8.4.3.

8.3.3. Electrocardiograms

Standard 12-lead ECGs (a single measurement with a 10-second rhythm strip) should be collected at the time points specified in the SoA (Per Amendment 3, ECG tests will be performed as clinically indicated at Investigator's discretion) using an ECG machine that automatically calculates the mean HR and measures QTc interval and also collects QT interval, PR interval, and QRS complex. For ECG machines that do not report QTcF, calculation of QTcF from QT and HR, for example using online tools, is required. As per SoA, electrolytes (ie, Na, K, total Ca, Mg) must be performed near to each ECG assessment.

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All scheduled ECGs should be performed after the participant has rested quietly for at least 5 minutes in a supine position.

SLI only: Triplicate ECGs are defined as 3 consecutive ECGs performed approximately 2 minutes apart but within 10 minutes for all the 3 measurements as per SoA.

On dosing days, ECG will be collected predose.

When an ECG is to be performed at the same time point as a blood collection (eg, safety laboratory assessments, PK samples), the ECG should be performed first.

At any time, additional ECGs (triplicate or single at the discretion of the investigator), as well as electrolyte laboratory assessments (ie, Na, K, total Ca, Mg), may be performed as clinically indicated.

The ECG must be reviewed by qualified personnel at the site as soon as any abnormality is observed, including verifying that the machine reading is accurate, and that the Fridericia correction formula is applied. In some cases, it may be appropriate to repeat abnormal ECGs (ie, ≥2 times approximately 2 minutes apart) to rule out improper lead placement as contributing to the ECG abnormality. It is important that leads be placed in the same positions each time to achieve precise ECG recordings.

If readings confirm a QTc of ≥501 msec or >60 msec change from baseline, immediate search for reversible causes (including electrolyte abnormalities, hypoxia and concomitant medications for drugs that may potentially prolong the QTcF interval) should be performed.

Repeat ECG should be performed when one of the following conditions apply:

- When a post dose QTc interval remains >60 msec change from the baseline OR is ≥501 msec, repeat ECGs should be immediately performed as per investigator's best medical judgement until the QTc interval is ≤60 msec change from the baseline or the QTc interval falls ≤480 msec.

- QTc intervals get progressively longer.

A cardiologist should be consulted if QTc intervals do not return to less than the criteria listed above after 8 hours of monitoring (or sooner, at the discretion of the investigator).

If QTc interval reverts to <501 msec or ≤60 msec change from baseline, and in the judgment of investigator in consultation with the sponsor medical monitor, the cause is determined to be other than study intervention, treatment may be continued with regular ECG monitoring under hospital supervision.

If study intervention causality cannot be ruled out, study intervention dose modifications and/or interruption or permanent discontinuation, as applicable, should be performed

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according to dose modification instructions provided for vepdegestrant in Section 6.6.1.1 and for palbociclib in Section 6.6.1.2 and Table 10. Refer to the letrozole UK SmPC for dose modifications for the letrozole.

8.3.3.1. Alternative Facilities for Electrocardiograms

The participant may visit an alternative facility to have the ECGs performed (see Section 8.1.4). Qualified study site personnel must order, receive, and review results. ECGs can also be performed at home through home health vendors or through remote device collection.

Tests results are to be provided to the site staff as soon as possible. Participants should be instructed to bring original reports in at the next site visit. ECG reports should be filed in the participant's source documents/medical records. Relevant data from the ECG report should be recorded on the CRF. Upon Amendment 3 implementation, ECG results should be recorded in site source only and not in the eCRF

8.3.4. Clinical Safety Laboratory Assessments

Per Amendment 3, clinical safety laboratory tests will be performed as clinically indicated. See Appendix 2 for the list of clinical safety laboratory tests to be performed and the SoA for the timing and frequency. All protocol-required laboratory assessments, as defined in Appendix 2, must be conducted in accordance with the laboratory guidance (e.g. manual) and the SoA. Unscheduled clinical laboratory measurements may be obtained at any time during the study to assess any perceived safety issues.

The investigator must review the laboratory report, document this review, and record any clinically significant changes occurring during the study in the AE section of the CRF. Clinically significant abnormal laboratory test findings are those that are not associated with the underlying disease, unless judged by the investigator to be more severe than expected for the participant's condition.

All laboratory tests with values considered clinically significant and abnormal during participation in the study or within 28 days after the last dose of study intervention should be repeated until the values return to normal or baseline or are no longer considered clinically significant by the investigator or sponsor medical monitor.

If such values do not return to normal/baseline within a period of time judged reasonable by the investigator, the etiology should be identified, and the sponsor notified.

See Appendix 6 for suggested actions and follow-up assessments in the event of potential DILI.

See Appendix 7 for instructions for laboratory testing to monitor kidney function and reporting laboratory test abnormalities.

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8.3.4.1. Alternative Facilities for Clinical Safety Laboratory Assessment

Protocol-specified safety laboratory evaluations as per SoA may be conducted at a local laboratory, if permitted by local regulations. The local laboratory may be a standalone institution or within a hospital.

If a local laboratory is used, qualified study site personnel must order, receive, and review results. Site staff must collect the local laboratory reference ranges and certifications/ accreditations for filing at the site. Laboratory test results are to be provided to the site staff as soon as possible. Participants should be instructed to bring original reports in at the next site visit. The local laboratory reports should be filed in the participant's source documents/medical records. Relevant data from the local laboratory report should be recorded on the CRF. Upon Amendment 3 implementation, laboratory results should be recorded in site source only and not in the eCRF.

8.3.5. Pregnancy Testing

A serum pregnancy test is required at screening for IOCBP participants. Following a negative pregnancy test result at screening, appropriate contraception must be commenced, and a second negative pregnancy test result will be required at the baseline visit prior to the participant's receiving the study intervention. Pregnancy tests will be performed in IOCBP participants at the times listed in the SoA. Following screening, pregnancy tests may be serum or urine tests, and must have a sensitivity of at least 25 mIU/mL. If a urine test cannot be confirmed as negative (eg, an ambiguous result), a serum pregnancy test is required. In such cases, the participant must be excluded if the serum pregnancy result is positive. Pregnancy tests may also be performed at any time per investigator's judgement (eg, when a potential pregnancy is suspected) or if required by local regulations or IRBs/ECs.

8.3.5.1. At-Home Pregnancy Testing

If a participant requiring pregnancy testing cannot visit a local laboratory, a home urine pregnancy testing kit with a sensitivity of at least 25 mIU/mL may be used by the participant to perform the test at home, if compliant with local regulatory requirements. The pregnancy test outcome should be documented in the participant's source documents/medical records. Confirm that the participant is adhering to the contraception method(s) required in the protocol.

8.4. Adverse Events, Serious Adverse Events, and Other Safety Reporting

The definitions of an AE and an SAE can be found in Appendix 3.

AEs may arise from symptoms or other complaints reported to the investigator by the participant (or, when appropriate, by a caregiver, surrogate), or they may arise from clinical findings of the investigator or other healthcare providers (clinical signs, test results, etc).

The investigator and any qualified designees are responsible for detecting, documenting, and recording events that meet the definition of an AE or SAE and remain responsible to pursue and obtain adequate information both to determine the outcome and to assess whether the

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event meets the criteria for classification as an SAE or caused the participant to discontinue the study intervention (see Section 7.1).

During the active collection period as described in Section 8.3.1, each participant will be questioned about the occurrence of AEs in a nonleading manner.

In addition, the investigator may be requested by Pfizer Safety to obtain specific follow-up information in an expedited fashion.

8.4.1. Time Period and Frequency for Collecting AE and SAE Information

The time period for actively eliciting and collecting AEs and SAEs ("active collection period") for each participant begins from the time the participant provides informed consent, which is obtained before undergoing any study-related procedure and/or receiving study intervention, through and including a minimum of 28 calendar days, except as indicated below, after the last administration of the study intervention.

Follow-up by the investigator continues throughout and after the active collection period and until the AE or SAE or its sequelae resolve or stabilize at a level acceptable to the investigator.

When a clinically important AE remains ongoing at the end of the active collection period, follow-up by the investigator continues until the AE or SAE or its sequelae resolve or stabilize at a level acceptable to the investigator and Pfizer concurs with that assessment.

For participants who are screen failures, the active collection period ends when screen failure status is determined.

If the participant withdraws from the study and also withdraws consent for the collection of future information, the active collection period ends when consent is withdrawn.

If a participant permanently discontinues or temporarily discontinues study intervention because of an AE or SAE, the AE or SAE must be recorded on the CRF and the SAE reported using the PSSA.

Investigators are not obligated to actively seek information on AEs or SAEs after the participant has concluded study participation. However, if the investigator learns of any SAE, including a death, at any time after a participant has concluded study, participation and they consider the event to be reasonably related to the study intervention, the investigator must promptly report the SAE to Pfizer Safety.

8.4.1.1. Reporting Nonserious AEs and SAEs to Pfizer Safety

All SAEs occurring in a participant during the active collection period as described in Section 8.4.1 are reported to Pfizer Safety immediately upon awareness and under no circumstance should this exceed 24 hours, as indicated in Appendix 3. The investigator will submit any updated SAE data to the sponsor within 24 hours of these data being available.

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If a participant begins a new anticancer therapy, SAEs occurring during the above-indicated active collection period must still be reported to Pfizer Safety irrespective of any intervening treatment. Note that a switch to a commercially available version of the study intervention is considered as a new anticancer therapy for purposes of SAE reporting.

8.4.1.2. Recording Nonserious AEs and SAEs on the CRF

All nonserious AEs and SAEs occurring in a participant during the active collection period which begins after obtaining informed consent as described in Section 8.4.1, will be recorded on the AE section of the CRF.

The investigator is to record on the CRF all directly observed, and all spontaneously reported AEs and SAEs reported by the participant.

As part of ongoing safety reviews conducted by the sponsor, any nonserious AE that is determined by the sponsor to be serious will be reported by the sponsor as an SAE. To assist in the determination of case seriousness, further information may be requested from the investigator to provide clarity and understanding of the event in the context of the clinical study.

If a participant begins a new anticancer therapy, the recording period for nonserious AEs ends at the time the new treatment is started; however, SAEs must continue to be recorded on the CRF during the above-indicated active collection period. Note that a switch to a commercially available version of the study intervention is considered as a new anticancer therapy for the purposes of SAE reporting.

8.4.2. Method of Detecting AEs and SAEs

The method of recording, evaluating, and assessing causality of AEs and SAEs and the procedures for completing and transmitting SAE reports are provided in Appendix 3.

Care will be taken not to introduce bias when detecting AEs and/or SAEs. Open-ended and nonleading verbal questioning of the participant is the preferred method to inquire about AE occurrences.

8.4.3. Follow-Up of AEs and SAEs

After the initial AE or SAE report, the investigator is required to proactively follow each participant at subsequent visits/contacts. For each event, the investigator must pursue and obtain adequate information until resolution, stabilization, the event is otherwise explained, or the participant is lost to follow-up (as defined in Section 7.3).

Follow-up by the investigator continues throughout the active collection period and until the AE or SAE or its sequelae resolve or stabilize at a level acceptable to the investigator.

When a clinically important AE remains ongoing at the end of the active collection period, follow-up by the investigator continues until the AE or SAE or its sequelae resolve or stabilize at a level acceptable to the investigator and Pfizer concurs with that assessment.

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In general, follow-up information will include a description of the event in sufficient detail to allow for a complete medical assessment of the case and independent determination of possible causality. Any information relevant to the event, such as concomitant medications and illnesses, must be provided. In the case of a participant death, a summary of available autopsy findings must be submitted as soon as possible to Pfizer Safety.

In addition, the investigator may be requested by Pfizer Safety to obtain specific follow-up information in an expedited fashion.

Further information on follow-up procedures is provided in Appendix 3.

8.4.4. Regulatory Reporting Requirements for SAEs

Prompt notification by the investigator to the sponsor of an SAE is essential so that legal obligations and ethical responsibilities toward the safety of participants and the safety of a study intervention under clinical investigation are met.

The sponsor has a legal responsibility to notify both the local regulatory authority and other regulatory agencies about the safety of a study intervention under clinical investigation. The sponsor will comply with country-specific regulatory requirements relating to safety reporting to the regulatory authority, IRBs/ECs, and investigators.

Investigator safety reports must be prepared for SUSARs according to local regulatory requirements and sponsor policy and forwarded to investigators as necessary.

An investigator who receives SUSARs or other specific safety information (eg, summary or listing of SAEs) from the sponsor will review and then file it along with the SRSD(s) for the study and will notify the IRB/EC, if appropriate according to local requirements.

8.4.5. Environmental Exposure, Exposure During Pregnancy or Breastfeeding, and Occupational Exposure

Environmental exposure occurs when a person not enrolled in the study as a participant receives unplanned direct contact with or exposure to the study intervention. Such exposure may or may not lead to the occurrence of an AE or SAE. Persons at risk for environmental exposure include healthcare providers, family members, and others who may be exposed. An environmental exposure may include EDP, EDB, and occupational exposure.

Any such exposure to the study intervention under study are reportable to Pfizer Safety within 24 hours of investigator awareness.

8.4.5.1. Exposure During Pregnancy

An EDP occurs if:

- A participant is found to be pregnant while receiving or after discontinuing study intervention.

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- A participant who is receiving or has discontinued study intervention inseminates a partner.

- A nonparticipant is found to be pregnant while being exposed or having been exposed to study intervention because of environmental exposure. Below are examples of environmental EDP:

  - A family member or healthcare provider reports that they are pregnant after having been exposed to the study intervention by ingestion or skin contact.

  - A family member or healthcare provider who has been exposed to the study intervention by ingestion or skin contact then inseminates his female partner prior to or around the time of conception.

The investigator must report EDP to Pfizer Safety within 24 hours of the investigator's awareness, irrespective of whether an SAE has occurred. The initial information submitted should include the anticipated date of delivery (see below for information related to termination of pregnancy). For information on how to report to Pfizer Safety, refer to Section 10.3.4.

- If EDP occurs in a participant/participant's partner, the investigator must report this information to Pfizer Safety regardless of whether an SAE has occurred. Details of the pregnancy will be collected after the start of study intervention and until 28 days after the last dose of study intervention.

- If EDP occurs in the setting of environmental exposure, the investigator must report information to Pfizer Safety. Since the exposure information does not pertain to the participant enrolled in the study, the information is not recorded on a CRF; however, a copy of completed report is maintained in the investigator site file.

Follow-up is conducted to obtain general information on the pregnancy and its outcome for all EDP reports with an unknown outcome. The investigator will follow the pregnancy until completion (or until pregnancy termination) and notify Pfizer Safety of the outcome as a follow-up to the initial EDP. In the case of a live birth, the structural integrity of the neonate can be assessed at the time of birth. In the event of a termination, the reason(s) for termination should be specified and, if clinically possible, the structural integrity of the terminated fetus should be assessed by gross visual inspection (unless pre-procedural test findings are conclusive for a congenital anomaly and the findings are reported).

Abnormal pregnancy outcomes are considered SAEs. If the outcome of the pregnancy meets the criteria for an SAE (ie, ectopic pregnancy, spontaneous abortion, intrauterine fetal demise, neonatal death, or congenital anomaly in a live-born baby, a terminated fetus, or an intrauterine fetal demise, or a neonatal death), the investigator should follow the procedures

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for reporting SAEs. Additional information about pregnancy outcomes that are reported to Pfizer Safety as SAEs follows:

- Spontaneous abortion including miscarriage and missed abortion should be reported as an SAE;
- Neonatal deaths that occur within 1 month of birth should be reported, without regard to causality, as SAEs. In addition, infant deaths after 1 month should be reported as SAEs when the investigator assesses the infant death as related or possibly related to exposure to the study intervention.

Additional information regarding the EDP may be requested by the sponsor. Further follow-up of birth outcomes will be handled on a case-by-case basis (eg, follow-up on preterm infants to identify developmental delays). In the case of paternal exposure, the investigator will provide the participant with the Pregnant Partner Release of Information Form to deliver to their partner. The investigator must document in the source documents that the participant was given the Pregnant Partner Release of Information Form to provide to their partner.

8.4.5.2. Exposure During Breastfeeding

An EDB occurs if:

- A participant is found to be breastfeeding while receiving or after discontinuing study intervention.
- A nonparticipant is found to be breastfeeding while being exposed or having been exposed to study intervention (ie, environmental exposure). An example of environmental EDB is a family member or healthcare provider who reports breastfeeding after having been exposed to the study intervention by ingestion.

The investigator must report EDB to Pfizer Safety within 24 hours of the investigator's awareness, irrespective of whether an SAE has occurred. For information on how to report to Pfizer Safety, refer to Section 10.3.4. When EDB occurs in the setting of environmental exposure, the exposure information does not pertain to the participant enrolled in the study, so the information is not recorded on a CRF. However, a copy of the completed report is maintained in the investigator site file.

An EDB report is not created when a Pfizer drug specifically approved for use in breastfeeding individuals (eg, vitamins) is administered in accordance with authorized use. However, if the infant experiences an SAE associated with such a drug, the SAE is reported together with the EDB.

8.4.5.3. Occupational Exposure

The investigator must report any instance of occupational exposure to Pfizer Safety within 24 hours of the investigator's awareness regardless of whether there is an associated SAE. Since the information about the occupational exposure does not pertain to a participant

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enrolled in the study, the information is not recorded on a CRF; however, a copy of the completed report must be maintained in the investigator site file.

8.4.6. Cardiovascular and Death Events

Cardiovascular events will be reported as AE/SAE as per Section 8.4 and Appendix 3. Cardiovascular events are to be managed per the dose modification/interruption guidelines in Section 6.6. AEs and SAEs that result in death are to be reported per guidelines as outlined in Section 8.4.

8.4.7. Disease-Related Events and/or Disease-Related Outcomes Not Qualifying as AEs or SAEs

Not Applicable.

8.4.8. Protocol-Specified Adverse Events of Special Interest

Protocol-Specified AESIs are examined as part of routine safety data review procedures throughout the clinical trial and as part of signal detection processes. Should an aggregate analysis indicate that these prespecified events occur more frequently than expected, eg, based on epidemiological data, literature, or other data, then this will be submitted and reported in accordance with Pfizer's safety reporting requirements. Aggregate analyses of safety data will be performed on a regular basis per internal SOPs.

Protocol-specified AESIs are examined as part of routine safety data review procedures throughout the clinical trial and as part of signal detection processes.

AEs that are considered protocol-specified AESIs for vepdegestrant include QT prolongation and venous embolism. Based on current understanding of the safety profile, no expedited reporting by the investigator to sponsor is required for non-serious protocol-specified AESIs. Additional detail and mitigation strategies are summarized in Section 2.3.1.

Protocol-specified AESIs for palbociclib and letrozole and mitigation strategies are summarized in Section 2.3.1.

All protocol-specified AESIs must be reported as an AE or SAE following the procedures described in Sections 8.4.1 through 8.4.4. An AESI is to be recorded as an AE or SAE on the CRF. In addition, an AESI that is also an SAE must be reported to Pfizer Safety.

Protocol-Specified AESIs for vepdegestrant include QTc prolongation and VTE.

8.4.8.1. Lack of Efficacy

The investigator must report signs, symptoms, and/or clinical sequelae resulting from lack of efficacy. Lack of efficacy or failure of expected pharmacological action is reportable to Pfizer Safety whether or not it is associated with a nonserious AE or SAE according to section 8.4.

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8.4.9. Medical Device Deficiencies

Not applicable.

8.4.10. Medication Errors

Medication errors may result from the administration or consumption of the study intervention by the wrong participant, or at the wrong time, or at the wrong dosage strength.

Medication errors are recorded and reported as follows:

Recorded on the Medication Error Page of the CRF	Recorded on the AE Page of the CRF	Reported to Pfizer Safety Within 24 Hours of Awareness
All (regardless of whether associated with an AE)	Any AE or SAE associated with the medication error	Only if associated with an SAE

Medication errors include:

- Medication errors involving participant exposure to the study intervention.
- Potential medication errors or uses outside of what is foreseen in the protocol that do or do not involve the study participant.

Other examples include, but are not limited to:

- The administration of expired study intervention;
- The administration of an incorrect study intervention (ie, incorrect randomized assignment, or incorrect study intervention being used in this study);
- The administration of an incorrect dosage defined as underdose (see Section 6.5) or overdose (see Section 6.8).

Such medication errors occurring to a study participant are to be captured on the medication error page of the CRF, which is a specific version of the AE page.

Whether or not the medication error is accompanied by an AE, as determined by the investigator, such medication errors occurring to a study participant are recorded on the medication error page of the CRF which is a specific version of the AE page and, if applicable, any associated serious and nonserious AE(s), are recorded on the AE page of the CRF.

In the event of a medication dosing error, the sponsor should be notified within 24 hours.

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Medication errors should be reported to Pfizer Safety within 24 hours **only when associated with an SAE.**

8.5. Pharmacokinetics

Any pharmacokinetic assessments are no longer required.

Blood samples collection for measurements of plasma concentration of vepdegestrant, its epimer ARV-473, and palbociclib are detailed in Sections 8.5.1 and 8.5.2, respectively.

The actual times may change, but the number of samples will remain the same. All efforts will be made to obtain the samples at the exact nominal time and relative to dosing. Collection of samples within the sampling time window specified in the SoA will not be captured as a protocol deviation, as long as the exact time and date of the collection is noted on the source document and the CRF.

On the days of site visits, including PK sampling days, participants must be instructed to not take study interventions at home and to withhold their daily doses of study interventions until the pre-dose PK blood sample has been collected. On these days, study interventions should be taken in the clinic under the supervision of the study site personnel.

The actual time of sample collection and the most recent dosing time before and after each collection will be recorded on the CRF. The date of missing/reduced doses should also be recorded in the CRF.

If a scheduled blood sample collection cannot be completed for any reason, the missed sample time may be re-scheduled with agreement of the investigator, participant, and sponsor. In addition to samples collected at the scheduled times, every effort should be made to collect an additional blood sample from participants experiencing unexpected and /or serious AEs and to document the date and time of samples collection and of last dosing intake prior to PK sample collection on the CRF.

Genetic analyses will not be performed on these samples unless consent for this was included in the informed consent. Participant confidentiality will be maintained.

Any changes in the timing or addition of time points for any planned study assessments must be documented and approved by the relevant study team member and then archived in the sponsor and site study files but will not constitute a protocol amendment. The IRB/EC will be informed of any safety issues that require alteration of the safety monitoring scheme or amendment of the ICD.

Refer to Laboratory Manual for detailed collection, processing, and shipping procedures.

8.5.1. Pharmacokinetic Sampling for vepdegestrant

Blood samples for measurement of plasma concentrations of vepdegestrant (ARV-471) and its epimer ARV-473 will be collected as specified in the SoA. At each designated timepoint,

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blood sample of approximately 3.0 mL whole blood to provide approximately 1.5 mL plasma will be collected into appropriately labeled tube containing K2 EDTA for vepdegestrant and ARV-473. Instructions and details for the collection, shipping, and handling of biological samples will be provided in the laboratory manual or by the sponsor. The actual date and time (24-hour clock time) of each sample will be recorded.

Samples collected for analyses of vepdegestrant and its epimer ARV-473 concentration may also be used to evaluate safety or efficacy aspects related to concerns arising during or after the study, for metabolite identification and/or evaluation of the bioanalytical method, or for other internal exploratory purposes.

Samples collected for measurement of plasma concentrations of vepdegestrant and its epimer ARV-473 will be analyzed using a validated analytical method in compliance with applicable SOPs. Potential metabolites may be analyzed with either validated or exploratory methods.

The PK samples must be processed and shipped as indicated in the instructions provided for vepdegestrant to the investigator site to maintain sample integrity. Any deviations from the PK sample handling procedure (eg, sample collection, and processing steps, interim storage, or shipping conditions), including any actions taken, must be documented, and reported to the sponsor. On a case-by-case basis, the sponsor may make a determination as to whether sample integrity has been compromised.

8.5.2. Pharmacokinetic Sampling for Palbociclib

Blood samples for measurement of plasma concentrations of palbociclib will be collected as specified in the SoA. At each designated timepoint, blood sample of approximately 3.0 mL whole blood to provide approximately 1.5 mL plasma will be collected into appropriately labeled tube containing K2 EDTA. Instructions and details for the collection, shipping, and handling of biological samples will be provided in the laboratory manual for palbociclib or by the sponsor. The actual date and time (24-hour clock time) of each sample will be recorded.

Samples collected for analyses of palbociclib concentration may also be used to evaluate safety or efficacy aspects related to concerns arising during or after the study, for metabolite identification and/or evaluation of the bioanalytical method, or for other internal exploratory purposes.

Samples collected for measurement of plasma concentrations of palbociclib will be analyzed using a validated analytical method in compliance with applicable SOPs. Potential metabolites may be analyzed with either validated or exploratory methods.

The PK samples must be processed and shipped as indicated in the instructions for palbociclib provided to the investigator site to maintain sample integrity. Any deviations from the PK sample handling procedure (eg, sample collection, and processing steps, interim storage, or shipping conditions), including any actions taken, must be documented, and

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reported to the sponsor. On a case-by-case basis, the sponsor may make a determination as to whether sample integrity has been compromised.

8.6. Genetics

8.6.1. Specified Genetics

Specified genetics (analyses) are not evaluated in this study.

8.6.2. Retained Research Samples for Genetics

A single 4 mL whole blood sample optimized for DNA isolation (Prep D1: K2 EDTA) will be collected according to the SoA, as local regulations and IRBs/ECs allow.

Retained Research Samples may be used for research related to the study intervention(s) and aBC. Genes and other analytes (eg, proteins, RNA, nondrug metabolites) may be studied using the banked samples.

See Appendix 5 for information regarding genetic research. Details on processes for collection and shipment of these samples can be found in the Laboratory Manual.

NOTE: Retained Research Samples for genetics are not applicable for participants in China

8.7. Biomarkers and Pharmacodynamics

8.7.1. Biomarkers (Phase 3 only)

The Phase 3 portion of the study will not be conducted.

Tumor tissues and/or blood samples will be used to evaluate biomarkers (genes and proteins) involved in the biology of the participant's disease and mechanisms of sensitivity or resistance to vepdegestrant in combination with palbociclib, to conduct genomic, and/or molecular profiling analyses. The biomarkers involved in cellular proliferation, oncologic driver, and immunologic regulations, for example, but not limited to, Ki67 for protein marker, ESR1, and PIK3CA gene mutations, will be investigated. In addition, the quantitative changes (on treatment from baseline) of cell free tumor DNA concentration in participant blood will be analyzed to estimate its potential as a treatment response. Refer to the SoA for sample collection time points and Laboratory Manual for sample processing and shipping.

Unless prohibited by local regulations or IRBs/ECs, the following samples for biomarker research are required and will be collected from all participants in this study as specified in the SoA:

- Tumor Biopsy (see Section 8.7.1.1)
- Blood Samples (see Section 8.7.1.2)

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Optional samples for biomarker research that should be collected from participants in the study as appropriate are the following:

- Tumor tissue sample collection at the EOT visit.

Additional information on tissue collection procedures, sample preparations including processing, storage, and shipment to the sponsor designated laboratories for biomarkers analysis, will be provided in the Laboratory Manual.

Note: for participants in China, sample collection for biomarkers and biomarker testing will not be performed unless approved by HGRAC.

8.7.1.1. Tumor Biopsies

Tumor biopsy collected at the time of presentation with locoregionally advanced, or metastatic disease is mandatory for study participation. If no tissue taken at the time of presentation with locoregionally advanced or metastatic disease is available, a de novo biopsy is needed at screening.

Exception: for participants with bone lesions only, an archival tumor sample (ie, tumor sample from initial diagnosis) is acceptable.

Only core needle or excisional biopsies, or resection specimen are suitable. Cytologic preparations, such as fine needle aspirate biopsies, are not acceptable. Bone biopsies are not acceptable since the decalcification process renders the sample unusable for biomarker analysis.

For all participants, submission of FFPE tumor samples (blocks or 15 unstained slides [10 minimum] of 5 µm thickness on positively charged slides) will be required.

An optional de novo tumor biopsy may be collected at the EOT visit only for participants who discontinue treatment due to disease progression and when, in the investigator's judgment, such biopsy is feasible and can be safely performed. The EOT tumor tissue will be used to determine possible mechanisms of resistance. Tissue samples from all participants will be used for additional biomarker analyses.

8.7.1.2. Blood Samples

Peripheral blood and derivatives may be used to characterize potential correlations of tumor molecular profile and ctDNA levels with response to study treatment and to understand potential mechanisms of acquired resistance to study treatment. Assessment of the relationship between ctDNA burden and outcome is a secondary objective of this study. Additional analyses may be warranted based on emerging data.

Two 10 mL blood samples (one 10 mL sample for China) optimized for plasma preparation for ctDNA analysis will be collected predose as per SoA. DNA samples will be analyzed for

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the purpose of the assessment of changes in plasma ctDNA following treatment with vepdegestrant coadministered with palbociclib.

8.7.2. Pharmacodynamics (SLI only)

A 4-mL blood sample will be collected at the time points specified in 1.3 to evaluate surrogate pharmacodynamic markers involved in cell proliferation (eg, serum TK1 activity).

8.8. Immunogenicity Assessments

Immunogenicity assessments are not included in this study.

8.9. Health Economics (Phase 3 only)

The Phase 3 portion of the study will not be conducted.

PROs assessments will be administered to evaluate the impact on quality of life, functioning, symptoms, and general health status using the EQ-5D-5L and EORTC QoL questionnaires (QLQ-C30 and QLQ-BR23 [female and male questionnaire versions]).

All self-assessment questionnaires should be completed by the participants while in the clinic before any other study activities or medical procedures as per SoA.

Refer to Appendix 13 Section 10.13.1 for further information on the PRO assessments administered.

9. STATISTICAL CONSIDERATIONS

Detailed methodology for summary and statistical analyses of the data collected in this study is outlined here and further detailed in the SAP, which will be maintained by the sponsor. The SAP may modify what is outlined in the protocol where appropriate; however, any major modifications of the primary endpoint definitions or their analyses will also be reflected in a protocol amendment.

9.1. Statistical Hypotheses

9.1.1. Study Lead-in Estimands

The co-primary estimands are defined by the following attributes:

- Population: Participants with ER(+), HER2(-) breast cancer who have not received any prior systemic anti-cancer treatment for advanced disease, have been randomized in the SLI.

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- Endpoints:
  - Grade 4 neutropenia AE (as graded by NCI CTCAE v5.0) with onset within the SLI observation period (ie, up to cycle 4). Neutropenia will be reported as a cluster term including the MedDRA PTs neutropenia and neutrophils count decrease.
  - Dose reduction or drug discontinuation of any study drugs due to any cause occurring within the SLI observation period (ie, up to cycle 4).
- Intercurrent events:
  - Intercurrent events for Grade 4 neutropenia AE: All neutropenia AEs reported by investigator with onset within the SLI observation period will be considered. This includes neutropenia AEs with onsets during study treatment or after treatment discontinuation occurring during the SLI observation period.
  - No intercurrent events for dose reduction and drug discontinuation.
- Population-level summary measures:
  - Incidence of Grade 4 neutropenia AE is defined as the number of SLI SAS participants with Grade 4 neutropenia AE with onset within the first 4 cycles divided by the number of SLI SAS participants.
  - Incidence of dose reduction or discontinuation is defined as the number of participants reducing or discontinuing any study drug due to any cause within the first 4 cycles divided by the number of SLI SAS participants.

9.1.2. Phase 3 Estimands

The Phase 3 portion of the study will not be conducted.

9.1.2.1. Primary Estimand/Co-primary Estimands

The estimand is defined by the following attributes:

- Population: Participants with ER(+), HER2(-) breast cancer who have not received any prior systemic anti-cancer treatment for advanced disease and have been randomized in the Phase 3.
- Endpoint: PFS, defined as the time from the date of randomization to the date of first documented disease progression, as determined by BICR assessment per RECIST v1.1, or death due to any cause, whichever occurs first.
- Intercurrent events: PFS data will be censored on the date of the last adequate tumor assessment for participants with the following intercurrent events:
  - Discontinue the study treatment due to withdrawal of consent prior to an event

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- Start a new anti-cancer therapy prior to an event
- Have an event after an unacceptably long interval (2 or more missing or non-evaluable assessments)
- Lost to follow-up

- Censoring: See Table 12.
- Population-level summary: Hazard ratio for PFS including all randomized participants.

Table 12. PFS (Primary Definition) Outcome and Event Dates

Situation	Date of Event/Censoring	Outcome
No adequate baseline assessment	Date of randomization[a]	Censored[a]
PD or death - after at most one missing or inadequate post-baseline tumor assessment, OR - ≤24 weeks after the date of randomization	Date of PD or death	Event
PD or death - after 2 or more missing or inadequate tumor assessments	Date of last adequate tumor assessment[b] documenting no PD prior to new anti-cancer therapy or missed tumor assessments	Censored
No PD and no death		
New anti-cancer therapy given prior to PD or death		

a. If the participant dies ≤24 weeks after the date of randomization and did not initiate new anti-cancer therapy, the death is an event with date on death date.
b. If there are no adequate post-baseline tumor assessments prior to the PD or death, then the time without adequate assessment should be measured from the date of randomization; if the criteria were met the censoring will be on the date of randomization.

9.1.2.2. Key Secondary Estimands

The estimand is defined by the following attributes:

- Population: Patients with ER(+), HER2(-) breast cancer who have not received any prior systemic anti-cancer treatment for advanced disease and have been randomized in the Phase 3.
- Endpoint: OS, defined as the time from the date of randomization to the date of death due to any cause.

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- Intercurrent events: All data will be used regardless of the occurrence of intercurrent events (discontinuation of study treatment, use of subsequent treatments, discontinuation of study, etc.).

- Censoring: Participants last known to be alive are censored at date of last contact.

- Population-level summary: Hazard ratio for OS including all randomized participants.

9.1.3. Multiplicity Adjustment

The following 2 pairs of statistical hypotheses will be tested to formally evaluate the primary and the key secondary endpoints (PFS and OS):

$$H_{01}: HR_{PFS} \geq 1 \text{ vs. } H_{11}: HR_{PFS} < 1$$

$$H_{02}: HR_{OS} \geq 1 \text{ vs. } H_{12}: HR_{OS} < 1,$$

where HR_{PFS} is the hazard ratio for PFS for Arm A versus Arm B; HR_{OS} is the hazard ratio for OS for Arm A versus Arm B. The analyses of these 2 endpoints contain multiple interim looks, with possible futility (PFS) and efficacy (OS) stopping. As appropriate, the final sample size for testing the OS endpoint may be re-estimated and adjusted during its second interim analysis.

To control the overall Type-I error rate at α=0.025 (1-sided) for the analyses of PFS and OS, the following multiplicity adjustments will be taken:

- A gate-keeping hierarchical testing approach will be used, whereby PFS will be formally tested at an overall 1-sided 0.025 level of significance in the FAS first. If it is significant, then OS will be tested in the FAS at an overall 1-sided 0.025 level of significance.
- At each interim and final analysis, the critical values for rejecting the null hypotheses H_{01}, H_{02} are adjusted with the Lan-DeMets (O'Brien-Fleming) α-spending function for OS, and the non-binding futility boundary is determined by a Rho(1.5) β-spending function for PFS only (O'Brien & Fleming, 1979). The efficacy and futility critical values will be updated based on the actual numbers of observed events.

If the OS sample size is adjusted during its second interim analysis, then the test statistic for OS will be adjusted using the Cui–Hung–Wang (CHW) method (Cui, Hung & Wang, 1999).

According to (Hung, Wang & O'Neill, 2007), the above multiplicity adjustment can control the familywise error rate (FWER) for testing the primary and key secondary endpoints at level α=0.025 (1-sided), in the strong sense, for a range of scenarios including this study design.

9.2. Analysis Sets

For purposes of analysis, the following analysis sets are defined.

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The Phase 3 portion of the study will not be conducted.

Participant Analysis Set	Description
Enrolled	"Enrolled" means a participant's agreement to participate in a clinical study following completion of the informed consent process and randomization to study intervention.
Full Analysis Set (FAS)	All enrolled participants who were randomized in the Phase 3. Participants are analyzed according to the treatment to which they have been randomized.
SLI FAS	All enrolled participants who were randomized in the SLI. Participants are analyzed according to the treatment to which they have been randomized.
Safety Analysis Set (SAS)	All enrolled participants who were randomized in the Phase 3 and receive ≥1 dose of study intervention.
SLI Safety Analysis Set (SLI SAS)	All enrolled participants who were randomized in the SLI and receive ≥1 dose of study intervention.
PK Concentration Set	All enrolled participants who are in the SAS and have at least 1 analyte concentration.
SLI PK Concentration Set	All enrolled participants who are in the SLI SAS and have at least 1 analyte concentration.
Pharmacodynamic Analysis Set	A subset of the SLI SAS including participants with at least 1 of the pharmacodynamic parameter evaluated at pre and/or post dose.
Biomarker Analysis Set	A subset of the SAS including participants with at least 1 of the biomarkers evaluated at pre and/or post dose.
PRO Analysis Set	All participants in the FAS who completed a baseline (PRO assessment prior to or on first dose of study treatment) and at least one post-baseline PRO assessment.

9.3. Statistical Analyses

The SAP will be developed and finalized before any analyses are performed and will describe the analyses and procedures for accounting for missing, unused, and spurious data. This section is a summary of the planned statistical analyses of the primary and secondary endpoints.

9.3.1. General Considerations

All efficacy analyses will be performed using the FAS or SLI FAS, and all safety analyses will be performed using the SAS or SLI SAS.

In general, descriptive summaries will be presented for the efficacy and safety variables collected. Continuous variables will be summarized using mean, standard deviation, minimum, median, and maximum. Categorical variables will be summarized using frequency counts and percentages.

All primary and secondary endpoints based on radiological assessments of tumor burden (ie, PFS, OR, DoR, CBR) will be based on programmatically derived version of local

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radiologist's/investigator's assessment (the target lesion measurements, non-target lesion status, and new lesions status recorded on the CRF) per RECIST v1.1.

9.3.2. SLI Primary and Secondary Endpoints

All efficacy analyses will be performed using the SLI FAS, and all safety analyses will be performed using the SLI SAS.

9.3.2.1. SLI Primary Endpoints

Endpoint	Statistical Analysis Methods
Incidence of Grade 4 neutropenia AE with onset within the first 4 cycles.	Defined as the number of SLI SAS participants with Grade 4 neutropenia AE with onset in the SLI observation period divided by the number of SLI SAS participants. Neutropenia will be reported as a cluster term including the MedDRA PTs neutropenia and neutrophils count decrease.
Incidence of dose reduction or drug discontinuation of any study drugs due to any cause within the first 4 cycles.	Incidence of dose reduction is defined as the number of SLI SAS participants reducing a dose level due to any cause occurring during the SLI observation period divided by the number of SLI SAS participants. Dose reduction corresponds to palbociclib reduced from 100 mg to 75 mg (Section 6.6.1.2.1) or vepdegestrant reduced from 200 mg to 100 mg (Section 6.6.1.1). Incidence of drug discontinuation is defined as the number of SLI SAS participants discontinuing palbociclib or vepdegestrant due to any cause occurring during the SLI observation period divided by the number of SLI SAS participants.

Incidence of neutropenia by PTs, as well as all AEs, with onset in the SLI observation period will be reported. Exposure compliance and dose modifications of both study drugs will be reported by treatment arm.

9.3.2.2. SLI Secondary Efficacy Endpoints

All secondary endpoints based on radiological assessments of tumor burden (ie, OR, DoR, CBR) will be based on programmatically derived version of local radiologist's/investigator's assessment (the target lesion measurements, non-target lesion status, and new lesions status recorded on the CRF) per RECIST v1.1.

Endpoint	Statistical Analysis Methods
ORR, defined as the proportion of participants in the FAS with a confirmed objective response (CR or PR) by investigator assessment per RECIST v1.1 criteria.	Point estimates of ORR will be calculated along with the 2-sided 95% CIs using the Wilson Score method. The point estimate of the difference of the rates between treatment arm will be provided along with corresponding approximate 2-sided 95% confidence intervals based on normal distribution. The analysis for ORR will be performed using all tumor assessments available at the time of the SLI primary analysis.

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Endpoint	Statistical Analysis Methods
DoR, defined as the time from the first documentation of CR or PR to the date of the first documentation of PD as determined by investigator assessment per RECIST v1.1, or death due to any cause, whichever occurs first.	DoR will be calculated for the subgroup of participants with confirmed objective tumor response (CR/PR). DoR will be summarized by treatment arm using the Kaplan-Meier methods and displayed graphically where appropriate. The median DoR and 95% CI for the median will be provided for each treatment arm, as well as DoR rates at different time points. The CIs for the median will be calculated according to (Brookmeyer & Crowley, 1982) and the CIs for the survival function estimates at the time points will be derived using a log-log transformation (with back transformation to a CI on the untransformed scale) (Kalbfleisch & Prentice, 2002). The estimate of the standard error will be computed using Greenwood's formula.
CBR, defined as the proportion of participants in the FAS with a confirmed CR or PR at any time, or SD or non-CR/non-PD ≥24 weeks by investigator assessment per RECIST v 1.1.	The proportion of participants achieving CBR will be calculated along with the corresponding 2-sided 95% confidence interval calculated using the Wilson score method. The point estimate of the difference of the rates between treatment arm will be provided along with corresponding approximate 2-sided 95% confidence intervals based on normal distribution.

9.3.3. Phase 3 Primary and Secondary Endpoints

The Phase 3 portion of the study will not be conducted.

9.3.3.1. Primary Endpoint

The primary endpoint is PFS which is defined as the time from the date of randomization to the date of the first documentation of objective PD assessed by BICR per RECIST v1.1, or death due to any cause, whichever occurs first.

Sensitivity analyses for PFS (including PFS assessed by investigators) will be specified in the SAP.

Endpoint	Statistical Analysis Methods
PFS by BICR, defined as the time from the date of randomization to the date of the first documented disease progression, as determined by BICR assessment per RECIST v1.1, or death due to any cause, whichever occurs first.	PFS will be censored as outlined in Table 12, and calculated in months as: PFS (months) = (date of event or censoring – date of randomization +1) / 30.4375 A stratified log-rank test (1-sided) stratified by randomization stratification factors will be used to compare PFS time between the 2 treatment arms at the interim and/or final analyses with the overall significance level preserved at 0.025 (1-sided). The stratified Cox's proportional hazards model stratified by randomization stratification factors will be fitted to estimate the treatment hazard ratio and the corresponding 95% CI. In order to account for the

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Endpoint	Statistical Analysis Methods
	group sequential design, the RCI method (Jennison, 2000), will be used to construct the 2-sided RCIs for the hazard ratio at the interim and/or the final analyses of PFS. In addition, the unadjusted 95% CIs for the hazard ratio will also be reported at the interim and/or the final analyses for PFS. Kaplan-Meier estimates (product-limit estimates) will be presented by treatment arm together with a summary of associated statistics including the median PFS time with 2-sided 95% CIs. PFS rates at different time points will be estimated with corresponding 2-sided 95% CIs. The CIs for the median will be calculated according to (Brookmeyer & Crowley, 1982) and the CIs for the survival function estimates at the time points will be derived using a log-log transformation (with back transformation to a CI on the untransformed scale) (Kalbfleisch & Prentice, 2002). The estimate of the standard error will be computed using Greenwood's formula. If a large departure from the proportional hazards assumption is identified, then PFS will also be analyzed based on RMST differences. Sensitivity analyses for PFS including PFS assessed by investigator will be specified in the SAP.

9.3.3.2. Phase 3 Secondary Endpoint(s)/Estimands Analysis

See Section 9.3.6 for the analyses of safety endpoint.

9.3.3.3. Key Secondary Efficacy Endpoint

Endpoint	Statistical Analysis Methods
OS, defined as the time from date of randomization to date of death due to any cause.	OS will be calculated in months as: OS (months) = (date of death or censoring – date of randomization +1) / 30.4375 A stratified log-rank test (1-sided) stratified by randomization stratification factors will be used to compare OS time between the 2 treatment arms at the interim and/or final analyses with the overall significance level preserved at 0.025 (1-sided). To control the overall Type-I error rate at α=0.025 (1-sided) for the analyses of PFS and OS, a gate-keeping hierarchical testing approach will be used, whereby PFS will be formally tested first. If it is significant, then OS will be tested at an overall 1-sided 0.025 level of significance. The stratified Cox's proportional hazards model stratified by randomization stratification factors will be fitted to estimate the treatment hazard ratio and the corresponding 95% CI. In order to account for the group sequential design, the RCI method (Jennison, 2000), will be used to construct the 2-sided RCIs for the hazard ratio at the interim and/or the final analyses of OS. In addition, the unadjusted 95% CIs for the hazard ratio will also be reported at the interim and/or the final analyses for OS.

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Endpoint	Statistical Analysis Methods
	Kaplan-Meier estimates (product-limit estimates) will be presented by treatment arm together with a summary of associated statistics including the median OS time with 2-sided 95% CIs. OS rates at different time points will be estimated with corresponding 2-sided 95% CIs. The CIs for the median will be calculated according to (Brookmeyer & Crowley, 1982) and the CIs for the survival function estimates at the time points will be derived using a log-log transformation (with back transformation to a CI on the untransformed scale) (Kalbfleisch & Prentice, 2002). The estimate of the standard error will be computed using Greenwood's formula. If treatment-switching problem is identified, then supportive analyses by IPCW may be conducted. Other alternative methods such as RPSFT models may also be explored in this case. If a large departure from the proportional hazards assumption is identified, then OS will also be analyzed based on RMST differences. Sensitivity analyses for OS will be specified in the SAP.

9.3.3.4. Secondary Efficacy Endpoints

Endpoint	Statistical Analysis Methods
ORR, defined as the proportion of participants in the FAS with a confirmed objective response (CR or PR) by BICR assessment per RECIST v1.1 criteria.	Point estimates of ORR will be calculated along with the 2-sided 95% CIs using the Wilson Score method. The odds ratio and the corresponding 2-sided 95% confidence interval will be calculated to contrast the treatment effects on ORR. A Pearson test (unstratified) and CMH test stratified by randomization stratification factors will be used to compare ORR between the 2 treatment arms. The above analysis for ORR will also be done using the subset of patients with measurable disease.
DoR, defined as the time from the first documentation of CR or PR to the date of the first documentation of PD as determined by BICR assessment per RECIST v1.1, or death due to any cause, whichever occurs first.	DoR will be calculated for the subgroup of participants with confirmed objective tumor response (CR/PR). DoR will be summarized by treatment arm using the Kaplan-Meier methods and displayed graphically where appropriate. The median DoR and 95% CI for the median will be provided for each treatment arm, as well as DoR rates at different time points. The CIs for the median will be calculated according to (Brookmeyer & Crowley, 1982) and the CIs for the survival function estimates at the time points will be derived using a log-log transformation (with back transformation to a CI on the untransformed scale) (Kalbfleisch & Prentice, 2002). The estimate of the standard error will be computed using Greenwood's formula.
CBR, defined as the proportion of participants in the FAS with a confirmed CR or PR at any time, or SD or non-CR/non-	The proportion of participants achieving CBR will be calculated along with the corresponding 2-sided 95% confidence interval calculated using the Wilson score method. The odds ratio and the corresponding 2-sided 95% confidence interval will be calculated to contrast the treatment effects on CBR. A Pearson test (unstratified) and CMH test stratified by

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Endpoint	Statistical Analysis Methods
PD ≥24 weeks by BICR assessment per RECIST v 1.1.	randomization stratification factors will be used to compare CBR rate between the 2 treatment arms.
QoL	The following breast cancer-specific QoL measurements will be collected in this study: EuroQol EQ-5D-5L, EORTC QLQ-C30, and EORTC QLQ-BR23 (female and male questionnaire versions). Breast cancer-specific quality of life mean scores at each assessment time point and change from baseline scores will be compared between the treatment arms at various time points using a MMRM approach adjusting for specified covariates. In addition, analyses will be performed to determine if the change from baseline scores achieve the appropriate MID cut-off for the scale being examined. Participants who completed a baseline assessment and at least one post baseline assessment will be considered evaluable for the PRO analysis. In addition to the above analyses, an examination of the TTD also will be carried out using survival analysis methods. A composite definition for deterioration based on death, tumor progression, and MID will be used.

9.3.3.5. ctDNA Analysis

Analysis summaries of baseline levels, changes from baseline (as appropriate), and their association with efficacy outcome will be reported, by appropriate descriptive, graphical, and statistical methods.

9.3.4. Pharmacokinetic Analysis

Plasma concentration data of vepdegestrant (ARV-471), its epimer ARV-473, and palbociclib will be listed, summarized, and plotted for all participants in the PK concentration set as defined in Section 9.2. For summary statistics and mean/median plots by collection time/day, the nominal PK sample collection time/day will be used; for individual participant plots by time/day, the actual PK collection time/day will be used. PK sampling deviations from the protocol-specified collection time/day will be listed.

Population pharmacokinetic analysis of samples collected in this study will be performed in accordance with the FDA guidance on Population Pharmacokinetics (FDA, 2022). The PK concentration data set from this study may be pooled with datasets from other vepdegestrant clinical studies. In addition, population PK/PD modeling will be attempted to investigate any causal relationship between vepdegestrant and/or ARV-473 exposure and biomarker, safety, efficacy, and/or laboratory data. These modeling analyses will be reported separately from the final Clinical Study Report.

9.3.5. Exploratory Endpoints Analysis

9.3.5.1. Pharmacodynamic Endpoints (SLI only)

Correlation of surrogate biomarker involved in cell proliferation (eg, TK1) with exposure of vepdegestrant in combination with palbociclib will be analyzed and may be included in PK modelling.

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9.3.5.2. Biomarker Endpoints (Phase 3 only)

The Phase 3 portion of the study will not be conducted.

Biomarker data, including but not limited to DNA, RNA, and protein from analyses of tumor tissues and/or blood samples will be analyzed. Summaries of baseline levels, changes from baseline (where appropriate), and mutation status will be reported. For continuous variables, summary statistics may include the mean, standard deviation, median, 25th and 75th quartile, minimum and maximum levels of biomarker measures; for categorical variables, summary may include number and percentage, odds ratio as appropriate.

Exploratory graphical methods and statistical methods such as linear regression, t-test, and ANOVA may be used to investigate any possible relationship of biomarker levels and status and/or changes from baseline with vepdegestrant anti-tumor efficacy and other clinical outcome. Results of such analyses may not be included in the CSR.

9.3.6. Safety Analyses

All safety analyses will be performed on the Safety Analysis Set.

AEs, ECGs, BP, HR, and safety laboratory data will be reviewed and summarized on an ongoing basis during the study to evaluate the safety of participants. Any clinical laboratory, ECG, BP, and HR abnormalities of potential clinical concern will be described. Safety data will be presented in tabular and/or graphical format and summarized descriptively, where appropriate.

Medical history and physical examination and neurological examination information, as applicable, collected during the course of the study will be considered source data and will not be required to be reported, unless otherwise noted. However, any untoward findings identified on physical and/or neurological examinations conducted during the active collection period will be captured as AEs, if those findings meet the definition of an AE. Data collected at screening that are used for inclusion/exclusion criteria, such as laboratory data and ECGs, will be considered source data, and will not be required to be reported, unless otherwise noted. Demographic data collected at screening will be reported.

9.3.6.1. Electrocardiogram Analyses

The analysis of ECG results will be based on participants in the safety analysis set. In the SLI, the triplicate data will be averaged, and all summary statistics and data presentations will use the triplicate averaged data. Any data obtained from ECGs repeated for safety reasons after the nominal time-points will not be averaged along with the preceding triplicates.

ECG measurements (an average of triplicate measurements in the SLI) will be used for the statistical analysis and all data presentations. Interval measurements from repeated ECGs will be included in the outlier analysis (categorical analysis) as individual values obtained at unscheduled time points.

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QT intervals will be corrected for heart rate (ie, QTc) using standard correction factors (ie, Fridericia's (default correction; QTcF), and possibly a study specific factor, as appropriate). Data will be summarized and listed for QT, HR, QRS, QTc, and by study arm.

Descriptive statistics (n, mean, median, standard deviation, minimum, and maximum) will be used to summarize the absolute value of the QTc and changes from baseline in QTc after treatment by study arm. Categorical analysis will be conducted for the maximum change from baseline in QTc and the maximum post baseline QTc.

Shift tables will be provided for baseline versus worst on-treatment corrected QT. Shift tables will also be provided for ECG abnormality at baseline vs. on treatment. Participants experiencing clinically relevant morphological ECG changes will be summarized (including frequency and percentage).

9.3.6.2. Adverse Events

AEs will be graded by the investigator according to the NCI CTCAE v5.0 and coded using MedDRA. AE data will be reported in tables and listings. Simple summary statistics (descriptive) will be presented for participants with SAEs, AEs of special interest, laboratory test abnormalities and other secondary safety endpoints during the on-treatment period (defined as the time from the first dose of study treatment through minimum of 28 days + last dose of study treatment) unless otherwise described in the SAP.

9.3.6.3. Laboratory Test Abnormalities

The laboratory results will be graded according to the NCI CTCAE v5.0 severity grade. The number and percentage of participants who experienced laboratory test abnormalities will be summarized according to worst toxicity grade observed for each laboratory assay. The analyses will summarize laboratory tests on the entire study period. For laboratory tests without NCI CTCAE grade definitions, results will be categorized as normal, abnormal, or not done.

9.3.7. Other Analyses

Retained Research Samples for pharmacogenomic analyses may be collected as per SoA and retained for future analyses. The results of such analyses are not planned to be included in the CSR.

9.4. Interim Analyses

For the SLI, no formal interim analysis will be conducted. As the SLI is an open-label study, the sponsor may conduct unblinded reviews of the data during the course of the study for the purpose of safety assessment, and/or supporting clinical development.

The Phase 3 portion of the study will not be conducted.

For the Phase 3, interim analyses may be performed to assess futility and/or efficacy as described below. Interim analysis results may be used for decisions regarding stopping for futility, stopping for early success, conducting a sample size re-estimation, or adapting the

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study after the interim analysis. Participants may be discontinued from the study interventions/study as a result of interim analysis outcome, as described in Section 7.

The interim (IA) and final (FA) analyses for the primary endpoint, PFS, will be performed based on the Full Analysis Set after all planned participants have been randomized and the target number of events has occurred as described below. A maximum of 2 distinct analyses cutoffs for PFS are planned in the study:

- PFS IA (Futility only): at the time when approximately 288 PFS events (approximately 50% information fraction) by investigator assessment have occurred. This is anticipated to occur approximately 25 months after the first participant is randomized.
- PFS FA: at the time when approximately 575 PFS events by investigator assessment have occurred. This is anticipated to occur approximately 44.1 months after the first participant is randomized.

Table 13 displays the maximum number of analyses expected for PFS and the associated futility boundary. The futility boundary is non-binding but the study may be stopped for futility if PFS crosses the corresponding futility boundary at its IA. If PFS achieves statistical significance at its FA, then the primary objective of the study will have been demonstrated.

Table 13. PFS - Futility Boundary and Final Analysis

Analysis	PFS IA	PFS FA
Analysis cutoff trigger	288 PFS events	575 PFS events
Number of events (IF)	288 (50%)	575 (100%)
p-value (z-value) for efficacy	NA	< 0.025 (< -1.96)
p-value (z-value) for futility	> 0.292 (> -0.549)	NA

For PFS the non-binding futility boundary is determined by a Rho(1.5) β-spending function (O'Brien & Fleming, 1979).

The key secondary endpoint, OS, will be hierarchically tested for significance, provided the primary endpoint, PFS, is statistically significant at the final PFS analyses. A maximum of 3 efficacy analyses, based on the Full Analysis Set, are planned for OS:

- OS IA1: at the time of PFS final analysis, provided PFS is statistically significant at the final PFS analysis.

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- OS IA2 and OS sample size re-estimation: at the time when approximately 402 deaths have occurred (approximately 85% information fraction). This is anticipated to occur approximately 56.3 months after the first participant is randomized.
- OS FA: at the time when approximately 473 deaths have occurred. This is anticipated to occur approximately 66.9 months after the first participant is randomized.

If PFS is not significant at the final PFS analysis, OS will not be statistically tested. The formal efficacy boundary for the interim and final analyses of OS will be determined by using the Lan-DeMets (O'Brien-Fleming) α-spending function (O'Brien & Fleming, 1979). As appropriate, the final sample size for testing the OS endpoint may be adjusted using the method outlined by (Cui, Hung & Wang, 1999), with a discretized rather than a continuous sample-size adaptation rule described in (Liu & Hu, 2016) to limit the knowledge that can be inferred to help minimize risks to trial integrity. The FWER for testing the primary and key secondary endpoints will be preserved at or below 0.025 (1-sided test), as discussed in Section 9.1.3.

If the results of the interim analyses indicate serious safety concerns, the sponsor will communicate with the Health Authorities regarding stopping the clinical trial. Any safety evaluation performed at the time of the IAs will be based on the safety analysis set.

The Statistical Analysis Plan will describe the planned analyses in greater detail.

Before any interim analysis is performed, the details of the objectives, decision criteria, dissemination plan, and method of maintaining the study blind (if applicable) as per Pfizer's SOPs will be documented and approved in an E-DMC charter. In addition, the analysis details must be documented and approved in the SAP.

In the event that the E-DMC recommends continuing the trial based on the unblinded interim analysis results, the results will be disclosed together with the results of the primary and secondary endpoint analyses within 1 year after end of study, to maintain the blind while the study is ongoing.

9.5. Sample Size Determination

9.5.1. SLI

The planned sample size of approximately 50 participants (approximately 25 participants per dose level) is based on practical considerations. Assuming a true rate of Grade 4 neutropenia AE of 10%, the upper limit of the 2-sided 80% uncorrected Wilson score interval for 25 participants is 20.3%, approximately 10% increase.

9.5.2. Phase 3

The Phase 3 portion of the study will not be conducted.

A sufficient number of participants will be screened to achieve approximately 1130 participants randomly assigned in a 1:1 ratio to 1 of the 2 study arms (ie, approximately

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565 participants per arm), stratified by menopausal status at study entry (pre-/peri- [including men] vs post-), visceral disease: (yes/no), and de novo metastatic disease (yes/no).

The following statistical hypothesis will be tested to address the primary objective:

$$H_{01}: HR_{PFS} \geq 1 \text{ vs } H_{11}: HR_{PFS} < 1$$

where HR_{PFS} is the hazard ratio for PFS for Arm A vs Arm B.

The following statistical hypothesis will be tested to address the key secondary objective:

$$H_{02}: HR_{OS} \geq 1 \text{ vs. } H_{12}: HR_{OS} < 1$$

where HR_{OS} is the hazard ratio for OS for Arm A vs Arm B. The statistical hypotheses will be tested in a gate-keeping approach at the time of the PFS FA, whereby PFS will be formally tested first. If it is significant, then OS will be tested at an overall 1-sided 0.025 level of significance.

For the primary PFS comparison, a total of 575 PFS events will provide approximately 90% power to detect a target hazard ratio of 0.758 using a 1-sided log-rank test with a 0.025 significance level, and a 2-look group sequential design with a Rho(1.5) β-spending function to determine the non-binding futility boundary (O'Brien & Fleming, 1979).

For the OS comparison, a gate-keeping approach will be used at PFS FA to allow further testing of the key secondary endpoint of OS. If H_{01} is rejected, a total of 473 OS events will provide approximately 80% power to detect an HR of 0.77 using a 1-sided log-rank test with a 0.025 significance level, and a 3-look group sequential design with Lan-DeMets (O'Brien-Fleming) α-spending function to determine the efficacy boundaries (O'Brien & Fleming, 1979).

The sample size calculation is based on the following assumptions:

- The median PFS for participants receiving letrozole in combination with palbociclib is 25 months and the median PFS for participants receiving vepdegestrant in combination with palbociclib is 33 months; this corresponds to an HR of 0.758 under the exponential model assumption.
- The median OS for participants receiving letrozole in combination with palbociclib is 60 months and the median OS for participants receiving vepdegestrant in combination with palbociclib is 78 months; this corresponds to an HR of 0.77 under the exponential model assumption.
- Non-uniform participant accrual over 20 months.
- PFS drop-out rate of 15% by 44 months, and OS drop-out rate of 5% by 67 months.

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10. SUPPORTING DOCUMENTATION AND OPERATIONAL CONSIDERATIONS

10.1. Appendix 1: Regulatory, Ethical, and Study Oversight Considerations

10.1.1. Regulatory and Ethical Considerations

This study will be conducted in accordance with the protocol and with the following:

- Consensus ethical principles derived from international guidelines, including the Declaration of Helsinki and CIOMS International Ethical Guidelines;
- Applicable ICH GCP guidelines;
- Applicable laws and regulations, including applicable privacy laws.

The protocol, protocol amendments, ICD, SRSD(s), and other relevant documents (eg, advertisements) must be reviewed and approved by the sponsor, submitted to an IRB/EC by the investigator, and reviewed and approved by the IRB/EC before the study is initiated.

Any amendments to the protocol will require IRB/EC approval before implementation of changes made to the study design, except for changes necessary to eliminate an immediate hazard to study participants.

Protocols and any substantial amendments to the protocol will require health authority approval prior to initiation except for changes necessary to eliminate an immediate hazard to study participants.

The investigator will be responsible for the following:

- Providing written summaries of the status of the study to the IRB/EC annually or more frequently in accordance with the requirements, policies, and procedures established by the IRB/EC.
- Notifying the IRB/EC of SAEs or other significant safety findings as required by IRB/EC procedures.
- Providing oversight of the conduct of the study at the site and adherence to requirements of 21 CFR, ICH GCP guidelines, the IRB/EC, European regulation 536/2014 for clinical studies, European Medical Device Regulation 2017/745 for clinical device research, and all other applicable local regulations.

10.1.1.1. Reporting of Safety Issues and Serious Breaches of the Protocol or ICH GCP

In the event of any prohibition or restriction imposed (ie, clinical hold) by an applicable regulatory authority in any area of the world, or if the investigator is aware of any new information that might influence the evaluation of the benefits and risks of the study intervention, Pfizer should be informed immediately.

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In addition, the investigator will inform Pfizer immediately of any urgent safety measures taken by the investigator to protect the study participants against any immediate hazard, and of any serious breaches of this protocol or of the ICH GCP that the investigator becomes aware of.

10.1.2. Financial Disclosure

Investigators and subinvestigators will provide the sponsor with sufficient, accurate financial information as requested to allow the sponsor to submit complete and accurate financial certification or disclosure statements to the appropriate regulatory authorities. Investigators are responsible for providing information on financial interests during the course of the study and for 1 year after completion of the study.

10.1.3. Informed Consent Process

The investigator or qualified designee will explain the nature of the study, including the risks and benefits, to the participant and answer all questions regarding the study. The investigator may not delegate obtaining consent to a local healthcare professional.

The participant should be given sufficient time and opportunity to ask questions and to decide whether or not to participate in the trial.

Participants must be informed that their participation is voluntary. Participants (if allowed by local regulations) will be required to sign a statement of informed consent that meets the requirements of 21 CFR 50, local regulations, ICH GCP guidelines, privacy and data protection requirements, where applicable, and the IRB/EC or study center.

The investigator must ensure that each study participant is fully informed about the nature and objectives of the study, the sharing of data related to the study, and possible risks associated with participation, including the risks associated with the processing of the participant's personal data.

The participant must be informed that their personal study-related data will be used by the sponsor in accordance with local data protection law. The level of disclosure must also be explained to the participant.

The participant must be informed that their medical records may be examined by Clinical Quality Assurance auditors or other authorized personnel appointed by the sponsor, by appropriate IRB/EC members, and by inspectors from regulatory authorities.

The investigator further must ensure that each study participant is fully informed about his or her right to access and correct their personal data and to withdraw consent for the processing of his or her personal data.

The medical record must include a statement that written informed consent was obtained before the participant was enrolled in the study and the date on which the written consent was obtained. The authorized person obtaining the informed consent must also sign the ICD.

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Participants must be reconsented to the most current IRB/EC approved version of the ICD(s) during their participation in the study as required per local regulations.

A copy of the ICD(s) must be provided to the participant (if allowed by local regulations).

Participants who are rescreened are required to sign a new ICD.

10.1.4. Data Protection

All parties will comply with all applicable laws, including laws regarding the implementation of organizational and technical measures to ensure protection of participant data.

Participants' personal data will be stored at the study site in encrypted electronic and/or paper form and will be password-protected or secured in a locked room to ensure that only authorized study staff have access. The study site will implement appropriate technical and organizational measures to ensure that the personal data can be recovered in the event of disaster. In the event of a potential personal data breach, the study site will be responsible for determining whether a personal data breach has in fact occurred and, if so, providing breach notifications as required by law.

To protect the rights and freedoms of participants with regard to the processing of personal data, participants will be assigned a single, participant-specific numerical code. Any participant records or data sets that are transferred to the sponsor will contain the numerical code; participant names will not be transferred. All other identifiable data transferred to the sponsor will be identified by this single, participant-specific code. The study site will maintain a confidential list of participants who participated in the study, linking each participant's numerical code to their actual identity and medical record ID. In case of data transfer, the sponsor will protect the confidentiality of participants' personal data consistent with the clinical study agreement and applicable privacy laws.

Information technology systems used to collect, process, and store study-related data are secured by technical and organizational security measures designed to protect such data against accidental or unlawful loss, alteration, or unauthorized disclosure or access.

The sponsor maintains standard operating procedures on how to respond in the event of unauthorized access, use, or disclosure of sponsor information or systems.

10.1.5. Committees Structure

10.1.5.1. Data Monitoring Committee

The Phase 3 portion of the study will not be conducted.

This study will use an E-DMC in Phase 3. The E-DMC is independent of the study team and includes only external members. Data collected in the SLI will be provided to the E-DMC. The E-DMC charter describes the role of the E-DMC in more detail.

The E-DMC will be responsible for ongoing monitoring of the efficacy and safety of participants in the study according to the charter. The recommendations made by the E-DMC

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will be forwarded to the appropriate authorized Pfizer personnel for review and final decision. Pfizer will communicate such decisions, which may include summaries of aggregate analyses of safety data, to regulatory authorities and investigators, as appropriate.

10.1.5.2. Global Steering Committee

A Global Steering Committee, composed by external members who are experts in the treatment of breast cancer, will be established at the vepdegestrant program level. It will be responsible for providing objective strategic scientific advice and medical input on the ongoing and future clinical development plans for vepdegestrant in ER(+) breast cancer including for this study.

10.1.6. Dissemination of Clinical Study Data

Pfizer fulfills its commitment to publicly disclose clinical study results through posting the results of studies on www.clinicaltrials.gov (ClinicalTrials.gov), the EudraCT/CTIS, and/or www.pfizer.com, and other public registries and websites in accordance with applicable local laws/regulations. In addition, Pfizer reports study results outside of the requirements of local laws/regulations pursuant to its SOPs.

In all cases, study results are reported by Pfizer in an objective, accurate, balanced, and complete manner and are reported regardless of the outcome of the study or the country in which the study was conducted.

<u>www.clinicaltrials.gov</u>

Pfizer posts clinical trial results on www.clinicaltrials.gov for Pfizer-sponsored interventional studies (conducted in patients) that evaluate the safety and/or efficacy of a product, regardless of the geographical location in which the study is conducted. These results are submitted for posting in accordance with the format and timelines set forth by US law.

<u>EudraCT/CTIS</u>

Pfizer posts clinical trial results on EudraCT/CTIS for Pfizer-sponsored interventional studies in accordance with the format and timelines set forth by EU requirements following the end of the study globally.

<u>www.pfizer.com</u>

Pfizer posts CSR synopses and plain-language study results summaries on www.pfizer.com for Pfizer-sponsored interventional studies at the same time the corresponding study results are posted to www.clinicaltrials.gov. CSR synopses will have personally identifiable information anonymized.

<u>Documents within marketing applications</u>

Pfizer complies with applicable local laws/regulations to publish clinical documents included in marketing applications. Clinical documents include summary documents and CSRs

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including the protocol and protocol amendments, sample CRFs, and SAPs. Clinical documents will have personally identifiable information anonymized.

Data Sharing

Pfizer provides researchers secure access to participant-level data or full CSRs for the purposes of "bona-fide scientific research" that contributes to the scientific understanding of the disease, target, or compound class. Pfizer will make data from these trials available 18 months after study completion. Participant-level data will be anonymized in accordance with applicable privacy laws and regulations. CSRs will have personally identifiable information anonymized.

Data requests are considered from qualified researchers with the appropriate competencies to perform the proposed analyses. Research teams must include a biostatistician. Data will not be provided to applicants with significant conflicts of interest, including individuals requesting access for commercial/competitive or legal purposes.

10.1.7. Data Quality Assurance

All participant data relating to the study will be recorded on printed or electronic CRF unless transmitted to the sponsor or designee electronically (eg, laboratory data). The investigator is responsible for verifying that data entries are accurate and correct by physically or electronically signing the CRF.

Guidance on completion of CRFs will be provided in the CRF Completion Requirements document.

The investigator must ensure that the CRFs are securely stored at the study site in encrypted electronic and/or paper form and are password-protected or secured in a locked room to prevent access by unauthorized third parties.

QTLs are predefined parameters that are monitored during the study. Important deviations from the QTLs and any remedial actions taken will be summarized in the CSR.

The investigator must permit study-related monitoring, audits, IRB/EC review, and regulatory agency inspections and provide direct access to source records and documents. This verification may also occur after study completion. It is important that the investigator(s) and their relevant personnel are available during the monitoring visits and possible audits or inspections and that sufficient time is devoted to the process.

Monitoring details describing strategy, including definition of study-critical data items and processes (eg, risk-based initiatives in operations and quality, such as risk management and mitigation strategies and analytical risk-based monitoring), methods, responsibilities, and requirements, including handling of noncompliance issues and monitoring techniques (central, virtual, or on-site monitoring), are provided in the data management plan and monitoring plan maintained and utilized by the sponsor or designee.

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The sponsor or designee is responsible for the data management of this study, including quality checking of the data.

Records and documents, including signed ICDs, pertaining to the conduct of this study must be retained by the investigator for 15 years after study completion unless local regulations or institutional policies require a longer retention period. No records may be destroyed during the retention period without the written approval of the sponsor. No records may be transferred to another location or party without written notification to the sponsor. The investigator must ensure that the records continue to be stored securely for as long as they are maintained.

When participant data are to be deleted, the investigator will ensure that all copies of such data are promptly and irrevocably deleted from all systems.

The investigator(s) will notify the sponsor or its agents immediately of any regulatory inspection notification in relation to the study. Furthermore, the investigator will cooperate with the sponsor or its agents to prepare the investigator site for the inspection and will allow the sponsor or its agent, whenever feasible, to be present during the inspection. The investigator site and investigator will promptly resolve any discrepancies that are identified between the study data and the participant's medical records. The investigator will promptly provide copies of the inspection findings to the sponsor or its agent. Before response submission to the regulatory authorities, the investigator will provide the sponsor or its agents with an opportunity to review and comment on responses to any such findings.

10.1.8. Source Documents

Source documents provide evidence for the existence of the participant and substantiate the integrity of the data collected. Source documents are filed at the investigator site.

Data reported on the CRF or entered in the eCRF that are from source documents must be consistent with the source documents or the discrepancies must be explained. The investigator may need to request previous medical records or transfer records, depending on the study. Also, current medical records must be available.

Definition of what constitutes source document and its origin can be found in the Source Document Locator, which is maintained by the sponsor.

Description of the use of the computerized system is documented in the Data Management Plan, which is maintained by the sponsor.

The investigator must maintain accurate documentation (source record) that supports the information entered in the CRF.

The sponsor or designee will perform monitoring to confirm that data entered into the CRF by authorized site personnel are accurate, complete, and verifiable from source documents; that the safety and rights of participants are being protected; and that the study is being

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conducted in accordance with the currently approved protocol and any other study agreements, the ICH GCP guidelines, and all applicable regulatory requirements.

10.1.9. Use of Medical Records

There may be instances when copies of medical records for certain cases are requested by Pfizer Safety, where ethically and scientifically justified and permitted by local regulations, to ensure participant safety.

Due to the potential for a participant to be reidentified from their medical records, the following actions must be taken when medical records are sent to the sponsor or sponsor designee:

- The investigator or site staff must redact personal information from the medical record. The personal information includes, but is not limited to, the following: participant names or initials, participant dates (eg, birth date, date of hospital admission/discharge, date of death), participant identification numbers (eg, Social Security number, health insurance number, medical record number, hospital/institution identifier), participant location information (eg, street address, city, country, postal code, IP address), and participant contact information (eg, telephone/fax number, email address).

- Each medical record must be transmitted to the sponsor or sponsor designee using systems with technical and organizational security measures to ensure the protection of personal data (eg, Florence is the preferred system if available).

- There may be unplanned situations where the sponsor may request medical records (eg, sharing medical records so that the sponsor can provide study-related advice to the investigator). The medical records should be submitted according to the procedure described above.

10.1.10. Study and Site Start and Closure

The study start date is the date of the first participant's first visit.

The sponsor designee reserves the right to close the study site or terminate the study at any time for any reason at the sole discretion of the sponsor, including (but not limited to) regulatory authority decision, change in opinion of the IRB/EC, or change in benefit-risk assessment. Study sites will be closed upon study completion. A study site is considered closed when all required documents and study supplies have been collected and a study-site closure visit has been performed.

The investigator may initiate study-site closure at any time upon notification to the sponsor or designee/CRO if requested to do so by the responsible IRB/EC or if such termination is required to protect the health of study participants.

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Reasons for the early closure of a study site by the sponsor may include but are not limited to:

- Failure of the investigator to comply with the protocol, the requirements of the IRB/EC or local health authorities, the sponsor's procedures, or the ICH GCP guidelines;
- Inadequate recruitment of participants by the investigator;
- Discontinuation of further study intervention development;
- Negative benefit /risk assessment.

If the study is prematurely terminated or suspended, the sponsor shall promptly inform the investigators, the ECs/IRBs, the regulatory authorities, and any CRO(s) used in the study of the reason for termination or suspension, as specified by the applicable regulatory requirements. The investigator shall promptly inform the participant and should assure appropriate participant therapy and/or follow-up.

Study termination is also provided for in the clinical study agreement. If there is any conflict between the contract and this protocol, the contract will control as to termination rights.

10.1.11. Publication Policy

For multicenter trials, the primary publication will be a joint publication developed by the investigator and Pfizer reporting the primary endpoint(s) of the study covering all study sites. The investigator agrees to refer to the primary publication in any subsequent publications. Pfizer will not provide any financial compensation for the investigator's participation in the preparation of the primary congress abstract, poster, presentation, or primary manuscript for the study.

Investigators are free to publish individual center results that they deem to be clinically meaningful after publication of the overall results of the study or 12 months after primary completion date or study completion at all sites, whichever occurs first, subject to the other requirements described in this section.

The investigator will provide Pfizer an opportunity to review any proposed publication or any other type of disclosure of the study results (collectively, "publication") before it is submitted or otherwise disclosed and will submit all publications to Pfizer 30 days before submission. If any patent action is required to protect intellectual property rights, the investigator agrees to delay the disclosure for a period not to exceed an additional 60 days upon request from Pfizer. This allows Pfizer to protect proprietary information and to provide comments, and the investigator will, on request, remove any previously undisclosed confidential information before disclosure, except for any study-intervention or Pfizer-related information necessary for the appropriate scientific presentation or understanding of the study results. For joint publications, should there be disagreement regarding interpretation and/or presentation of specific analysis results, resolution of, and

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responsibility for, such disagreements will be the collective responsibility of all authors of the publication.

For all publications relating to the study, the investigator and Pfizer will comply with recognized ethical standards concerning publications and authorship, including those established by the International Committee of Medical Journal Editors. The investigator will disclose any relationship with Pfizer and any relevant potential conflicts of interest, including any financial or personal relationship with Pfizer, in any publications. When applicable, editorial or technical support provided by a third party and paid for by Pfizer, or provided by a Pfizer employee, may be a reportable transfer of value under the Sunshine Act for US licensed physicians or other healthcare professionals.

All authors will have access to the relevant statistical tables, figures, and reports (in their original format) required to develop the publication.

10.1.12. Sponsor's Medically Qualified Individual

The sponsor will designate a medically qualified individual (MQI, also known as the medical monitor) to advise the investigator on study-related medical questions. The contact information for the study medical monitor is documented in the Study Team Contact List located in the Investigator Site File.

Participants are provided with a Pfizer study information card at the time of informed consent which includes contact information for their investigator in case of study-related medical questions. The study information card contains, at a minimum, (a) study number, (b) participant's study identification number, and (c) principal investigator contact information.

10.1.13. Transfer of Obligation Statement

Arvinas Estrogen Receptor, Inc. ("Arvinas") and Pfizer entered into a collaboration agreement dated as of July 21, 2021, to co-develop the compound vepdegestrant. According to such agreement, Arvinas is the Sponsor for studies involving vepdegestrant conducted in the United States (US) and holds the US Investigational New Drug Application (IND) for such studies. Pfizer is the sponsor for Pfizer-led studies involving vepdegestrant conducted under comparable applications outside of the US and is the acting sponsor for Pfizer-led studies involving vepdegestrant within the US. Arvinas, as the US Sponsor of the IND, is transferring to Pfizer responsibility for all of its sponsor obligations under 21 CFR Part 312 to conduct Study C4891002, including responsibility for preparing the protocol, and overall conduct and oversight of the study, including contracting with study sites. Pfizer will be the "interested party" with regards to US Importer of Record.

For the purposes of the protocol, sponsor here refers to Pfizer.

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10.2. Appendix 2: Clinical Laboratory Assessments

The following safety laboratory tests may be performed at times defined in the SoA section of this protocol. After implementation of Amendment 3, clinical laboratory tests may be performed as clinically indicated and documented in site source but not in the eCRF.

Protocol-specific requirements for inclusion and exclusion of participants are detailed in Section 5.1 and Section 5.2, respectively.

Additional laboratory results may be reported on these samples as a result of the method of analysis or the type of analyzer used by the clinical laboratory, or as derived from calculated values. These additional tests would not require additional collection of blood. Unscheduled clinical laboratory measurements may be obtained at any time during the study to assess any perceived safety issues.

Laboratory assessments
Hematology
Hemoglobin
Platelets
White Blood Cells
Absolute Neutrophils (or % as per local practice)
Chemistry
Alanine aminotransferase
Aspartate aminotransferase
Alkaline Phosphatase
Sodium
Potassium
Magnesium
Total Calcium
Total Bilirubin[a]
BUN or Urea
Serum Creatinine
eGFR as per 2021 CKD-EPI equations (at screening only)
Albumin
Glucose (fasting)
Triglycerides (fasting)
Cholesterol (Total Cholesterol, LDL, HDL) (fasting)
Coagulation (at screening only)
aPTT
INR
PT
Additional Tests
FSH test (at screening only) for female participants under 60 years of age with no menses for 12 months and without an alternative physiological or pathological cause.
Other Tests
HBV, HCV and HIV (at screening only) if required by local regulations
Pregnancy Test
Serum pregnancy test for IOCBP participants required at screening. Following screening, additional pregnancy tests, serum or urine, must have sensitivity of least 25 mIU/mL.

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Laboratory assessments
a. For Hy's Law potential cases, in addition to repeating AST and ALT, laboratory tests should include albumin, creatine kinase, total bilirubin, direct and indirect bilirubin, gamma-glutamyl transferase, prothrombin time (PT)/INR, eosinophils (%) and alkaline phosphatase

The investigator must review the laboratory report, document this review, and record any clinically relevant changes occurring during the study in the AE section of the CRF.

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10.3. Appendix 3: Adverse Events: Definitions and Procedures for Recording, Evaluating, Follow-Up, and Reporting

10.3.1. Definition of AE

AE Definition
• An AE is any untoward medical occurrence in a patient or clinical study participant, temporally associated with the use of study intervention, whether or not considered related to the study intervention. • Note: An AE can therefore be any unfavorable and unintended sign (including an abnormal laboratory finding), symptom, or disease (new or exacerbated) temporally associated with the use of study intervention.

Events <u>Meeting</u> the AE Definition

• Any abnormal laboratory test results (hematology, clinical chemistry, or urinalysis) or other safety assessments (eg, ECG, radiological scans, vital sign measurements), including those that worsen from baseline, considered clinically significant in the medical and scientific judgment of the investigator. Any abnormal test results that meet any of the conditions below must be recorded as an AE:
 	
  • Is associated with accompanying symptoms;
 	
  • Requires additional diagnostic testing or medical/surgical intervention;
 	
  • Leads to a change in study dosing (outside of any protocol-specified dose adjustments) or discontinuation from the study, significant additional concomitant drug treatment, or other therapy.
 	
• Exacerbation of a chronic or intermittent preexisting condition, including an increase in either frequency and/or intensity of the condition.
 	
• New condition detected or diagnosed after study intervention administration, even though it may have been present before the start of the study.
 	
• Signs, symptoms, or the clinical sequelae of a suspected drug-drug interaction.
 	
• Signs, symptoms, or the clinical sequelae of a suspected overdose of either study intervention or a concomitant medication. Overdose per se will not be reported as an AE or SAE unless it is an intentional overdose taken with possible suicidal/self-harming intent. Such overdoses should be reported regardless of sequelae.

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Events NOT Meeting the AE Definition
• Any clinically significant abnormal laboratory findings or other abnormal safety assessments that are associated with the underlying disease, unless judged by the investigator to be more severe than expected for the participant's condition. • The disease/disorder being studied or expected progression, signs, or symptoms of the disease/disorder being studied, unless more severe than expected for the participant's condition. • Worsening of signs and symptoms of the malignancy under study should be recorded as AEs in the appropriate section of the CRF. Disease progression assessed by measurement of malignant lesions on radiographs or other methods should not be reported as AEs. • Medical or surgical procedure (eg, endoscopy, appendectomy): the condition that leads to the procedure is the AE. • Situations in which an untoward medical occurrence did not occur (social and/or convenience admission to a hospital). • Anticipated day-to-day fluctuations of preexisting disease(s) or condition(s) present or detected at the start of the study that do not worsen.

10.3.2. Definition of an SAE

An SAE is defined as any untoward medical occurrence that, at any dose, meets 1 or more of the criteria listed below:
a. Results in death
b. Is life-threatening The term "life-threatening" in the definition of "serious" refers to an event in which the participant was at risk of death at the time of the event. It does not refer to an event that hypothetically might have caused death if it were more severe.
c. Requires inpatient hospitalization or prolongation of existing hospitalization In general, hospitalization signifies that the participant has been detained (usually involving at least an overnight stay) at the hospital or emergency ward for observation and/or treatment that would not have been appropriate in the physician's office or outpatient setting. Complications that occur during hospitalization are AEs. If a complication prolongs hospitalization or fulfills any other serious criteria, the event is serious. When in doubt as to whether "hospitalization" occurred or was necessary, the AE should be considered serious.

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Hospitalization for elective treatment of a preexisting condition that did not worsen from baseline is not considered an AE.
d. Results in persistent or significant disability/incapacity • The term disability means a substantial disruption of a person's ability to conduct normal life functions. • This definition is not intended to include experiences of relatively minor medical significance, such as uncomplicated headache, nausea, vomiting, diarrhea, influenza, and accidental trauma (eg, sprained ankle), that may interfere with or prevent everyday life functions but do not constitute a substantial disruption.
e. Is a congenital anomaly/birth defect.
f. Is a suspected transmission via a Pfizer product of an infectious agent, pathogenic or non-pathogenic. The event may be suspected from clinical symptoms or laboratory findings indicating an infection in a participant exposed to a Pfizer product. The terms "suspected transmission" and "transmission" are considered synonymous. These cases are considered unexpected and handled as serious expedited cases by pharmacovigilance personnel. Such cases are also considered for reporting as product defects, if appropriate.
g. Other situations: • Medical or scientific judgment should be exercised by the investigator in deciding whether SAE reporting is appropriate in other situations, such as significant medical events that but may jeopardize the participant or may require medical or surgical intervention to prevent one of the other outcomes listed in the above definition. These events should usually be considered serious. • Examples of such events include invasive or malignant cancers, intensive treatment in an emergency room or at home for allergic bronchospasm, blood dyscrasias or convulsions that do not result in hospitalization, or development of drug dependency or drug abuse. • Progression of the malignancy under study (including signs and symptoms of progression) should not be reported as an SAE unless the outcome is fatal within the active collection period. Hospitalization due to signs and symptoms of disease progression should not be reported as an SAE. If the malignancy has a fatal outcome during the study, the event leading to death must be recorded as an AE on the CRF, and as an SAE with CTCAE Grade 5 (see the Assessment of Intensity section) if it occurs during the active collection period.

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10.3.3. Recording/Reporting and Follow-Up of AEs and/or SAEs During the Active Collection Period

AE and SAE Recording/Reporting

The table below summarizes the requirements for recording AEs on the CRF and for reporting SAEs to Pfizer Safety throughout the active collection period. These requirements are delineated for 3 types of events: (1) SAEs; (2) nonserious AEs; and (3) exposure to the study intervention under study during pregnancy or breastfeeding, and occupational exposure.

It should be noted that reporting SAE information to Pfizer Safety is not the same as recording on the AE page of the CRF. When the same data are collected, these 2 reporting methods must be completed in a consistent manner. AEs should be recorded using concise medical terminology and the same AE term should be used on both the CRF and the reporting method used for reporting of SAE information to Pfizer Safety.

Safety Event	Recorded on the CRF	Reported to Pfizer Safety	Reported Within 24 Hours of Awareness
SAE	All	All	Yes
Nonserious AE	All	None	No
Exposure to the study intervention under study during pregnancy or breastfeeding	All AEs/SAEs associated with EDP or EDB **Note:** Instances of EDP or EDB not associated with an AE or SAE are not captured in the CRF, unless the study is managed via SAE+	All instances of EDP are reported (whether or not there is an associated SAE) All instances of EDB are reported (whether or not there is an associated SAE)	Yes
Environmental or occupational exposure to the product under study to a non-participant (not involving EDP or EDB)	None. Exposure to a study non-participant is not collected on the CRF	The exposure (whether or not there is an associated AE or SAE) must be reported	Yes

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- When an AE or SAE occurs, it is the responsibility of the investigator to review all documentation (eg, hospital progress notes, laboratory reports, and diagnostic reports) related to the event.
- The investigator will then record all relevant AE or SAE information on the CRF.
- It is **not** acceptable for the investigator to send photocopies of the participant's medical records to Pfizer Safety when the reporting method is paper (eg, in lieu of completion of the CT SAE Report Form/AE or SAE CRF page).
- There may be instances when copies of medical records for certain cases are requested by Pfizer Safety. In this case, all participant identifiers, with the exception of the participant number, will be redacted on the copies of the medical records before submission to Pfizer Safety. Refer to Section 10.1.9 Use of Medical Records for actions that must be taken when medical records are sent to the sponsor or sponsor designee.
- The investigator will attempt to establish a diagnosis of the event based on signs, symptoms, and/or other clinical information. Whenever possible, the diagnosis (not the individual signs/symptoms) will be documented as the AE or SAE.

Assessment of Intensity

- The investigator will make an assessment of intensity for each AE and SAE reported during the study and assign it to 1 of the categories listed below (as defined by the NCI CTCAE system).
- If required on the AE page of the CRF, the investigator will use the following definitions of severity in accordance with the current CTCAE version to describe the maximum intensity of the AE:

GRADE	Clinical Description of Intensity
1	MILD AE
2	MODERATE AE
3	SEVERE AE
4	LIFE-THREATENING; urgent intervention indicated
5	DEATH RELATED TO AE
	An event is defined as "serious" when it meets at least 1 of the predefined outcomes as described in the definition of an SAE, NOT when it is rated as severe.

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Assessment of Causality
• The investigator is obligated to assess the relationship between study intervention and each occurrence of each AE or SAE. The investigator will use clinical judgment to determine the relationship. • A "reasonable possibility" of a relationship conveys that there are facts, evidence, and/or arguments to suggest a causal relationship, rather than a relationship cannot be ruled out. • Once the causality assessment is determined, the investigator must document it in the CRF and, for safety reporting purposes, document the relatedness as "Yes" (for related) or "No" (for unrelated). Alternative causes, such as underlying disease(s), concomitant therapy, and other risk factors, as well as the temporal relationship of the event to study intervention administration, will be considered and investigated. • The investigator will also consult the IB and/or product information, for marketed products, in their assessment. • For each AE or SAE, the investigator **must** document in the medical notes that they have reviewed the AE or SAE and have provided an assessment of causality. • There may be situations in which an SAE has occurred, and the investigator has minimal information to include in the initial report to the sponsor. However, **it is very important that the investigator always make an assessment of causality for every event before the initial transmission of the SAE data to the sponsor.** • The investigator may change their opinion of causality in light of follow-up information and send an SAE follow-up report with the updated causality assessment. • The causality assessment is one of the criteria used when determining regulatory reporting requirements. • If the investigator does not know whether or not the study intervention caused the event, then the event will be handled as "related to study intervention" for reporting purposes, as defined by the sponsor. In addition, if the investigator determines that an SAE is associated with study procedures, the investigator must record this causal relationship in the source documents and CRF, and report such an assessment in the dedicated section of the selected reporting method and in accordance with the SAE reporting requirements.
Outcome Assessment

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Record the appropriate outcome of the event in relation to the participant's status:

- Fatal: The termination of life as a result of an AE.
- Not Recovered/Not Resolved: One of the possible results of an AE outcome that indicates that the event has not improved or recuperated.
- Recovered/Resolved: One of the possible results of an AE outcome that indicates that the event has improved or recuperated.
- Recovered/Resolved with Sequalae: One of the possible results of an AE outcome where the participant recuperated but retained pathological conditions resulting from the prior disease or injury.
- Recovering/Resolving: One of the possible results of an AE outcome that indicates that the event is improving.
- Unknown: Not known, not observed, not recorded, or refused.

Follow-Up of AEs and SAEs

- The investigator is obligated to perform or arrange for the conduct of supplemental measurements and/or evaluations, as medically indicated or as requested by the sponsor, to elucidate the nature and/or causality of the AE or SAE as fully as possible. This may include additional laboratory tests or investigations, histopathological examinations, or consultation with other healthcare providers.
- New or updated information will be recorded in the originally submitted documents.
- The investigator will submit any updated SAE data to the sponsor within 24 hours of receipt of the information.

10.3.4. Reporting of SAEs

SAE Reporting to Pfizer Safety via an Electronic Data Capture

- The primary mechanism for reporting an SAE to Pfizer Safety will be the EDC (eg, eSAE or SAE+) or PSSA.
- For studies managed via SAE+, information pertaining to EDP or EDB is collected in the EDC and sent to Pfizer via PDF file.
- If the electronic system is unavailable, then the site will use the paper SAE report form (see next section) to report the event within 24 hours.

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- The site will enter the SAE data into the EDC or PSSA or paper form (as applicable) as soon as the data become available.
- After the study is completed at a given site, the EDC will be taken off-line to prevent the entry of new data or changes to existing data.
- If a site receives a report of a new SAE from a participant or receives updated data on a previously reported SAE after the EDC has been taken off-line, then the site can report this information on a paper SAE form (see next section) or to Pfizer Safety by telephone.

SAE Reporting to Pfizer Safety via the CT SAE Report Form

- Facsimile transmission of the CT SAE Report Form is the back-up method to transmit this information to Pfizer Safety in case the EDC or PSSA is unavailable for more than 24 hours.
- In circumstances when the facsimile is not working, an alternative method should be used, eg, secured (Transport Layer Security) or password-protected email. If none of these methods can be used, notification by telephone is acceptable with a copy of the CT SAE Report Form sent by overnight mail or courier service.
- Initial notification via telephone does not replace the need for the investigator to complete and sign the CT SAE Report Form pages within the designated reporting time frames.

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10.4. Appendix 4: Contraceptive and Barrier Guidance

10.4.1. Male Participant Reproductive Inclusion Criteria

The SLI Phase enrollment has been completed; enrollment in the Phase 3 portion of the study will not proceed. Overall, no male participants have been enrolled in the study.

Male participants are eligible to participate if they agree to the following requirements during the intervention period and for at least 97 days after the last dose of study intervention, which corresponds to the time needed to eliminate reproductive safety risk of the study intervention(s) ***plus*** an additional 90 days (a spermatogenesis cycle):

- Refrain from donating sperm for at least 97 days after last dose of study intervention.

PLUS either:

- Be abstinent from heterosexual or homosexual intercourse as their preferred and usual lifestyle (abstinent on a long-term and persistent basis) and agree to remain abstinent.

OR

- Must agree to use a male condom when engaging in any activity that allows for passage of ejaculate to another person.

In addition to male condom use, a highly effective method of contraception may be considered in WOCBP partners of male participants.

10.4.2. Female Participant Reproductive Inclusion Criteria

The criteria below are part of Inclusion Criterion #1 (Age and Sex; Section 5.1) and specify the reproductive requirements for including female participants. Refer to Section 10.4.4 for a complete list of contraceptive methods permitted in the study.

A participant is eligible to participate if the participant (a) is not pregnant or breastfeeding; and (b) at least 1 of the following conditions applies:

- Is not an IOCBP (see definition in Section 10.4.3).

OR

- Is an IOCBP and agrees to use a contraceptive method that is highly effective (failure rate of <1% per year) with low user dependency during the intervention period and for at least 21 days after the last dose of study intervention, which corresponds to the time needed to eliminate any reproductive safety risk of the study intervention(s). The investigator should confirm the use of the appropriate contraceptive method in relationship to the first dose of study intervention.

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OR

- Is an IOCBP and agrees to use a highly effective (failure rate of <1% per year) user-dependent method of contraception during the intervention period and for at least 21 days after the last dose of study intervention, which corresponds to the time needed to eliminate any reproductive safety risk of the study intervention(s). The investigator should confirm the use of the appropriate contraceptive method in relationship to the first dose of study intervention.

The investigator is responsible for review of medical history, menstrual history, and recent sexual activity to decrease the risk for inclusion of an individual with an early undetected pregnancy.

10.4.3. Individual of Childbearing Potential

An individual is considered fertile following menarche and until becoming post-menopausal unless permanently sterile (see below).

If fertility is unclear (eg, amenorrhea or oligomenorrhea) and a menstrual cycle cannot be confirmed before the first dose of study intervention, additional evaluation should be considered.

Individuals in the following categories are not considered IOCBP:

1. Pre-menopausal female with 1 of the following:

    - Documented hysterectomy;
    - Documented bilateral salpingectomy;
    - Documented bilateral oophorectomy.

    For individuals with permanent infertility due to a medical cause other than the above (eg, mullerian agenesis, androgen insensitivity), investigator judgement will determine childbearing status and study eligibility.

    Note: Documentation for any of the above categories can come from the site personnel's review of the participant's medical records, medical examination, or medical history interview. The method of documentation should be recorded in the participant's medical record for the study.

2. Post-menopausal individual:

    - A post-menopausal state is defined as no menses for 12 consecutive months without an alternative medical cause. In addition,

        - FSH level in the postmenopausal range (as per the reference range used by the laboratory) must also be used to confirm a post-menopausal state in individuals

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under 60 years old with no alternative physiological (including drugs) or pathological cause and not using hormonal contraception or HRT.

- Individuals whose menopausal status is in doubt will be required to use one of the highly effective nonestrogen hormonal contraception methods.

10.4.4. Contraception Methods

Contraceptive use by participants should be consistent with local availability/regulations regarding the use of contraceptive methods for those participating in clinical trials.

IOCBP participants for which contraception is needed as per Section 10.4.2, must discuss with their investigators the most appropriate contraception method to be implemented.

The following contraceptive methods are appropriate for this study:

Highly Effective Methods That Have Low User Dependency

1. Intrauterine device (Copper IUD). Hormone based IUD is NOT allowed.
2. Bilateral tubal occlusion (eg, bilateral tubal ligation).
3. Vasectomized partner.
   - Vasectomized partner is a highly effective contraceptive method provided that the partner is the sole sexual partner of the IOCBP, and the absence of sperm has been confirmed. If not, an additional highly effective method of contraception should be used. The spermatogenesis cycle is approximately 90 days.
4. Highly Effective Methods That Are User-DependentSexual abstinence
   - Sexual abstinence is considered a highly effective method only if defined as refraining from heterosexual intercourse during the entire period of risk associated with the study intervention. The reliability of sexual abstinence needs to be evaluated in relation to the duration of the study and the preferred and usual lifestyle of the participant.

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10.5. Appendix 5: Genetics

Use/Analysis of DNA

- Genetic variation may impact a participant's response to study intervention, susceptibility to, and severity and progression of disease. Therefore, where local regulations and IRBs/ECs allow, a blood sample will be collected for DNA analysis.

- The scope of the genetic research may be narrow (eg, 1 or more candidate genes) or broad (eg, the entire genome), as appropriate to the scientific question under investigation.

- The samples may be analyzed as part of a multistudy assessment of genetic factors involved in the response to study intervention or study interventions of this class to understand treatments for the disease(s) under study or the disease(s) themselves.

- The results of genetic analyses may be reported in the CSR or in a separate study summary or may be used for internal decision making without being included in a study report.

- The sponsor will store the DNA samples in a secure storage space with adequate measures to protect confidentiality.

- The samples will be retained as indicated:

  - Retained sample~~s~~ will be stored indefinitely or for another period as per local requirements.

- Participants may withdraw their consent for the storage and/or use of their Retained Research Samples at any time by making a request to the investigator; in this case, any remaining material will be destroyed. Data already generated from the samples will be retained to protect the integrity of existing analyses.

- Samples for genetic research will be labeled with a code. The key between the code and the participant's personally identifying information (eg, name, address) will be held securely at the study site.

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10.6. Appendix 6: Liver Safety: Suggested Actions and Follow-Up Assessments

Potential Cases of Drug-Induced Liver Injury

Humans exposed to a drug who show no sign of liver injury (as determined by elevations in transaminases) are termed "tolerators," while those who show transient liver injury but adapt are termed "adaptors." In some participants, transaminase elevations are a harbinger of a more serious potential outcome. These participants fail to adapt and therefore are "susceptible" to progressive and serious liver injury, commonly referred to as DILI. Participants who experience a transaminase elevation above 3 × ULN should be monitored more frequently to determine if they are "adaptors" or are "susceptible."

In the majority of DILI cases, elevations in AST and/or ALT precede T bili elevations (>2 × ULN) by several days or weeks. The increase in T bili typically occurs while AST/ALT is/are still elevated above 3 × ULN (ie, AST/ALT and T bili values will be elevated within the same laboratory sample). In rare instances, by the time T bili elevations are detected, AST/ALT values might have decreased. This occurrence is still regarded as a potential DILI. Therefore, abnormal elevations in either AST OR ALT in addition to T bili that meet the criteria outlined below are considered potential DILI (assessed per Hy's law criteria) cases and should always be considered important medical events, even before all other possible causes of liver injury have been excluded.

The threshold of laboratory abnormalities for a potential DILI case depends on the participant's individual baseline values and underlying conditions. Participants who present with the following laboratory abnormalities should be evaluated further as potential DILI (Hy's law) cases to definitively determine the etiology of the abnormal laboratory values:

- Participants with AST/ALT and T bili baseline values within the normal range who subsequently present with AST OR ALT values ≥3 × ULN AND a T bili value ≥2 × ULN with no evidence of hemolysis and an alkaline phosphatase value <2 × ULN or not available.

For participants with baseline AST **OR** ALT **OR** T bili values above the ULN, the following threshold values are used in the definition mentioned above, as needed, depending on which values are above the ULN at baseline:

- Preexisting AST or ALT baseline values above the normal range: AST or ALT values ≥2 times the baseline values AND ≥3 × ULN; or ≥8 × ULN (whichever is smaller).

- Preexisting values of T bili above the normal range: T bili level increased from baseline value by an amount of ≥1 × ULN **or** if the value reaches ≥3 × ULN (whichever is smaller).

Rises in AST/ALT and T bili separated by more than a few weeks should be assessed individually based on clinical judgment; any case where uncertainty remains as to whether it represents a potential Hy's law case should be reviewed with the sponsor.

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The participant should return to the investigator site and be evaluated as soon as possible, preferably within 48 hours from awareness of the abnormal results. This evaluation should include laboratory tests, detailed history, and physical assessment.

In addition to repeating measurements of AST and ALT and T bili for suspected Hy's law cases, additional laboratory tests should include albumin, CK, direct and indirect bilirubin, GGT, PT/INR, eosinophils (%) and alkaline phosphatase. Consideration should also be given to drawing a separate tube of clotted blood and an anticoagulated tube of blood for further testing, as needed, for further contemporaneous analyses at the time of the recognized initial abnormalities to determine etiology. A detailed history, including relevant information, such as review of ethanol, acetaminophen/paracetamol (either by itself or as a coformulated product in prescription or over-the-counter medications), recreational drug, or supplement (herbal) use and consumption, family history, sexual history, travel history, history of contact with a jaundiced person, surgery, blood transfusion, history of liver or allergic disease, and potential occupational exposure to chemicals, should be collected. Further testing for acute hepatitis A, B, C, D, and E infection, total bile acids, liver imaging (eg, biliary tract), and collection of serum samples for acetaminophen/paracetamol drug and/or protein adduct levels may be warranted.

All cases demonstrated on repeat testing as meeting the laboratory criteria of AST/ALT and T bili elevation defined above should be considered potential DILI (Hy's law) cases if no other reason for the LFT abnormalities has yet been found. **Such potential DILI (Hy's law) cases are to be reported as SAEs, irrespective of availability of all the results of the investigations performed to determine etiology of the LFT abnormalities.**

A potential DILI (Hy's law) case becomes a confirmed case only after all results of reasonable investigations have been received and have excluded an alternative etiology.

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10.7. Appendix 7: Kidney Safety: Monitoring Guidelines

10.7.1. Age-Specific Kidney Function Calculations: Adult Participants: 18 years and Older – 2021 CKD-EPI Equations

eGFR (mL/min/1.73m²)

2021 CKD-EPI Screat Only	**Screat *(mg/dL)***	**Scys *(mg/L)***	**Recommended eGFR Equation[a]**
Female	if ≤0.7	NA	eGFR = 142 × $(\text{Screat}/0.7)^{-0.241}$ × $(1.012)^{\text{Age}}$
Female	if >0.7	NA	eGFR = 142 × $(\text{Screat}/0.7)^{-0.241}$ × $(1.012)^{\text{Age}}$
Male	if ≤0.9	NA	eGFR = 142 × $(\text{Screat}/0.9)^{-0.302}$ × $(0.9938)^{\text{Age}}$
Male	if >0.9	NA	eGFR = 142 × $(\text{Screat}/0.9)^{-0.302}$ × $(0.9938)^{\text{Age}}$
2012 CKD-EPI Scys	**Screat *(mg/dL)***	**Scys *(mg/L)***	**Recommended eGFR equation**
Female	any	if ≤0.8	eGFR = 133× $(\text{Scys}/0.8)^{-0.499}$e×$0.932^{\text{Age}}$
Female	any	if >0.8	eGFR = 133× $(\text{Scys}/0.8)^{-1.328}$×$0.932^{\text{Age}}$
Male	any	if ≤0.8	eGFR = 133× $(\text{Scys}/0.8)^{-0.499}$×$0.9962^{\text{Age}}$
Male	any	if >0.8	eGFR = 133× $(\text{Scys}/0.8)^{-1.328}$×$0.9962^{\text{Age}}$
2021 CKD-EPI Screat-Scys Combined	**Screat *(mg/dL)***	**Scys *(mg/L)***	**Recommended eGFR Equation**
Female	if ≤0.7	if ≤0.8	eGFR = 135 × $(\text{Screat}/0.7)^{-0.219}$ × $(\text{Scys}/0.8)^{-0.323}$ × $(0.9961)^{\text{Age}}$
Female	if ≤0.7	if >0.8	eGFR = 135 × $(\text{Screat}/0.7)^{-0.219}$ × $(\text{Scys}/0.8)^{-0.778}$ × $(0.9961)^{\text{Age}}$
Female	if >0.7	if ≤0.8	eGFR = 135 × $(\text{Screat}/0.7)^{-0.544}$ × $(\text{Scys}/0.8)^{-0.323}$ × $(0.9961)^{\text{Age}}$
Female	if >0.7	if >0.8	eGFR = 135 × $(\text{Screat}/0.7)^{-0.544}$ × $(\text{Scys}/0.8)^{-0.778}$ × $(0.9961)^{\text{Age}}$
Male	if ≤0.9	if ≤0.8	eGFR = 135 × $(\text{Screat}/0.9)^{-0.144}$ × $(\text{Scys}/0.8)^{-0.323}$ × $(0.9961)^{\text{Age}}$
Male	if ≤0.9	if >0.8	eGFR = 135 × $(\text{Screat}/0.9)^{-0.144}$ × $(\text{Scys}/0.8)^{-0.778}$ × $(0.9961)^{\text{Age}}$
Male	if >0.9	if ≤0.8	eGFR = 135 × $(\text{Screat}/0.9)^{-0.544}$ × $(\text{Scys}/0.8)^{-0.323}$ × $(0.9961)^{\text{Age}}$
Male	if >0.9	if >0.8	eGFR = 135 × $(\text{Screat}/0.9)^{-0.544}$ × $(\text{Scys}/0.8)^{-0.778}$ × $(0.9961)^{\text{Age}}$

a. Inker et al, 2021.

10.7.2. Kidney Function Calculator

The sponsor recommends the following online calculators to calculate age- and sex-specific kidney function.

The United States National Kidney Foundation Online Calculators.

- Adults (18 years and above) - 2021 CKD-EPI Creatinine Online Calculator (eGFR): https://www.kidney.org/professionals/KDOQI/gfr_calculator

Investigational sites are responsible to ensure that the correct age- and sex-specific equations are selected if local laboratories are used and that they provide the units used for assessments. Investigators are responsible for the clinical oversight of the participant eligibility process, kidney function calculations, and dose selection and adjustments per study protocol.

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Investigators are encouraged to direct questions or uncertainties regarding kidney function and dosing to the Pfizer study team and medical monitor, if needed.

10.7.3. Adverse Event Grading for Kidney Safety Laboratory Abnormalities

AE grading for decline in kidney function (ie, eGFR or eCrCl or creatinine) will be according to NCI CTCAE criteria for adult participants.

CTCAE term (2017) Criteria	**Grade 1**	**Grade 2**	**Grade 3**	**Grade 4**	**Grade 5**
Acute Kidney Injury (AKI)	N/A	N/A	Hospitalization indicated	Life-threatening consequences: dialysis indicated	Death
AKI: A disorder characterized by acute loss of kidney function (within 2 weeks) and is traditionally classified as pre-renal (low blood flow into kidneys), renal (kidney damage), or post-renal causes (ureteral or bladder outflow obstruction.					
Chronic Kidney Disease (CKD)	eGFR or CrCl <LLN to 60 mL/min/1.73 m² or Proteinuria 2+ present OR Urine protein/ creatinine >0.5	eGFR or CrCl 30 to 59 mL/min/1.73m²	eGFR or CrCl 15 to 29 mL/min/1.73m²	eGFR or CrCl <15 mL/min/1.73m² OR dialysis or renal transplant indicated	Death
CKD: A disorder characterized by gradual and usually permanent loss of kidney function resulting in kidney failure.					
Creatinine increased	>ULN to 1.5 x ULN	>1.5 to 3.0 x baseline OR >1.5 to 3.0 x ULN	>3.0 x baseline OR >3.0 to 6.0 x ULN	>6.0 x ULN	N/A
Creatinine increased: A finding based on laboratory test results that indicate increased levels of creatinine in a biological specimen.					
Proteinuria	Proteinuria 1+ OR Urinary protein ⩾ULN to <1.0 g/24 h	Proteinuria 2+ or 3+ OR Urinary protein 1.0 to <3.5 g/24h	Proteinuria 4+ OR Urinary protein ⩾ 3.5 g/24h	N/A	N/A
Proteinuria: A disorder characterized by laboratory test results that indicate the presence of excessive protein in the urine. It is predominantly albumin, but also globulin.					

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10.8. Appendix 8: ECG Findings of Potential Clinical Concern

ECG Findings That May Qualify as AEs
• Marked sinus bradycardia (rate <40 bpm) lasting minutes. • New PR interval prolongation >280 ms. • New prolongation of QTcF to >480 ms (absolute). • New prolongation of QTcF by ≥60 ms from baseline. • New-onset atrial flutter or fibrillation, with controlled ventricular response rate: ie, rate <120 bpm. • New-onset type I second-degree (Wenckebach) AV block of >30 seconds duration. • Frequent PVCs, triplets, or short intervals (<30 seconds) of consecutive ventricular complexes.
ECG Findings That May Qualify as SAEs

• QTcF prolongation >500 ms.
• New ST-T changes suggestive of myocardial ischemia.
• New-onset LBBB (QRS complex >120 ms).
• New-onset right bundle branch block (QRS complex >120 ms).
• Symptomatic bradycardia.
• Asystole:
  • In awake, symptom-free participants in sinus rhythm, with documented asystolic pause ≥3 seconds or any escape rate <40 bpm, or with an escape rhythm that is below the AV node.
  • In awake, symptom-free participants with atrial fibrillation and bradycardia with 1 or more asystolic pauses of at least 5 seconds or longer.
• Atrial flutter or fibrillation, with rapid ventricular response rate: rapid = rate >120 bpm.
• Sustained supraventricular tachycardia (rate >120 bpm) ("sustained" = short duration with relevant symptoms or lasting >1 minute).

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• Ventricular rhythms >30 second duration, including idioventricular rhythm (heart rate <40 bpm), accelerated idioventricular rhythm (HR >40 bpm to <100 bpm), and monomorphic/polymorphic ventricular tachycardia (HR >100 bpm [such as torsades de pointes]). • Type II second-degree (Mobitz II) AV block. • Complete (third-degree) heart block.
ECG Findings That Qualify as SAEs
• Change in pattern suggestive of new myocardial infarction. • Sustained ventricular tachyarrhythmias (>30 second duration). • Second- or third-degree AV block requiring pacemaker placement. • Asystolic pauses requiring pacemaker placement. • Atrial flutter or fibrillation with rapid ventricular response requiring cardioversion. • Ventricular fibrillation/flutter. • At the discretion of the investigator, any arrhythmia classified as an adverse experience.

The major events of potential clinical concern listed above are recommended as "alerts" or notifications from the core ECG laboratory to the investigator and Pfizer study team, and not to be considered as all-inclusive of what is to be reported as AEs/SAEs.

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10.9. Appendix 9: Concomitant Medications That May Result in DDI

10.9.1. CYP3A inhibitors/inducers

The concurrent use of the following **treatments is prohibited** throughout the duration of the Active Treatment Phase **in this study**.

- **vepdegestrant:** CYP3A4 accounted for 85% of the CYP metabolism of vepdegestrant based on *in vitro* data. Co-administration with drugs that are strong CYP3A inhibitors and inducers may change the plasma concentrations of vepdegestrant in humans.

- **Palbociclib:** palbociclib is primarily metabolized by CYP3A and sulfotransferase (SULT) enzyme SULT2A1. Then the use of strong CYP3A inhibitors and inducers is prohibited throughout the duration of the Active Treatment Phase (Palbociclib [IBRANCE] Investigator's Brochure).

- **Other IMPs**: No DDI is expected with the concurrent use with letrozole.

Prior use of medications, food or herbal supplements that are strong inhibitors of CYP3A must be stopped 7 days before randomization. Strong inducers of CYP3A must be stopped 14 days before randomization.

Examples of prohibited concomitant medications are provided in this Appendix. This is not an all-inclusive list (examples including, but not limited to the drugs provided below).

This list of drugs prohibited for potential DDI concerns with the IMP may be revised during the course of the study with written notification from sponsor to include or exclude specific drugs or drug categories for various reasons (eg, emerging DDI results for the IMP, availability of new information in literature on the DDI potential of other drugs), if the overall benefit: risk assessment is not impacted or if the changes do not significantly impact the safety of participants or the scientific value of the trial.

Site staff should consult with the sponsor or designee with any questions regarding potential DDI.

Investigators should consult the product prescribing information for any other medication used throughout the duration of the Active Treatment Phase for information regarding medication that is prohibited for concurrent use.

Drug Category	Drugs
Strong CYP3A inhibitors*	**Antibiotics:** clarithromycin, josamycin, telithromycin, troleandomycin **Antidepressants:** nefazodone **Antifungals:** itraconazole, ketoconazole, posaconazole, voriconazole **Antiprogestins:** mifepristone

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Drug Category	Drugs
	Antivirals: boceprevir, danoprevir, elvitegravir, ensitrelvir, indinavir, lopinavir, nelfinavir, paritaprevir and ritonavir and (ombitasvir and/or dasabuvir), ritonavir, saquinavir, telaprevir, tipranavir **Calcium channel blocker:** mibefradil **Pharmacokinetic enhancer**: cobicistat **Vasopressin antagonists:** conivaptan **Food products:** grapefruit, grapefruit juice or any product containing grapefruit **Farnesyltransferase inhibitor:** lonafarnib
Strong CYP3A inducers*	**Antibiotics:** rifampin, rifapentine **Anticonvulsants:** carbamazepine, phenytoin **Cystic fibrosis treatments:** lumacaftor **Herbal medications:** St. John's wort

* University of Washington (UW) Drug Interaction Database (DIDB) and Food and Drug Administration (FDA) listings as sources:

- (University of Washington, 2023)
- (FDA, 2020)

10.9.2. Drugs Known to Predispose to Torsade de Pointes or QT interval prolongation

Concurrent use of study interventions with drugs of known risk of causing Torsade de Pointes or QT interval prolongation should be **used with caution** throughout the duration of the Active Treatment Phase **in this study**.

This is not an all-inclusive list (examples including, but not limited to the drugs provided below).

Drug Category	Drugs
Drugs known to predispose to Torsade de Pointes	**Anesthetic**: sevoflurane, propofol **Antianginal**: bepridil **Antiarrhythmic**: ibutilide, dofetilide, dronedarone, flecainide, nifekalant, hydroquinidine, quinidine, disopyramide, amiodarone, sotalol, procainamide **Antibiotic**: azithromycin, ciprofloxacin, clarithromycin, erythromycin, gatifloxacin*, grepafloxacin*, levofloxacin, moxifloxacin, roxithromycin, sparfloxacin* **Antidepressant**: citalopram, escitalopram **Antiemetic**: domperidone, ondansetron **Antifungal**: pentamidine, fluconazole **Antihistamine**: terfenadine*, astemizole* **Antilipemic**: probucol* **Antimalarial**: halofantrine, chloroquine, hydroxychloroquine **Antiparasitic**: meglumine antimoniate

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Drug Category	Drugs
	Antipsychotic: thioridazine, levomepromazine (methotrimeprazine), mesoridazine*, haloperidol, chlorprothixene, levosulpiride, pimozide **Antipsychotic/antiemetic**: chlorpromazine, droperidol, sertindole, sulpiride, sultopride **Cholinesterase inhibitor**: donepezil **GI stimulant**: cisapride* **Muscle relaxant**: terodiline **Opioid agonist**: methadone, levomethadyl acetate* **Phosphodiesterase 3 inhibitor**: anagrelide, cilostazol **Psychedelic**: ibogaine **Vasoconstrictor**: terlipressin **Vasodilator**: papaverine HCl (intra-coronary)

*removed from U.S. market

Adapted from crediblemeds.org (organization that curates the University of Arizona Cancer Center for Education and Research on Therapeutics database: "Risk Categories for Drugs that Prolong QT & induce Torsades de Pointes (TdP) – Known Risk of TdP".

For current list, refer to: https://crediblemeds.org/index.php/druglist

10.9.3. P-gp Sensitive Substrates

vepdegestrant is an *in vitro* inhibitor of P-gp. Treatment with vepdegestrant has potential for increasing exposure of concomitant medications that are sensitive substrates for P-gp transporters. Drugs that are P-gp sensitive substrates may be **used with caution in this study**.

This is not an all-inclusive list (examples including, but not limited to the drugs provided below).

Drug Category*	Drugs
P-gp sensitive substrates	**Anticoagulants**: dabigatran etexilate **Antiarrhythmics**: digoxin/digitoxin **Histamine H-1 receptor antagonists**: fexofenadine

* University of Washington (UW) Drug Interaction Database (DIDB) and Food and Drug Administration (FDA) listings as sources:

- (University of Washington, 2023)
- (FDA, 2020)

10.9.4. CYP3A Sensitive Substrates Whose Minimal Concentration Changes May Lead to Serious Toxicities

CYP3A sensitive substrates whose minimal concentration changes may lead to serious toxicities (formerly known as Narrow Therapeutic Window [NTI]). These drugs should be used **with caution in this study** as palbociclib may increase the drug exposure.

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This is not an all-inclusive list (examples including, but not limited to the drugs provided below).

Drug Category	Drugs*
CYP3A sensitive substrate whose minimal concentration changes may lead to serious toxicities	**Antiarrhythmics**: quinidine **Antipsychotics**: pimozide **Ergot alkaloids**: dihydroergotamine, ergotamine **Immunosuppressant**: cyclosporine, sirolimus, and tacrolimus **Opioids**: alfentanil, fentanyl

* Refer to Palbociclib capsule UK SmPC

10.9.5. PPIs

Co-administration of PPIs may reduce vepdegestrant absorption. Therefore, co-administration of PPIs with vepdegestrant **is not recommended in this study**. If PPI treatment is required, drug intake with a moderate-fat meal (400-800 calories, approximately 35% fat) is recommended.

This is not an all-inclusive list (examples including, but not limited to the drugs provided below).

Drug Category	Drugs*
Proton pump inhibitors (PPIs)	Esomeprazole, lansoprazole, omeprazole, pantoprazole, rabeprazole, dexlansoprazole.

* Refer to corresponding concomitant drugs USPIs or local product prescribing information.

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10.10. Appendix 10: Country-Specific Requirements

10.10.1. European Union

This study will be conducted in compliance with Regulation (EU) No 536/2014. The recruitment plans for each EU Member State concerned are included in the respective Recruitment and Informed Consent Procedure documents.

The sponsor will notify EU Member States concerned of the following:

- Annual safety reports via submission to CTIS
- Any SUSAR via reporting to the EudraVigilance database no later than 7 days or 15 days for a fatal or life-threatening SUSAR or nonfatal or non-life-threatening SUSAR, respectively, after becoming aware of that event. A SUSAR is defined as a serious adverse reaction, the nature, severity, or outcome of which is not consistent with the reference safety information.
- Any unexpected event that affects the benefit-risk profile of the study, but are not SUSARs, no later than 15 days of becoming aware of that event
- Any serious breach, as described in Section 10.1.1.1, no later than 7 days of becoming aware of that breach
- Any urgent safety measure, as described in Section 10.1.1.1, no later than 7 days of the measure being taken
- Any inspection report of a third-country authority concerning the study

Records and documents, including signed ICDs, pertaining to the conduct of this study must be retained by the investigator for 25 years or longer if required by other European Union law.

10.10.2. France

Contrat Unique

1. GCP Training

   Before enrolling any participants, the investigator and any subinvestigators will complete the Pfizer-provided Good Clinical Practice training course ("Pfizer GCP Training") or training deemed equivalent by Pfizer. Any investigators who later join the study will do the same before performing study-related duties. For studies of applicable duration, the investigator and subinvestigators will complete Pfizer GCP Training or equivalent every 3 years during the term of the study, or more often if there are significant changes to the ICH GCP guidelines or course materials.

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2. Study Intervention

   No participants or third-party payers will be charged for study intervention.

3. Urgent Safety Measures

   In addition, the investigator will inform Pfizer immediately of any urgent safety measures taken by the investigator to protect the study participants against any immediate hazard, and of any serious breaches of this protocol or of ICH GCP that the investigator becomes aware of.

4. Termination Rights

   Pfizer retains the right to discontinue development of vepdegestrant at any time.

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10.11. Appendix 11: Response Evaluation Criteria in Solid Tumors (RECIST) v1.1

***Adapted from** (Eisenhauer et al, 2009).*

Categorizing Lesions at Baseline

All sites of disease must be assessed at screening as close as possible to randomization date (≤28 days prior to randomization date unless otherwise specified in Section 8.2.1). If screening assessment is inadequate, subsequent statuses generally should be not evaluable.

Lesions are selected at screening and followed as either target or non-target lesions. All target lesions must be measurable. Any lesions or sites of disease not classified as target lesions including those that are non-measurable are classified as non-target. Definitions of measurable, non-measurable, target, and non-target lesions are provided below.

Measurable Lesions

Measurable lesions (including both nodal and non-nodal) are lesions that can be accurately measured as follows:

- Lesions with longest diameter ≥10 mm in the axial plane (bone lesions are not included except for soft tissue expansile masses arising from bone) when assessed by CT or MRI.
- Lesions with longest diameter ≥20 mm when assessed by chest X-ray
- Superficial lesions with longest diameter ≥10 mm when assessed by caliper
- Malignant lymph nodes with the short axis ≥15 mm. Nodal size is normally reported as 2 dimensions in the plane in which the image is obtained (for CT scan this is almost always the axial plane; for MRI the smaller of the axial, sagittal or coronal will be considered the short axis).

NOTE: Normal nodes: Nodes with short axis <10 mm are considered normal and should not be recorded or followed either as measurable or non-measurable disease.

Non-measurable disease

All other disease including lesions too small to be considered measurable, pleural or pericardial effusions, ascites, bone disease, inflammatory breast disease, leptomeningeal disease, lymphangitic involvement of skin or lung, clinical lesions that cannot be accurately measured with calipers, abdominal masses identified by physical exam that are not measurable by reproducible imaging techniques, nodes with short axis ≥10 mm but <15 mm, disease documented by indirect evidence only (eg, by lab values), or previously radiated lesions that have not progressed, are considered non-measurable.

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Target Lesions

All measurable lesions up to a maximum of 2 lesions per organ, 5 lesions in total, representative of all involved organs, should be identified as target lesions at baseline. Target lesions should be selected on the basis of size (lesions with longest diameter for non-lymph nodes ≥10 mm and nodes with longest short axis ≥15 mm) and suitability for accurate repeated measurements. Measurements are recorded in mm and must be provided for target lesions.

Non-Target Lesions

All non-measurable lesions are non-target. All measurable lesions not identified as target lesions are also included as non-target lesions.

OBJECTIVE STATUS AT EACH EVALUATION

Response is assessed by the investigators, with objective status at each assessment recorded on the IOTA CRF.

Objective Status is to be recorded at each post-baseline evaluation.

Sites of disease should be assessed using the same technique as baseline, including consistent administration of contrast agents (see Section 8.2). If a change needs to be made the case should be discussed with the radiologist and the sponsor to determine if substitution is possible.

If target lesions were present at screening, screening sum of diameters is calculated as the total sum of the longest diameters for non-nodal lesions and short axis for nodal lesions. Sum of diameters at subsequent assessments can be calculated in the same manner using the same set of target lesions.

Response evaluation of target lesions

- **Complete Response (CR):** Complete disappearance of all target lesions, with the exception of nodal disease. All nodes must decrease to normal (short axis <10 mm). All target lesions must be assessed.

- **Partial Response (PR):** At least a 30% decrease from baseline in the sum of diameters of all target lesions. The short diameter is used in the sum for nodal target lesions, while the longest diameter is used in the sum for non-nodal target lesions. All target lesions must be assessed.

- **Progressive Disease (PD)**: At least a 20% increase in the sum of diameters of target measurable lesions above nadir (smallest sum observed considering baseline and all assessments prior to the timepoint under evaluation), with a minimum absolute increase

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of 5 mm relative to nadir. If only a subset of target lesions is assessed, and the sum of the non-missing lesion diameters results in an increase of at least 5 mm and at least 20% above the nadir then the progression criteria will have been met.

- **Stable Disease (SD):** Does not qualify for CR, PR, PD. All target lesions must be assessed.

- **Not Evaluable (NE):** PD has not been documented, and at least one of the following is true:
    - one or more target lesions have not been assessed
    - assessment methods used were inconsistent with those used at baseline (unless considered "interchangeable")
    - one or more target lesions cannot be measured accurately unless due to being too small to measure
    - one or more target lesions were excised or irradiated and have not reappeared or increased.

General considerations on Response Evaluation of Target lesions:

- A target lesion that is irradiated or excised post-screening is no longer measurable and generally should be assessed as Not Evaluable. Increase or reappearance of such lesions should constitute progression.

- For lesions that become necrotic in the center, the longest diameter should still be used in the sum of diameters.

- Measurements for target lesions that become small should continue to be recorded;
    - if the lesion is considered to have disappeared, 0 mm should be recorded;
    - if a lesion is determined to be present but too small to measure (ie, lesion less than 10 mm with an exact measurement that cannot be performed), the lesion status will indicate "too small to measure" and 5 mm will be used in the calculation of the sum of the diameters.

- If 2 target lesions coalesce, the measurement of the coalesced mass is used. If a large target lesion splits, the sum of the parts is used.

- If a subset of the target lesions identified at baseline were not measured post-baseline, the objective status of target lesions at the visit is Not Evaluable, unless the partial sum of diameters for the subset of target lesions assessed at the visit already meets the PD criteria, ie, at least 5 mm and 20% increase above the smallest sum of diameters observed, in which case the objective status of target lesions at the visit is PD.

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- If a lesion disappears and reappears at a subsequent time point, it should continue to be measured. The patient's response at the point in time when the lesion reappears will depend upon the status of his/her other lesions and previous response. For example, if the patient's tumor had reached a CR status and a non-lymph node target lesion reappeared, then the patient's response would be considered PD at the time of reappearance.

- In contrast, if the tumor status was a PR or SD and 1 lesion which had disappeared then reappears, its maximal diameter should be added to the sum of the remaining lesions for a calculated response.

Response evaluation of non-target lesions

- **Complete Response (CR)/Absent:** Complete disappearance of all non-target lesions. All nodes must decrease to normal (short axis <10 mm). All disease sites must be assessed.

- **Non-CR/Non-PD/Present without unequivocal progression:** Persistence of 1 or more non-target lesion(s).

- **Progressive Disease (PD)/Unequivocal progression**: Unequivocal progression of existing non-target lesions.

- **Not All Evaluated/Inevaluable:** Progression has not been documented and non-target lesions are not all assessable or disease assessed with inconsistent methods (unless considered "interchangeable") or one or more target lesions were excised or irradiated and have not reappeared or increased.

  Note: there is no standard RECIST method of using non-target disease assessments to determine unequivocal progression in non-target disease; by the nature of non-measurable disease, this is not possible. Unequivocal progression is based on clinical judgment.

New lesions evaluation criteria

- If a lesion is identified in a location not scanned previously, this will be considered progression of disease (new lesion).

- If a new lesion is equivocal at one assessment (classified as "Equivocal") and verified at the next, (classified as "Unequivocal"), the earlier date is used as the progression date.

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Objective Response Status at each assessment

Assessment is based on the target and non-target lesions response as described above and in Table 14 and Table 15, respectively.

For participants with target disease (ie, at least one measurable lesion at screening), Table 14 will be used.

Table 14. Objective Response Status at Each Assessment for Patients with Measurable Disease at Baseline

Target Lesions	Non-Target Lesions	New Lesions	Objective Status
CR	CR	No	CR
CR	Non-CR/Non-PD or not all evaluated	No	PR
PR	Non-PD* or not all evaluated	No	PR
SD	Non-PD* or not all evaluated	No	SD
Not all evaluated	Non-PD	No	NE
PD	Any	Yes or No	PD
Any	PD	Yes or No	PD
Any	Any	Yes**	PD

*Non-PD includes CR and Non-CR/Non-PD
** New lesions must be unequivocal
Abbreviations: CR = complete response; PD = progressive disease; PR = partial response; SD = stable disease; NE = not evaluable.
Note: cases for which target disease is assessed as SD or better but non-target assessments are missing, must be reviewed carefully. By considering the whole sequence, it may be reasonable to assume missed non-target assessments were not progression.

For participants with only non-target disease, Table 15 will be used.

Table 15. Objective Response Status at Each Assessment for Patients with Non-Target Disease Only

Non-Target Disease	New Lesions	Objective Status
CR/Absent	No	CR
Non-CR/Non-PD/ Present without unequivocal progression	No	Non-CR/Non-PD
Not all evaluated/Inevaluable	No	NE

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Table 15. Objective Response Status at Each Assessment for Patients with Non-Target Disease Only

Non-Target Disease	New Lesions	Objective Status
PD/Unequivocal progression	Yes or No	PD
Any	Yes*	PD

*New lesions must be unequivocal

Abbreviations: CR = complete response; PD = progressive disease; NE = not evaluable.

Date of Response

If there are multiple scan dates associated with a tumor evaluation, ie, radiological assessments occur over a series of days rather than the same day, the earliest scan date associated with the evaluation will be used as the date of the assessment. The same convention should be used for the investigator assessment (IOTA CRF page).

Best Overall Response (BOR)

The BOR will be assessed based on reported overall lesion responses at different evaluation time points from the randomization date until the first documentation of PD. For CR and PR, the patient's best response assignment will depend on the achievement of both measurement and confirmation criteria. CR and PR must be confirmed by 2 measurements at least 4 weeks apart. In the case of SD, follow up measurements must have met the SD criteria at least once after randomization.

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10.12. Appendix 12: Bone Marrow Reserve in Adults

Adapted from (Ellis, 1961).

Marrow Distribution of the Adult

SITE		MARROW wt. (g)	FRACTION RED MARROW AGE 40	RED MARROW wt. (g) AGE 40	% TOTAL RED MARROW	
CRANIUM AND MANDIBLE	Head:			136.6		
	Cranium	165.8	0.75	124.3	13.1	**13.1**
	Mandible	16.4	0.75	12.3		
HUMERI, SCAPULAE, CLAVICLES	Upper Limb Girdle :			86.7		
	2 Humerus, head & neck	26.5	0.75	20.0	8.3	8.3
	2 Scapulae	67.4	0.75	50.5		
	2 Clavicles	21.6	0.75	16.2		
STERNUM AND RIBS	Sternum	39.0	0.6	23.4	2.3	
	Ribs:			82.6		
	1 pair	10.2	All 0.4	4.1		
	2	12.6		5.0		
	3	16.0		6.4		
	4	18.6		7.4		
	5	23.8		9.5	7.9	**10.2**
	6	23.6		9.4		
	7	25.0		10.0		
	8	24.0		9.6		
	9	21.2		8.5		
	10	16.0		6.4		
	11	11.2		4.5		
	12	4.6		1.8		
PELVIC BONES	Sacrum	194.0	0.75	145.6	13.9	
	2 os coxae	310.6	0.75	233.0	22.3	**36.2**
FEMUR	2 Femoral head and neck	53.0	0.75	40.0		**3.8**

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Marrow Distribution of the Adult (cont'd)

SITE		MARROW wt. (g)	FRACTION RED MARROW AGE 40	RED MARROW wt. (g) AGE 40	% TOTAL RED MARROW	
VERTEBRAE	Vertebrae (Cervical):			35.8		
	1	6.6	All 0.75	5.0		
	2	8.4		6.3		
	3	5.4		4.1	3.4	
	4	5.7		4.3		
	5	5.8		4.4		
	6	7.0		5.3		
	7	8.5		6.4		
	Vertebrae (Thoracic):			147.9		
	1 pair	10.8	All 0.75	8.1		
	2	11.7		8.8		
	3	11.4		8.5		
	4	12.2		9.1		
	5	13.4		10.1	14.1	**28.4**
	6	15.3		11.5		
	7	16.1		12.1		
	8	18.5		13.9		
	9	19.7		14.8		
	10	21.2		15.9		
	11	21.7		16.3		
	12	25.0		18.8		
	Vertebrae (Lumbar):			114.1		
	1 pair	27.8	All 0.75	20.8		
	2	29.1		21.8	10.9	
	3	31.8		23.8		
	4	32.1		24.1		
	5	31.4		23.6		
TOTAL		**1497.7**		**1045.7**	**100.0**	**100.0**

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10.13. Appendix 13: Additional Study Assessments, Inventories, and Questionnaires

10.13.1. Patient-Reported Outcomes Assessments (Phase 3 only)

The Phase 3 portion of the study will not be conducted.

The following PRO assessments will be administered:

- EQ-5D-5L and VAS
- EORTC QLQ-C30
- EORTC QLQ-BR23 (female and male questionnaire versions)

The **EQ-5D-5L** is a 5-item patient-completed questionnaire designed to assess health status in terms of a single index value or utility score. There are 2 components, a Health State Profile which has individuals rate their level of problems (none, slight, moderate, severe, extreme/unable) in 5 areas (mobility, self-care, usual activities, pain/discomfort, and anxiety/depression), and a VAS in which patients rate their overall health status from 0 (worst imaginable) to 100 (best imaginable). Published weights are available that allow for the creation of a single summary score. Overall scores range from 0 to 1, with lower scores representing a higher level of dysfunction (Rabin & de Charro, 2001).

The **EORTC QLQ-C30** is a well-known, validated, and self-administered PRO. The EORTC QLQ-C30 is a 30-question survey, which can be grouped into 5 functional domain subscales, including a physical functioning subscale, a role functioning subscale, an emotional functioning subscale, a cognitive functioning subscale, and a social functioning subscale. Higher scores on the functional domains are indicative of higher levels of functioning. Oncology related symptoms assessed by the EORTC QLQ-C30 include fatigue (3 items), pain (2 items), nausea and vomiting (2 items), and dyspnea, insomnia, appetite loss, constipation, diarrhea, and financial impact (1 item each). Higher scores are reflective of a greater presence of symptoms (Aaronson et al, 1993).

The **EORTC QLQ-BR23** is a 23-item breast cancer-specific companion module to the EORTC QLQ-C30 and consists of 4 functional scales (body image, sexual functioning, sexual enjoyment, future perspective) and 4 symptom scales (systemic side-effects, breast symptoms, arm symptoms, upset by hair loss). Like the QLQ-C30, higher scores represent a better level of functioning/QoL than lower scores on functional and global QoL scales. However, for symptom-oriented scales, higher scores represent greater symptom severity (Sprangers et al, 1996). Female and male questionnaire versions will be used depending on participant gender.

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10.13.2. Eastern Cooperative Oncology Group (ECOG) Performance Status

*Developed by the Eastern Cooperative Oncology Group, Robert L. Comis, MD, Group Chair.**

Grade	Description
0	Fully active, able to carry on all pre-disease performance without restriction.
1	Restricted in physically strenuous activity, but ambulatory and able to carry out work of a light or sedentary nature, ie, light house work, office work.
2	Ambulatory and capable of all self-care but unable to carry out any work activities. Up and about more than 50% of waking hours.
3	Capable of only limited self-care, confined to bed or chair more than 50% of waking hours.
4	Completely disabled. Cannot carry on any self-care. Totally confined to bed or chair.

*(Oken et al, 1982).

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10.14. Appendix 14: Protocol Amendment History

The protocol amendment summary of changes table for the current amendment is located directly before the TOC. The protocol amendment summary of changes tables for past amendment(s) can be found below:

Amendment 2 (28 September 2023)

Overall Rationale for the Amendment: The protocol was amended to include the 25 mg palbociclib strength to be used for potential palbociclib dose reduction below 75 mg QD. Additional changes include addition of lipid tests due to new drug class effects reported for elacestrant, as well as addition of HBV, HCV and HIV tests related to local requirements.

Description of Change	Brief Rationale	Section # and Name
Substantial Modification(s)		
Palbociclib 25 mg strength added.	The palbociclib 25 mg strength was added to be used in SLI and Phase 3 Experimental Arm (Arm A), for potential palbociclib dose reduction to 50 mg QD. This dose reduction will be applied at PI discretion and, where required, subject to regulatory approval.	Section 1.1 Synopsis, Study Lead--in and Phase 3 Study Interventions tables. Section 6.1 Study Intervention(s) administered, Tables 5 and 7. Section 6.6.1.2.1 Palbociclib Dose Reduction for SLI and Phase 3 Experimental Arm (Arm A). Section 6.6.1.3 Recommended vepdegestrant and Palbociclib Dose Modifications, Table 12.
Non-Substantial Modification(s)		
Generic name of ARV-471 added.	Generic name of ARV-471 ie, vepdegestrant became available.	Cover page Section 2, Introduction
ClinicalTrials.gov ID has been added.	To provide the ClinicalTrials.gov ID.	Cover page, Section 1.1 Synopsis
Text added to Inclusion Criterion 3b.	To clarify that HER2 status for inclusion should be assessed on the same tumor sample used for characterizing the ER status.	Section 1.1 Synopsis, Inclusion Criteria Section 5.1 Inclusion Criteria
Letrozole formulation to be used in Phase 3 Control	Due to change of the study design as per Amendment 1 ie, from Double Blind to Open	Section 1.1 Synopsis, Phase 3 Study Arms table.

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Description of Change	Brief Rationale	Section # and Name
Arm (Arm B), updated from capsule to tablet.	Label, encapsulation of letrozole tablets used in Phase 3 to blind the drug, is not anymore needed.	Section 6.1 Study Intervention(s) administered, Tables 7-8. Section 6.1.1 Administration Section 6.1.1.3 Letrozole (Phase 3 Control Arm only)
Periodic monitoring of cholesterol and triglycerides added.	ORSERDU® (elacestrant), a new estrogen receptor antagonist approved by FDA in January 2023, includes dyslipidemia in USPI "Warning and Precautions" and "Adverse Reactions" sections. The newly added lipid tests are not related to any new safety signal for vepdegestrant but will support the characterization of vepdegestrant in combination with palbociclib on lipid metabolism and support participant safety.	Section 1.3 Schedule of Activities -Tables 1 and 3. Section 10.2 Appendix 2
Text added for blood sample collection for vepdegestrant and palbociclib PK.	To clarify that in Phase 3 blood for vepdegestrant PK will be collected for participants in Arm A only while blood for palbociclib PK will be collected in both arms.	Section 1.3 Schedule of Activities, Table 3, Blood sample for vepdegestrant and palbociclib PK.
Updated language in LHRH Schedule of Activities for: • allowing the initiation of the 3-month formulation on C1D1 in addition of the 28-day formulation. • including the option of supply by the sponsor if needed by local law/regulations and if available to source in-country.	To clarify the 3-month LHRH formulation may be started on Cycle 1 Day 1, if not already ongoing. To clarify that LHRH may be provided by the Sponsor in certain circumstances.	Section 1.3 Schedule of Activities, Tables 1 and 3, LHRH Agonist Section 6.9.1. Luteinizing Hormone-Releasing Hormone Agonist.

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Description of Change	Brief Rationale	Section # and Name
HBV, HCV, HIV laboratory tests added.	These tests, not mandatory per protocol at screening for eligibility purpose, were added to satisfy local regulations that mandated them.	Section 1.3 Schedule of Activities (Table 1 & 3). Section 10.2 Clinical Laboratory Assessments, Appendix 2.
Hypersensitivity added as an example of the medical conditions.	To satisfy Health Authority request to add, as guidance for investigators, hypersensitivity as an example of medical conditions preventing eligibility in Exclusion Criterion 1.	Section 5.2 Exclusion Criteria, Exclusion Criterion #1
The word "preferably" added.	To increase flexibility in the assumption of study interventions considering no issues from a safety perspective have been identified in case study interventions are taken later in the day.	Section 6.1.1 Administration
Recommendation for the use of P-gp substrate, H2 receptor antagonists, antacids with vepdegestrant added.	Instruction to consult the P- gp substrate prescribing information before concomitant use with vepdegestrant was added as advice since the management of the concomitant drug depends on factors such as participant's comorbidities and indication for the P-gp substrate. Management of the concomitant medication should be assessed by the PI on a case-by-case basis after consultation of the local prescribing information. Additional H2 receptor antagonists and antacid recommendation for use, were added to include an alternative option instead of staggering them.	Section 6.9.4 Concomitant Treatments to be used with Caution or Not Recommended.
Added references to RECIST v1.1 and Appendix 11 to the reason	To clarify that objective disease progression is defined as per	Section 7.1. Discontinuation of Study Intervention.

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Description of Change	Brief Rationale	Section # and Name
for treatment discontinuation reported in Section 7.1.	RECIST v1.1 detailed in Appendix 11.	
Updated language on the collection of Follow-up anticancer therapy.	To clarify information on Follow-up anticancer therapy may be collected by telephone call and other type of visits.	Section 7.1.1.2 Long-Term Follow-Up (SLI only) Section 8.1.2 Telehealth Visits Section 8.1.3 Home Health Visits.
List of strong CYP3A inhibitors updated.	The list of strong CYP3A inhibitors was revised per update of the University of Washington Drug Interaction Database.	Section 10.9.1 CYP3A Inhibitors/Inducers Appendix 9.
Updates made to align with Sponsor new protocol template.	To align with Sponsor protocol template.	Cover page Section 1.1 Synopsis Section 2.3.1 Risk Assessment Section 6.8. Treatment of Overdose Section 8.4. Adverse Events, Serious Adverse Events, and Other Safety Reporting Section 9.2 Analysis Sets Section 10.1 Appendix 1 Regulatory, Ethical, and Study Oversight Considerations Section 10.3. Appendix 3: Adverse Events: Definitions and Procedures for Recording, Evaluating, Follow Up, and Reporting. Section 10.4.3. Woman of Childbearing Potential. Section 10.6. Appendix 6: Liver Safety: Suggested Actions and Follow Up Assessments. Section 10.7. Appendix 7: Kidney Safety Monitoring Guidelines.

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Description of Change	Brief Rationale	Section # and Name
		Section 10.8. Appendix 8: ECG Findings of Potential Clinical Concern.
Protocol updated per PACL dated 18 August 2023. • Correct the appendix number referenced in the exclusion criterion 7. • Remove the reference to blinded bottles reported in Table 7. • Add a cross reference to Section 9.4 Interim analysis in the End of Study Definition.	Changes implemented through a PACL were incorporated.	• Section 5.2 Exclusion criterion 7. • Section 6.1 Administration and Intervention Table 7. • Section 4.4, End of Study Definition.
Minor editorial changes throughout the protocol.	Typographical errors corrected and minor edits performed.	Section 2.2.3.3 ARV-471-mBC-101 FIH Study: Preliminary Safety, Efficacy and Pharmacokinetic Data in Humans. Section 5.2 Exclusion Criterion 11. Section 6.6.1.3 Recommended vepdegestrant and Palbociclib Dose Modifications, Table 12. Section 6.6.3 Dose Delays 9.3.2.1. SLI Primary Endpoints. Section 9.3.6.1. Electrocardiogram Analyses.

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Table 16. Schedule of Activities: Study Lead-in

Protocol Activity	**Screening[a]**	**Active Treatment Phase[b] (1 Cycle = 28 days)**			**EOT / Withdrawal[c]**	**Post-Treatment FU**		**Notes:** **a. Screening: screening procedures should be completed within 28 days from randomization unless otherwise specified.** **b. Active Treatment Phase assessments should be performed, and safety tests reviewed, <u>prior to dosing</u> on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) ECG; 2) blood drawn for PK; 3) blood drawn for lab tests/pharmacodynamics/ Retained Research Sample; 4) PE/Vital signs.** **c. EOT assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 8 or 12 weeks (tumor assessment other than bone scan), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere.** **d. Safety FU is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1).** **e. Minus 2-day time window for Cycle 1 Day 1.**
		Cycles 1 &2		**Cycles ≥3**		**Safety FU[d]**	**Long Term FU**	
Study Day	**≤ 28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**at 3 mos. from EOT**	
Visit Window (days)		**±2[e]**	**±2**	**±2**		**+7d**	**±14d**	
ical Assessments								
Informed Consent	X							• ICD must be obtained ≤28 days from randomization and prior to any protocol required assessments being performed, unless otherwise specified eg, assessments by imaging (see Section 8.2.1). • See Appendix 1 Section 10.1.3.
Screen for Inclusion/Exclusion Criteria	X							• See Section 5.1 and Section 5.2
Physical Examination & Vital signs	X	X		X	X			• C1D1 PE is not required if acceptable screening assessment is performed ≤7 days from randomization. • Height at screening only and weight at screening on Day 1 of each cycle and at the EOT. • See Sections 8.3.1 and 8.3.2.
Medical/ Oncological History	X							• See Section 8.1.1.

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		Cycles 1 &2		**Cycles ≥3**		**Safety FU[d]**	**Long Term FU**	
Study Day	**≤ 28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**at 3 mos. from EOT**	
Visit Window (days)		**±2[e]**	**±2**	**±2**		**+7d**	**±14d**	
ECOG PS	X							• Participants must have an ECOG PS ≤2 as per IC #6 to be eligible. • See Appendix 13 Section 10.13.2.
12-Lead ECG	X	X	X	X (*Cycles 3 & 4 only*)	X			• Triplicate ECGs will be requested at each timepoint. • C1D1 is not required if acceptable screening assessment is performed ≤7 days from randomization. • Electrolytes (Na, K, total Ca, Mg) must be performed near to each ECG assessment (see Note for Active Treatment Phase above for the sequence of assessments). • See Section 8.3.3.
LHRH agonist		*(Every 28 days or every 3 months starting from Cycle 1 Day 1 if not already ongoing)*					X (*at PI discretion)*	• Pre-/peri-menopausal female participants and male participants must start (if not already ongoing) an LHRH agonist on Cycle 1 Day 1 and continue every 28 days or every 3 months (depending on LHRH formulation) throughout Active Treatment Phase irrespectively of

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Table 16. Schedule of Activities: Study Lead-in

Protocol Activity	**Screening[a]**	**Active Treatment Phase[b] (1 Cycle = 28 days)**			**EOT / Withdrawal[c]**	**Post-Treatment FU**		**Notes:** **a. Screening: screening procedures should be completed within 28 days from randomization unless otherwise specified.** **b. Active Treatment Phase assessments should be performed, and safety tests reviewed, prior to dosing on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) ECG; 2) blood drawn for PK; 3) blood drawn for lab tests/pharmacodynamics/ Retained Research Sample; 4) PE/Vital signs.** **c. EOT assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 8 or 12 weeks (tumor assessment other than bone scan), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere.** **d. Safety FU is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1).** **e. Minus 2-day time window for Cycle 1 Day 1.**
		Cycles 1 &2		**Cycles ≥3**		**Safety FU[d]**	**Long Term FU**	
Study Day	**≤ 28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**at 3 mos. from EOT**	
Visit Window (days)		**±2[e]**	**±2**	**±2**		**+7d**	**±14d**	
								cycle duration and until 28 days after the last dose of study intervention. • Participants already on treatment with a 28-day or 3-month formulation may continue it. • Long Term FU: LHRH agonist should be continued at PI discretion. • If LHRH agonist is administered at home, a diary will be implemented for collection of LHRH administration details. • See Section 6.9.1.
Contraception & check (if applicable)	X	X		X	X	X (*WOCBP only*)	X *(Men only)*	• Mandatory for WOCBP participants and male participants. • Regular checks regarding appropriate contraception use are required at times noted in the SoA. • See Appendix 4 for details on contraception requirements and guidance.

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Table 16. Schedule of Activities: Study Lead-in

Protocol Activity	**Screening[a]**	**Active Treatment Phase[b] (1 Cycle = 28 days)**			**EOT / Withdrawal[c]**	**Post-Treatment FU**		**Notes:** **a. Screening: screening procedures should be completed within 28 days from randomization unless otherwise specified.** **b. Active Treatment Phase assessments should be performed, and safety tests reviewed, prior to dosing on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) ECG; 2) blood drawn for PK; 3) blood drawn for lab tests/pharmacodynamics/ Retained Research Sample; 4) PE/Vital signs.** **c. EOT assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 8 or 12 weeks (tumor assessment other than bone scan), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere.** **d. Safety FU is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1).** **e. Minus 2-day time window for Cycle 1 Day 1.**
		Cycles 1 &2		**Cycles ≥3**		**Safety FU[d]**	**Long Term FU**	
Study Day	**≤ 28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**at 3 mos. from EOT**	
Visit Window (days)		**±2[e]**	**±2**	**±2**		**+7d**	**±14d**	
ratory Studies								
Hematology	X	X	X	X	X	X *(only if laboratory abnormalities not recovered at EOT)*		• C1D1 assessment not required if acceptable screening assessment is performed ≤7 days from randomization. • See Sections 8.3.4, 8.4.1 and Appendix 2
Blood Chemistry	X	X	X	X	X	X *(only if laboratory abnormalities not*		• C1D1 assessment not required if acceptable screening assessment is performed ≤7 days from randomization. • On Active Treatment Phase: o Total Ca and Mg are required ONLY up to Cycle 3. o Glucose (fasting) is required ONLY on C1D1, then every 3 cycles (eg, Cycles 4, 7, 10 up to Cycle 25).

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Table 16. Schedule of Activities: Study Lead-in

Protocol Activity	**Screening[a]**	**Active Treatment Phase[b] (1 Cycle = 28 days)**			**EOT / Withdrawal[c]**	**Post-Treatment FU**		**Notes:** **a. Screening: screening procedures should be completed within 28 days from randomization unless otherwise specified.** **b. Active Treatment Phase assessments should be performed, and safety tests reviewed, <u>prior to dosing</u> on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) ECG; 2) blood drawn for PK; 3) blood drawn for lab tests/pharmacodynamics/ Retained Research Sample; 4) PE/Vital signs.** **c. EOT assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 8 or 12 weeks (tumor assessment other than bone scan), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere.** **d. Safety FU is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1).** **e. Minus 2-day time window for Cycle 1 Day 1.**
		Cycles 1 &2		**Cycles ≥3**		**Safety FU[d]**	**Long Term FU**	
Study Day	**≤ 28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**at 3 mos. from EOT**	
Visit Window (days)		**±2[e]**	**±2**	**±2**		**+7d**	**±14d**	
						recovered at EOT)		○ Cholesterol and triglycerides (fasting) are required on C4D1, then every 6 cycles (eg, Cycles 10, 16) and at EOT. Results of these tests are not needed for retreatment. • See Sections 8.3.4, 8.4.1 and Appendix 2.
Coagulation	X							• See Appendix 2.
FSH (as applicable)	X							• To confirm a post-menopausal status of female participants under 60 years of age, with cessation of regular menses for 12 consecutive months and with no alternative medical cause. • See Appendix 2 and Appendix 4.
HBV, HCV and HIV Tests (*If required by local regulations*)	X							• See Appendix 2

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Table 16. Schedule of Activities: Study Lead-in

Protocol Activity	**Screening[a]**	**Active Treatment Phase[b] (1 Cycle = 28 days)**			**EOT / Withdrawal[c]**	**Post-Treatment FU**		**Notes:** **a. Screening: screening procedures should be completed within 28 days from randomization unless otherwise specified.** **b. Active Treatment Phase assessments should be performed, and safety tests reviewed, prior to dosing on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) ECG; 2) blood drawn for PK; 3) blood drawn for lab tests/pharmacodynamics/ Retained Research Sample; 4) PE/Vital signs.** **c. EOT assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 8 or 12 weeks (tumor assessment other than bone scan), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere.** **d. Safety FU is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1).** **e. Minus 2-day time window for Cycle 1 Day 1.**
		Cycles 1 &2		**Cycles ≥3**		**Safety FU[d]**	**Long Term FU**	
Study Day	**≤ 28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**at 3 mos. from EOT**	
Visit Window (days)		**±2[e]**	**±2**	**±2**		**+7d**	**±14d**	
Pregnancy Test (as applicable)	X	X		X	X	X		• WOCBP participants: a serum pregnancy test is mandatory at screening. • Following screening, additional pregnancy tests per SoA may be serum or urine tests with a sensitivity of at least 25 mIU/mL. • Additional pregnancy tests may be performed at any time per investigator's judgement or if required by local regulations or IRBs/ECs. • See Section 8.3.5
ase Assessments								
Tumor Assessments (other than Bone scan)	X	◄--► At 8, 16 and 24 weeks (±7 days) from the date of randomization; then every 12 weeks (±7 days) from the date of randomization			X			• Tumor assessments by imaging should continue until one of the reasons outlined in Section 8.2.1 has occurred.

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Table 16. Schedule of Activities: Study Lead-in

Protocol Activity	**Screening[a]**	**Active Treatment Phase[b] (1 Cycle = 28 days)**			**EOT / Withdrawal[c]**	**Post-Treatment FU**		**Notes:**
		Cycles 1 &2		**Cycles ≥3**		**Safety FU[d]**	**Long Term FU**	
Study Day	**≤ 28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**at 3 mos. from EOT**	
Visit Window (days)		**±2[e]**	**±2**	**±2**		**+7d**	**±14d**	
Bone Scan, Whole Body	X	◄--► Every 24 weeks (±7 days) from the date of randomization			X			• On Active Treatment Phase, bone scan should be performed only if needed as per Section 8.2.1 requirements. • See Section 8.2.2 for further details on treatment continuation beyond disease progression. • Post-Treatment tumor assessments (including bone scan, if applicable) are required only for participants who discontinued the study treatment for reason other than PD. • See Sections 8.2.1 and 8.2.2.
lomization and Study Treatment								
Randomization	X							• Randomization must occur after having reviewed all eligibility criteria and confirmed they are satisfied. • De-randomization is NOT allowed as outlined in Section 6.3.

Notes:
a. Screening: screening procedures should be completed within 28 days from randomization unless otherwise specified.
b. Active Treatment Phase assessments should be performed, and safety tests reviewed, prior to dosing on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) ECG; 2) blood drawn for PK; 3) blood drawn for lab tests/pharmacodynamics/ Retained Research Sample; 4) PE/Vital signs.
c. EOT assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 8 or 12 weeks (tumor assessment other than bone scan), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere.
d. Safety FU is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1).
e. Minus 2-day time window for Cycle 1 Day 1.

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Table 16. Schedule of Activities: Study Lead-in

Protocol Activity	**Screening[a]**	**Active Treatment Phase[b] (1 Cycle = 28 days)**			**EOT / Withdrawal[c]**	**Post-Treatment FU**		**Notes:** **a. Screening: screening procedures should be completed within 28 days from randomization unless otherwise specified.** **b. Active Treatment Phase assessments should be performed, and safety tests reviewed, prior to dosing on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) ECG; 2) blood drawn for PK; 3) blood drawn for lab tests/pharmacodynamics/ Retained Research Sample; 4) PE/Vital signs.** **c. EOT assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 8 or 12 weeks (tumor assessment other than bone scan), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere.** **d. Safety FU is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1).** **e. Minus 2-day time window for Cycle 1 Day 1.**
		Cycles 1 &2		**Cycles ≥3**		**Safety FU[d]**	**Long Term FU**	
Study Day	**≤ 28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**at 3 mos. from EOT**	
Visit Window (days)		**±2[e]**	**±2**	**±2**		**+7d**	**±14d**	
								• At randomization, randomization number and dose level are assigned. • Study treatment must start ≤3 days from randomization. • See Section 6.3.
vepdegestrant		◄--► Once Daily						• vepdegestrant is given orally with food in continuous daily dosing for a 28-day cycle. • See Section 6.1.1 and Section 6.1.1.1 for details on drug administration.
Palbociclib		◄--► Once Daily on Days 1-21 followed by 7 days off						• Palbociclib is given orally with food in daily dosing from Day 1 to Day 21 (21 days) followed by 7 days off-treatment, for a 28-day cycle. Palbociclib drug intake should occur at the same time of vepdegestrant. • If the Day 1 visit of the subsequent cycle is scheduled during the Day -2 time window, participant must be instructed to complete their 7-day off-treatment period of

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Table 16. Schedule of Activities: Study Lead-in

Protocol Activity	**Screening[a]**	**Active Treatment Phase[b] (1 Cycle = 28 days)**			**EOT / Withdrawal[c]**	**Post-Treatment FU**		**Notes:** **a. Screening: screening procedures should be completed within 28 days from randomization unless otherwise specified.** **b. Active Treatment Phase assessments should be performed, and safety tests reviewed, prior to dosing on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) ECG; 2) blood drawn for PK; 3) blood drawn for lab tests/pharmacodynamics/ Retained Research Sample; 4) PE/Vital signs.** **c. EOT assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 8 or 12 weeks (tumor assessment other than bone scan), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere.** **d. Safety FU is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1).** **e. Minus 2-day time window for Cycle 1 Day 1.**
		Cycles 1 &2		**Cycles ≥3**		**Safety FU[d]**	**Long Term FU**	
Study Day	**≤ 28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**at 3 mos. from EOT**	
Visit Window (days)		**±2[e]**	**±2**	**±2**		**+7d**	**±14d**	
								the current cycle prior to resuming the drugs given for the subsequent cycle. • See Section 6.1.1 and Section 6.1.1.2 for details on drug administration.
r Clinical Assessments								
Drug Compliance		◄--►						• A dosing diary will be implemented for collection of administration details. Any unused tablets/capsules must be returned to the clinic for drug accountability. • From Cycle 2 onward, compliance will be performed on Day 1 of each cycle, prior to dispensing study interventions for the following cycle. • See Sections 6.2 and 6.5.
Adverse Event Reporting	X	X	X	X	X	X		• See Section 8.4.1 and Appendix 3 for details on AE/SAE reporting requirements during the reporting period and

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Table 16. Schedule of Activities: Study Lead-in

Protocol Activity	**Screening[a]**	**Active Treatment Phase[b] (1 Cycle = 28 days)**			**EOT / Withdrawal[c]**	**Post-Treatment FU**		**Notes:** **a. Screening: screening procedures should be completed within 28 days from randomization unless otherwise specified.** **b. Active Treatment Phase assessments should be performed, and safety tests reviewed, prior to dosing on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) ECG; 2) blood drawn for PK; 3) blood drawn for lab tests/pharmacodynamics/ Retained Research Sample; 4) PE/Vital signs.** **c. EOT assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 8 or 12 weeks (tumor assessment other than bone scan), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere.** **d. Safety FU is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1).** **e. Minus 2-day time window for Cycle 1 Day 1.**
		Cycles 1 &2		**Cycles ≥3**		**Safety FU[d]**	**Long Term FU**	
Study Day	**≤ 28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**at 3 mos. from EOT**	
Visit Window (days)		**±2[e]**	**±2**	**±2**		**+7d**	**±14d**	
								for SAE reporting requirements after the end of reporting period.
Concomitant Medications/ Treatments	X	X	X	X	X	X		• See Section 6.9.
FU anticancer therapy						X		• The start date of the first FU anticancer therapy will be collected. • See Section 7.1.1.2
Pharmacokinetic assessments								
Special Laboratory Studies								

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Table 17. Schedule of Activities: PK and Special Laboratory Assessments for Study Lead-in

	Active Treatment Phase (1 Cycle = 28 days)																**Notes:**
Cycle	**Cycle 1**											**Cycle 2**			**Cycle 3**	**Cycle 5**	**On the visit days, participants should be instructed not to take study interventions at home and to withhold their daily dose until the predose PK sample has been collected.**
Day	**Day 1**					**Day 2**	**Day 15**					**Day 1**	**Day 15**		**Day 1**	**Day 1**	
Visit window	**-2**					**-**	**±2**					**±2**	**±2**		**±2**	**±2**	
Hours post dose	**0**	**1**	**2**	**4**	**7**	**0**	**0**	**1**	**2**	**4**	**7**	**0**	**0**	**5-7**	**0**	**0**	**On the visit days, assessments (if performed on the same day), should be performed unless otherwise specified, using the following sequence: 1) ECG; 2) blood drawn for PK; 3) blood drawn for lab tests/pharmacodynamics/ Retained Research Sample; 4) PE/Vital signs.**
Time window		± 6 min	± 12 min	± 24 min	± 42 min	≤ 2 hr prior to dose	≤ 2 hr prior to dose	± 6 min	± 12 min	± 24 min	± 42 min	≤ 2 hr prior to dose	≤ 2 hr prior to dose	-	≤ 2 hr prior to dose	≤ 2 hr prior to dose	
Blood Samples for vepdegestrant PK	-	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	• A single 3 mL whole blood sample will be collected at each timepoints. • See Section 8.5
Blood Samples for palbociclib PK	-	X	X	X	X	X	X	X	X	X	X	X	X	X	-	-	• A single 3 mL whole blood sample will be collected at each timepoints. • See Section 8.5
Blood Samples for Pharmacodynamics	X	-	-	-	-	-	X	X	X	X	X	-	-	-	-	-	• A single 4 mL whole blood samples at each timepoint will be collected. • See Section 8.7.2
Retained Research Blood Sample for Genetics	X (*predose*)	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	• A single 4 mL whole blood sample for DNA extraction (Prep D1) will be collected, unless prohibited by local regulations, IRBs/ECs. • If not collected on the designated collection day, collect at the next available time point when biospecimens are being collected in conjunction with a participant visit. • See Section 8.6.2

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Table 18. Schedule of Activities: Phase 3

Protocol Activity	Screening[a]	Active Treatment Phase[b] (1 Cycle = 28 days)			EOT / Withdrawal[c]	Post-Treatment FU		Notes
		Cycles 1 & 2		Cycles ≥3		Safety FU[d]	Survival FU[e]	a. **Screening:** screening procedures should be completed within 28 days from randomization unless otherwise specified.
Study Day	**≤28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**Every 3 mos. from EOT**	b. **Active Treatment Phase** assessments should be performed, and safety tests reviewed, prior to dosing on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) PROs; 2) ECGs; 3) blood drawn for PK; 4) blood drawn for lab tests/ctDNA/Retained Research Sample; 5) PE/Vital signs.
Visit Window (days)		**±2[f]**	**±2**	**±2**		**+7**	**±14**	c. **EOT** assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 12 weeks (tumor assessment), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere. d. **Safety FU** is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1). e. **Survival FU** is every 3 months from EOT (see Section 7.1.1.3). f. Minus 2-day time window for Cycle 1 Day 1.
Clinical Assessments								
Informed Consent	X							• ICD must be obtained ≤28 days from randomization and prior to any protocol required assessments being performed, unless otherwise specified eg, assessments by imaging (see Section 8.2.1). • See Appendix 1 Section 10.1.3.
Screen for Inclusion/Exclusion Criteria	X							• See Section 5.1 and Section 5.2.
Physical Examination /Vital signs	X	X		X	X			• C1D1 PE is not required if acceptable screening assessment is performed ≤7 days from randomization. • Height at screening only and weight at screening on Day 1 of each cycle and at the EOT. • See Sections 8.3.1 and 8.3.2.
Medical/ Oncological History	X							• See Section 8.1.1.
ECOG PS	X							• Participants must have an ECOG PS ≤2 as per IC #6 to be eligible. • See Appendix 13 Section 10.13.2.

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Table 18. Schedule of Activities: Phase 3

Protocol Activity	Screening[a]	Active Treatment Phase[b] (1 Cycle = 28 days)			EOT / Withdrawal[c]	Post-Treatment FU		Notes
		Cycles 1 & 2		Cycles ≥3		Safety FU[d]	Survival FU[e]	a. **Screening:** screening procedures should be completed within 28 days from randomization unless otherwise specified.
Study Day	**≤28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**Every 3 mos. from EOT**	b. **Active Treatment Phase** assessments should be performed, and safety tests reviewed, prior to dosing on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) PROs; 2) ECGs; 3) blood drawn for PK; 4) blood drawn for lab tests/ctDNA/Retained Research Sample; 5) PE/Vital signs.
Visit Window (days)		**±2[f]**	**±2**	**±2**		**+7**	**±14**	c. **EOT** assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 12 weeks (tumor assessment), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere. d. **Safety FU** is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1). e. **Survival FU** is every 3 months from EOT (see Section 7.1.1.3). f. Minus 2-day time window for Cycle 1 Day 1.
12-Lead ECG • Approx. the first 100 participants per arm	X	X	X		X			• C1D1 is not required if acceptable screening assessment is performed ≤7 days from randomization. • Electrolytes (Na, K, total Ca, Mg) must be performed near to each ECG assessment (see note for Active Treatment Phase for the sequence of assessments). • See Section 8.3.3.
12-Lead ECG • All remaining participants	X	X *(Cycle 1 only)*			X			
LHRH agonist		X *(Every 28 days or every 3 months starting from Cycle 1 Day 1 if not already ongoing)*					X (*at PI discretion*)	• **Pre-/peri-menopausal female participants and male participants** must start (if not already ongoing) an LHRH agonist on Cycle 1 Day 1 and continue every 28 days or every 3 months (depending on LHRH formulation) throughout Active Treatment Phase irrespectively of cycle duration and until 28 days after the last dose of study intervention. • Participants already on treatment with a 28-day or 3-month formulation may continue it. • **Survival FU**: LHRH agonist should be continued at PI discretion. • If LHRH agonist is administered at home, a diary will be implemented for collection of LHRH administration details.

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Table 18. Schedule of Activities: Phase 3

Protocol Activity	Screening[a]	Active Treatment Phase[b] (1 Cycle = 28 days)			EOT / Withdrawal[c]	Post-Treatment FU		Notes
		Cycles 1 & 2		Cycles ≥3		Safety FU[d]	Survival FU[e]	a. **Screening:** screening procedures should be completed within 28 days from randomization unless otherwise specified.
Study Day	**≤28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**Every 3 mos. from EOT**	b. **Active Treatment Phase** assessments should be performed, and safety tests reviewed, prior to dosing on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) PROs; 2) ECGs; 3) blood drawn for PK; 4) blood drawn for lab tests/ctDNA/Retained Research Sample; 5) PE/Vital signs.
Visit Window (days)		**±2[f]**	**±2**	**±2**		**+7**	**±14**	c. **EOT** assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 12 weeks (tumor assessment), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere. d. **Safety FU** is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1). e. **Survival FU** is every 3 months from EOT (see Section 7.1.1.3). f. Minus 2-day time window for Cycle 1 Day 1.
								• See Section 6.9.1.
Contraception & check (if applicable)	X	X		X	X	X (*for IOCBP only*)	X (*for men only*)	• **Mandatory for IOCBP participants** and **male participants.** • Regular checks regarding appropriate contraception use are required at times noted in the SoA. • See Appendix 4 for details on contraception requirements and guidance.
Laboratory Studies								
Hematology	X	X	X	X	X	X *(only if laboratory abnormalities not recovered at EOT)*		• C1D1 assessment not required if acceptable screening assessment is performed ≤7 days from randomization. • See Sections 8.3.4, 8.4.1 and Appendix 2.

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Table 18. Schedule of Activities: Phase 3

Protocol Activity	Screening[a]	Active Treatment Phase[b] (1 Cycle = 28 days)			EOT / Withdrawal[c]	Post-Treatment FU		Notes
		Cycles 1 & 2		Cycles ≥3		Safety FU[d]	Survival FU[e]	a. **Screening:** screening procedures should be completed within 28 days from randomization unless otherwise specified.
Study Day	**≤28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**Every 3 mos. from EOT**	b. **Active Treatment Phase** assessments should be performed, and safety tests reviewed, prior to dosing on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) PROs; 2) ECGs; 3) blood drawn for PK; 4) blood drawn for lab tests/ctDNA/Retained Research Sample; 5) PE/Vital signs.
Visit Window (days)		**±2[f]**	**±2**	**±2**		**+7**	**±14**	c. **EOT** assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 12 weeks (tumor assessment), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere. d. **Safety FU** is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1). e. **Survival FU** is every 3 months from EOT (see Section 7.1.1.3). f. Minus 2-day time window for Cycle 1 Day 1.
Blood Chemistry	X	X	X	X	X	X (*only if laboratory abnormalities not recovered at EOT*)		• C1D1 assessment not required if acceptable screening assessment is performed ≤7 days from randomization. • On Active Treatment Phase: ○ Total Ca and Mg are required ONLY up to Cycle 3. ○ Glucose (fasting) is required ONLY on C1D1, then every 3 cycles (eg, Cycles 4, 7, 10 up to Cycle 25). ○ Cholesterol and triglycerides (fasting) are required on C4D1, then every 6 cycles (eg, Cycles 10, 16) and at EOT. Results of these tests are not needed for retreatment. • See Sections 8.3.4, 8.4.1 and Appendix 2.
Coagulation	X							• See Appendix 2.
FSH (as applicable)	X							• To confirm a post-menopausal status of female participants under 60 years of age, with cessation of regular menses for 12 consecutive months and with no alternative medical cause. • See Appendix 2 and Appendix 4.
HBV, HCV and HIV Tests (*If required by local regulations*)	X							• See Appendix 2.

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Table 18. Schedule of Activities: Phase 3

Protocol Activity	Screening[a]	Active Treatment Phase[b] (1 Cycle = 28 days)			EOT / Withdrawal[c]	Post-Treatment FU		Notes
		Cycles 1 & 2		Cycles ≥3		Safety FU[d]	Survival FU[e]	a. **Screening:** screening procedures should be completed within 28 days from randomization unless otherwise specified.
Study Day	**≤28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**Every 3 mos. from EOT**	b. **Active Treatment Phase** assessments should be performed, and safety tests reviewed, prior to dosing on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) PROs; 2) ECGs; 3) blood drawn for PK; 4) blood drawn for lab tests/ctDNA/Retained Research Sample; 5) PE/Vital signs.
Visit Window (days)		**±2[f]**	**±2**	**±2**		**+7**	**±14**	c. **EOT** assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 12 weeks (tumor assessment), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere. d. **Safety FU** is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1). e. **Survival FU** is every 3 months from EOT (see Section 7.1.1.3). f. Minus 2-day time window for Cycle 1 Day 1.
Pregnancy Test (as applicable)	X	X		X	X	X		• IOCBP participants: a serum pregnancy test is mandatory at screening. • Following screening, additional pregnancy tests per SoA may be serum or urine tests with a sensitivity of at least 25 mIU/mL. • Additional pregnancy tests may be performed at any time per investigator's judgement or if required by local regulations or IRBs/ECs. • See Section 8.3.5.
Disease Assessments								
Tumor Assessments (other than Bone scan)	X	◄--► Every 12 weeks (±7 days) from the date of randomization			X			

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Table 18. Schedule of Activities: Phase 3

Protocol Activity	Screening[a]	Active Treatment Phase[b] (1 Cycle = 28 days)			EOT / Withdrawal[c]	Post-Treatment FU		Notes
		Cycles 1 & 2		Cycles ≥3		Safety FU[d]	Survival FU[e]	a. **Screening:** screening procedures should be completed within 28 days from randomization unless otherwise specified.
Study Day	**≤28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**Every 3 mos. from EOT**	b. **Active Treatment Phase** assessments should be performed, and safety tests reviewed, prior to dosing on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) PROs; 2) ECGs; 3) blood drawn for PK; 4) blood drawn for lab tests/ctDNA/Retained Research Sample; 5) PE/Vital signs.
Visit Window (days)		**±2[f]**	**±2**	**±2**		**+7**	**±14**	c. **EOT** assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 12 weeks (tumor assessment), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere. d. **Safety FU** is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1). e. **Survival FU** is every 3 months from EOT (see Section 7.1.1.3). f. Minus 2-day time window for Cycle 1 Day 1.
Bone Scan, Whole Body	X	◄--► Every 24 weeks (±7 days) from the date of randomization			X			• Images and other information as per Section 8.2.3, should be sent to BICR on an ongoing basis (as soon as they are available) up to PD by BICR, irrespectively of initiation of a new anticancer therapy. • Tumor assessments by imaging should continue until one of the reasons outlined in Section 8.2.1 has occurred. • On Active Treatment Phase, bone scan should be performed only if needed as per Section 8.2.1 requirements. • See Section 8.2.2 for further details on treatment continuation beyond disease progression. • Post-Treatment tumor assessments (including bone scan, if applicable) are required only for participants who discontinued the study treatment for reason other than PD. • See Sections 8.2.1 and 8.2.2.
Randomization and Study Treatment								
Randomization	X							• Randomization must occur after having reviewed all eligibility criteria and confirmed they are satisfied. • De-randomization is NOT allowed as outlined in Section 6.3. • At randomization, randomization number and treatment arm are assigned.

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Table 18. Schedule of Activities: Phase 3

Protocol Activity	Screening[a]	Active Treatment Phase[b] (1 Cycle = 28 days)			EOT / Withdrawal[c]	Post-Treatment FU		Notes
		Cycles 1 & 2		Cycles ≥3		Safety FU[d]	Survival FU[e]	a. **Screening:** screening procedures should be completed within 28 days from randomization unless otherwise specified.
Study Day	**≤28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**Every 3 mos. from EOT**	b. **Active Treatment Phase** assessments should be performed, and safety tests reviewed, prior to dosing on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) PROs; 2) ECGs; 3) blood drawn for PK; 4) blood drawn for lab tests/ctDNA/Retained Research Sample; 5) PE/Vital signs.
Visit Window (days)		**±2[f]**	**±2**	**±2**		**+7**	**±14**	c. **EOT** assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 12 weeks (tumor assessment), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere. d. **Safety FU** is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1). e. **Survival FU** is every 3 months from EOT (see Section 7.1.1.3). f. Minus 2-day time window for Cycle 1 Day 1.
								• Study treatment must start ≤3 days from randomization. • See Section 6.3.
vepdegestrant		◄--► Once Daily						• vepdegestrant is given orally with food in continuous daily dosing for a 28-day cycle. • See Section 6.1.1 and Section 6.1.1.1 for details on drug administration.
Letrozole		◄--► Once Daily						• Letrozole are given orally with food in continuous daily dosing for a 28-day cycle. • See Section 6.1.1 and Section 6.1.1.3 for details on drug administration.
Palbociclib (both arms)		◄--► Once Daily on Days 1-21 followed by 7 days off						• Palbociclib is given orally with food in daily dosing from Day 1 to Day 21 (21 days) followed by 7 days off-treatment, for a 28-day cycle. Palbociclib drug intake should occur at the same time of vepdegestrant and letrozole. • If the Day 1 visit of the subsequent cycle is scheduled during the Day -2 time window, participant must be instructed to complete their 7-day off-treatment period of the current cycle prior to resuming the drugs given for the subsequent cycle.

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Table 18. Schedule of Activities: Phase 3

Protocol Activity	Screening[a]	Active Treatment Phase[b] (1 Cycle = 28 days)			EOT / Withdrawal[c]	Post-Treatment FU		Notes
		Cycles 1 & 2		Cycles ≥3		Safety FU[d]	Survival FU[e]	a. **Screening:** screening procedures should be completed within 28 days from randomization unless otherwise specified.
Study Day	**≤28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**Every 3 mos. from EOT**	b. **Active Treatment Phase** assessments should be performed, and safety tests reviewed, prior to dosing on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) PROs; 2) ECGs; 3) blood drawn for PK; 4) blood drawn for lab tests/ctDNA/Retained Research Sample; 5) PE/Vital signs.
Visit Window (days)		**±2[f]**	**±2**	**±2**		**+7**	**±14**	c. **EOT** assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 12 weeks (tumor assessment), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere. d. **Safety FU** is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1). e. **Survival FU** is every 3 months from EOT (see Section 7.1.1.3). f. Minus 2-day time window for Cycle 1 Day 1.
								• See Section 6.1.1 and Section 6.1.1.2 for details on drug administration.
Other Clinical Assessments								
Drug Compliance		◄--►						• A dosing diary will be implemented for collection of administration details. Any unused tablets/capsules must be returned to the clinic for drug accountability. • From Cycle 2 onward, compliance will be performed on Day 1 of each cycle, prior to dispensing study interventions for the following cycle. • See Sections 6.2 and 6.5.
Adverse Event Reporting	X	X	X	X	X	X		• See Section 8.4.1 and Appendix 3 for details on AE/SAE reporting requirements during the reporting period and for SAE reporting requirements after the end of reporting period.
Concomitant Medications/ Treatments	X	X	X	X	X	X		• See Section 6.9.
PRO Assessments:		X		X	X			• All self-assessment questionnaires should be completed by the participants while in the clinic prior to any Day 1 procedure.

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Table 18. Schedule of Activities: Phase 3

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		Cycles 1 & 2		Cycles ≥3		Safety FU[d]	Survival FU[e]	a. **Screening:** screening procedures should be completed within 28 days from randomization unless otherwise specified.
Study Day	**≤28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**Every 3 mos. from EOT**	b. **Active Treatment Phase** assessments should be performed, and safety tests reviewed, prior to dosing on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) PROs; 2) ECGs; 3) blood drawn for PK; 4) blood drawn for lab tests/ctDNA/Retained Research Sample; 5) PE/Vital signs.
Visit Window (days)		**±2[f]**	**±2**	**±2**		**+7**	**±14**	c. **EOT** assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 12 weeks (tumor assessment), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere. d. **Safety FU** is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1). e. **Survival FU** is every 3 months from EOT (see Section 7.1.1.3). f. Minus 2-day time window for Cycle 1 Day 1.
• EuroQol EQ-5D-5L • EORTC QLQ-C30 • EORTC QLQ-BR23				(*Cycle 3, then every odd cycle, ie, Cycles 5, 7, etc)*				• See Section 8.9 and Appendix 13 Section 10.13.1. • For EORTC QLQ-BR23, female and male questionnaire versions will be used depending on participant gender.
Survival Follow-up							X	• Survival status will also include collection of FU anticancer therapies. • Survival may be collected also by telephone contact, email, etc. • See Section 7.1.1.3.
Pharmacokinetic assessments								
Blood sample for vepdegestrant PK (Arm A only)		X *(Cycle 2 only)*	X	X *(Cycles 3 & 5 only)*				• A single 3 mL whole blood sample will be collected at predose (within 2 hours prior to dosing) and, as applicable, at any time between 5 to 7 hours from drug intake, at each of the following timepoints: o Cycle 1 Day 15: predose & 5-7 hr postdose o Cycle 2 Day 1: predose o Cycle 2 Day 15: predose & 5-7 hr postdose

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Table 18. Schedule of Activities: Phase 3

Protocol Activity	Screening[a]	Active Treatment Phase[b] (1 Cycle = 28 days)			EOT / Withdrawal[c]	Post-Treatment FU		Notes
		Cycles 1 & 2		Cycles ≥3		Safety FU[d]	Survival FU[e]	a. **Screening:** screening procedures should be completed within 28 days from randomization unless otherwise specified.
Study Day	**≤28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**Every 3 mos. from EOT**	b. **Active Treatment Phase** assessments should be performed, and safety tests reviewed, prior to dosing on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) PROs; 2) ECGs; 3) blood drawn for PK; 4) blood drawn for lab tests/ctDNA/Retained Research Sample; 5) PE/Vital signs.
Visit Window (days)		**±2[f]**	**±2**	**±2**		**+7**	**±14**	c. **EOT** assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 12 weeks (tumor assessment), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere. d. **Safety FU** is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1). e. **Survival FU** is every 3 months from EOT (see Section 7.1.1.3). f. Minus 2-day time window for Cycle 1 Day 1.
								○ Cycle 3 Day 1: predose ○ Cycle 5 Day 1: predose • See Section 8.5.
Blood sample for palbociclib PK (Both arms)		X *(Cycle 2 only)*	X					• A single 3 mL whole blood sample will be collected at predose (within 2 hours prior to dosing) and, as applicable, at any time between 5 to 7 hours from drug intake, at each of the following timepoints: ○ Cycle 1 Day 15: predose & 5-7 hr postdose ○ Cycle 2 Day 1: predose ○ Cycle 2 Day 15: predose & 5-7 hr postdose • See Section 8.5.
Special Laboratory Studies								
ctDNA Blood Sample		X	X *(Cycle 1 only)*	X *(Cycle 6 only)*	X			• Two 10 mL whole blood samples (one 10 mL sample for China) at each timepoint will be collected. • See Section 8.7.1 and Laboratory Manual.
Retained Research Blood Sample for Genetics		X *(Cycle 1 only)*						• A single 4 mL whole blood sample for DNA extraction (Prep D1) will be collected, unless prohibited by local regulations, IRBs/ECs.

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Table 18. Schedule of Activities: Phase 3

Protocol Activity	Screening[a]	Active Treatment Phase[b] (1 Cycle = 28 days)			EOT / Withdrawal[c]	Post-Treatment FU		Notes
		Cycles 1 & 2		Cycles ≥3		Safety FU[d]	Survival FU[e]	a. **Screening:** screening procedures should be completed within 28 days from randomization unless otherwise specified.
Study Day	**≤28 days**	**Day 1**	**Day 15**	**Day 1**		**28 days from last dose**	**Every 3 mos. from EOT**	b. **Active Treatment Phase** assessments should be performed, and safety tests reviewed, prior to dosing on the visit days, unless otherwise specified, using the following sequence (if performed on the same day): 1) PROs; 2) ECGs; 3) blood drawn for PK; 4) blood drawn for lab tests/ctDNA/Retained Research Sample; 5) PE/Vital signs.
Visit Window (days)		**±2[f]**	**±2**	**±2**		**+7**	**±14**	c. **EOT** assessments should be performed only if they have NOT been completed 1) during the previous 4 weeks (safety assessments), unless needed for monitoring of safety findings; 2) during the previous 12 weeks (tumor assessment), 24 weeks for bone scan if bone scan is the preferred technique to follow bone lesion/s on treatment, unless PD has been confirmed elsewhere. d. **Safety FU** is a minimum of 28 days from last dose OR prior to initiation of a new anticancer therapy, whichever comes first (See Section 7.1.1.1). e. **Survival FU** is every 3 months from EOT (see Section 7.1.1.3). f. Minus 2-day time window for Cycle 1 Day 1.
								• If not collected on the designated collection day, collect at the next available time point when biospecimens are being collected in conjunction with a participant visit. • See Section 8.6.2.
Tumor Tissue for Biomarkers	X *(mandatory)*				X *(optional)*			• See Section 8.7.1.1.

Amendment 1 (07 March 2023)

Overall Rationale for the Amendment: The protocol was amended to incorporate a Study Lead-in based on regulatory agency feedback and additional data following an vepdegestrant (ARV-471) Investigator's Brochure update (v. 4.0).

Section # and Name	Description of Change	Brief Rationale	Substantial or Non-substantial
Section 1 Protocol Summary **Section 2.2.3.3** vepdegestrant Preliminary Safety, Efficacy and Pharmacokinetic Data in Humans. **Section 2.3** Benefit/Risk Assessment **Section 3** Objective/Endpoints and Estimands **Section 4.1** Overall Design **Section 4.3** Justification for dose **Section 4.4** End of Study Definition **Section 6.1 to Section 6.8** Study Intervention(s) Administration to Treatment of Overdose **Section 7** Discontinuation of Study Intervention and Participant Discontinuation/Withdrawal **Section 8.2** Efficacy Assessments **Section 9** Statistical Considerations	Updated text, adding an open-label Study Lead-in before initiation of Phase 3. Study Lead-in Schedule of Activities were added. Study Lead-in will test vepdegestrant with lower palbociclib doses (100 and 75 mg) compared to the registered dose of palbociclib (125 mg). The Phase 3 study design was changed from blinded to open-label. In Phase 3, primary endpoint of PFS by investigator was changed to PFS by BICR. Efficacy PFS IA (IA2), was removed and the number of events for futility PFS IA was increased to 50%.	Safety and PK from Part C of ARV-471-mBC-101 indicated a need for optimization of the palbociclib RP3D in combination with vepdegestrant. Lower doses of palbociclib (100 and 75 mg) in combination with vepdegestrant will be tested in the Study Lead-in. As agreed with FDA, a Study Lead-in was added; the Phase 3 study design and the primary endpoint were updated and the PFS IA2 was removed. Phase 3 study design was changed because of the different palbociclib doses to be used in the experimental and control arm. To keep the study blinded, it is	Substantial

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Section # and Name	Description of Change	Brief Rationale	Substantial or Non-substantial
	Language in the sections affected by these changes were updated accordingly.	needed to have either a double-dummy design (high pill burden and increased risk of poor compliance/nausea) or an attempt to over-encapsulate the palbociclib which would make the capsules prohibitively large for an average patient to swallow.	
Section 1.1 Synopsis Section 3 Objective/Endpoints/Estimands	Vital signs removed from safety endpoints	They were not considered as critical parameters for the definition of the safety of the combination.	Substantial
Section 5.2 Exclusion Criteria Section 10.2 Appendix 2, Clinical Laboratory Assessments	Updated text of EC #11 on renal impairment removing serum creatinine as parameter for defining exclusion of participant with inadequate renal function. eGFR cutoff set to less than 45 mL/min/m^2 based on ARV-471-CPhm-103 study indicating renal clearance of vepdegestrant is less than 0.1% of total clearance.	Health Authorities require the use of eGFR equation because is more inclusive and the most reliable overall index of kidney function.	Substantial

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Section # and Name	Description of Change	Brief Rationale	Substantial or Non-substantial
Section 2.2.3 vepdegestrant	Added details on anti-tumor activity of vepdegestrant alone and in combination with CDK4/6i	To align with updated vepdegestrant IB.	Substantial
Section 2.2.3.1 Nonclinical Pharmacokinetics and Metabolism	Added details on CYP metabolism and doses administered in dogs to measure exposure of vepdegestrant.	To align with updated vepdegestrant IB.	Substantial
Section 2.2.3.2, Potential Drug-Drug Interaction Section 10.9.3, Sensitive Substrates of Transporters Section 10.9.5 PPIs	Updated text about impact of vepdegestrant on CYP2B6 substrates and substrates of human transporters (P-gp and BCRP) effect of PPI on vepdegestrant.	To align with updated vepdegestrant IB.	Substantial
Section 1.1 Synopsis Section 2.2.3.3, vepdegestrant Preliminary Safety, Efficacy and Pharmacokinetics Data in Humans Section 2.3.1 Risk Assessment Section 4.3 Justification for dose Section 10.9.2, Drugs Known to Predispose to Torsade de Pointes or QT interval prolongation	Updated safety, efficacy and PK data based on ARV-471-mBC-101 (01-Nov-2022 data cut) and ARV-471-CPhm-103 studies. Update included concentration-QTc modeling analysis of data from the vepdegestrant-mBC-101 study.	To align with updated vepdegestrant IB and ARV-471-mBC-101 (01-Nov-2022 data cut).	Substantial
Section 2.3.1, Risk Assessment Section 2.3.2, Benefit Assessment Section 2.3.3, Overall Benefit/Risk conclusion	Updated text about QTc prolongation and venous embolism, safety and efficacy from ARV-471-mBC-101 study (01-Nov-2022 data cut).	To align with updated vepdegestrant IB and ARV-471-mBC-101 (01-Nov-2022 data-cut)	Substantial

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Section # and Name	Description of Change	Brief Rationale	Substantial or Non-substantial
Section 1.1 Synopsis Section 1.3 Schedule of Activities (Table 1 and Table 3) - LHRH agonist use Section 4.1 Overall Design 6.9.1 Luteinizing Hormone-Releasing Hormone Agonist	Updated text on LHRH agonist use moving the initiation from at least 28 days before randomization to Cycle 1 Day 1 and allowing participants already on treatment with the 3-month formulation to continue it.	To align with NCCN Breast Cancer guidelines.	Substantial
Section 1.1 Synopsis -Inclusion criteria Section 1.3 Schedule of Activities (Table 1 and Table 3) - Contraception Section 4.2.3, Choice of Contraception/ Barrier Requirements Section 5.1 Inclusion criteria. Section 8.1.2 Telehealth Visits Section 8.1.3 Home Heath Visits Section 10.4.2 Female Participant Reproductive Inclusion Criteria	Updated text to specify that contraception and contraception checks are required for IOCBP up to 30 days after the last dose of study intervention. IC #1 language was updated accordingly. Duration of contraception updated as per changes in palbociclib Core Data Sheet.	To align with LHRH agonists prescribing information (eg, USPI Zoladex), stating that an LHRH agonist must be associated with the use of nonhormonal contraception.	Substantial
Section 1.3 Schedule of Activities - Pregnancy test Section 8.3.5, Pregnancy Testing Section 10.2 Appendix 2, Clinical Laboratory Assessments	Updated text on contraception requirements and frequency of pregnancy tests requested for IOCBP.	Due to the lack of effectiveness of LHRH agonist to prevent pregnancy in WOCBP.	Substantial
Section 6.9.4 Concomitant Treatments to be used with Caution or Not Recommended	Updated details with reference to the use of H2 receptor	To align with updated vepdegestrant IB.	Substantial

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Section # and Name	Description of Change	Brief Rationale	Substantial or Non-substantial
	antagonists and antacids.		
Section 1.3 SoA – Physical Examination note Section 8.3.1 Physical Examinations	Added weight collection on Day 1 of each cycle and at EOT.	To monitor the impact of endocrine therapy on body weight.	Substantial
Section 1.1, Synopsis	Removed exploratory objective/endpoint from the synopsis.	To align with sponsor protocol template.	Non-substantial
Section 1.3, Schedule of Activities - General notes	Added clarifications on: 1) safety assessments should be reviewed prior to dosing, on visit days; 2) time window for Cycle 1 Day 1 is -2 days; 3) EOT assessment time window, including time window for bone scan.	To add clarification on safety assessments review.	Non-substantial
Section 1.3 Schedule of Activities – Drug Compliance Note Section 6.5 Study Intervention Compliance	Clarified that compliance, not drug accountability will be assessed prior to the dispensing of study interventions for the next cycle.	To align text of this section to sponsor protocol template.	Non-substantial
Section 1.3 Schedule of Activities (Table 2) – vepdegestrant and palbociclib note	Updated timing of PK sample collection.	The timing for post-dose sample collection has been restricted to 5-7 h to align with expected $T_{max.}$	Non-Substantial
Section 1.3 Schedule of Activities (Table 3) – PROs note Section 4.1 Overall Design Section 8.9 Health Economics	Clarified that EORTC-QLQ-BR23 questionnaire has female and male versions.	To clarify that QLQ-BR23 questionnaire has gender specific versions, and that	Non-substantial

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Section # and Name	Description of Change	Brief Rationale	Substantial or Non-substantial
Section 9.3.3.4 Secondary Efficacy Endpoints Section 10.13.1 Patient-Reported Outcomes Assessments		data will be analyzed separately by gender.	
Section 1.1 Synopsis Section 4.1 Overall Design Section 10.1.5.2 Global Steering Committee	Added description of the program level Global Steering Committee.	To clarify a Global Steering Committee was set up.	Non-substantial
Section 8.6.2 Retained Research Samples for Genetics Section 8.7 Biomarkers	For China: Clarified that Retained Research samples for genetics will not be collected for participants and that samples collection for biomarker analysis and analysis will be performed upon approval by HGRAC.	Clarification added for consistency with the statement reported on the HGRAC application form.	Non-substantial
Throughout the protocol	Typographical errors were corrected, and minor edits were made. Synopsis was streamlined and SLI and Phase 3 safety endpoints were split into separate bullets.	For clarification and to ensure consistency	Non-Substantial

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10.15. Appendix 15: Abbreviations

The following is a list of abbreviations that may be used in the protocol.

Abbreviation	Term
1L	first-line
2L	second-line
aBC	advanced breast cancer
AE	adverse event
AESI	adverse event of special interest
AI	aromatase inhibitor
AIDS	acquired immunodeficiency syndrome
AJCC	American Joint Committee on Cancer
AKI	acute kidney injury
ALP	alkaline phosphatase
ALT	alanine aminotransferase
ANC	absolute neutrophil count
ANOVA	analysis of variance
aPTT	activated partial thromboplastin time
ASCO	American Society of Clinical Oncology
ASCO/CAP	American Society of Clinical Oncology / College of American Pathologists
AST	aspartate aminotransferase
AUC	area under the concentration time curve
AUC_{0-24}	area under the concentration time curve from time zero to 24 hours
AUC_{inf}	area under the concentration time curve from time zero to infinity
AUC_{tau}	area under the concentration-time curve during the dosing interval (τ)
AV	atrioventricular
AxMP	auxiliary medicinal product
BC	breast cancer
BCRP	breast cancer resistance protein
BCS	biopharmaceutics classification system
BICR	blinded independent central review
BID	twice daily
BOR	best overall response
BP	blood pressure
bpm	beats per minute
BUN	blood urea nitrogen
C	cycle
C1D1	Cycle 1 Day 1
C1D15	Cycle 1 Day 15
Ca	calcium

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Abbreviation	Term
CAP	chest, abdomen and pelvis
CBR	clinical benefit response
CDK4/6	cyclin-dependent kinase 4/6
CDK4/6i	cyclin-dependent kinase 4/6 inhibitor
CERAN	complete estrogen receptor antagonist
CFR	Code of Federal Regulations
CHW	Cui–Hung–Wang
CI	confidence interval
CIOMS	Council for International Organizations of Medical Sciences
CK	creatine kinase
CKD	chronic kidney disease
CKD-EPI	chronic kidney disease epidemiology collaboration
C_{max}	maximum observed concentration,
CMH	Cochran–Mantel–Haenszel
CNS	central nervous system
CONSORT	Consolidated Standards of Reporting Trials
COVID-19	coronavirus disease 2019
CR	complete response
CRF	case report form
CRO	contract research organization
CSR	Clinical Study Report
CT	computed tomography/clinical trial/chemotherapy
CTCAE	Common Terminology Criteria for Adverse Events
ctDNA	circulating tumor DNA
CTIS	Clinical Trials Information System
CYP	cytochrome P450
D	Day
DCT	data collection tools
DDI	drug-drug interaction
DIDB	Drug Interaction Database
DILI	drug-induced liver injury
dL	deciliter
DLT	dose-limiting toxicity
DNA	deoxyribonucleic acid
DoR	duration of response
DU	dispensable unit
EC	Ethics Committee/exclusion criterion
ECG	electrocardiogram
ECOG PS	Eastern Cooperative Oncology Group Performance Status
eCrCl	estimated creatinine clearance
eCRF	electronic case report form

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Abbreviation	Term
EDB	exposure during breastfeeding
E-DMC	external data monitoring committee
EDP	exposure during pregnancy
eGFR	estimated glomerular filtration rate
EORTC	European Organisation for Research and Treatment of Cancer
EOT	end of treatment
EQ-5D-5L	European Quality of Life 5 Domain 5 Level Scale
ER	estrogen receptor
ER alpha	estrogen receptor subtype alpha
eSAE	electronic data collection tool (DCT) for serious adverse event data
ESR1	Estrogen Receptor 1 gene
ET	endocrine therapy
EU	European Union
EudraCT	European Union Drug Regulating Authorities Clinical Trials (European Clinical Trials Database)
EuroQol	EuroQol Group comprises an international network of multidisciplinary researchers
FA	Final Analysis
FAS	Full Analysis Set
FDA	Food and Drug Administration (United States)
FFPE	formalin-fixed paraffin-embedded
FIH	first-in-human
FSH	follicle-stimulating hormone
FU	follow-up
FWER	familywise error rate
g	gram
GCP	Good Clinical Practice
GCSF	Granulocyte colony-stimulating factor
GGT	gamma-glutamyl transferase
GI	gastrointestinal
GM-CSF	granulocyte macrophage colony-stimulating factor
h	hours
H1	histamine type 1 receptor
H2	histamine type 2 receptor
HBV	hepatitis B virus
HCV	hepatitis C virus
HDL	high-density lipoprotein
HER2	human epidermal growth factor receptor 2
HGRAC	Human Genetics Resources Administration of China
HIV	human immunodeficiency virus
HR	hazard ratio/heart rate/hormone receptor

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Abbreviation	Term
IA	interim analysis
IB	Investigator's Brochure
IC	inclusion criterion
IC50	inhibitory concentration 50
ICD	informed consent document
ICH	International Council for Harmonisation of Technical Requirements for Pharmaceuticals for Human Use
ID	identification
IF	information fraction
IHC	immunohistochemistry
ILD	interstitial lung disease
IMP	Investigational Medicinal Product
IND	Investigational New Drug
INR	international normalized ratio
IoR	Importer of Record
IOCBP	individual of childbearing potential
IOTA	Investigator Overall Objective Tumor Assessment
IP	Investigational Product
IPAL	Investigational Product Accountability Log
IPCW	Inverse-Probability-of-Censoring-Weighting
IPM	Investigational Product Manual
IRB	Institutional Review Board
IRT	Interactive Response Technology
IUD	intrauterine device
IV	intravenous
IWR	Interactive Web Response
K2 EDTA	dipotassium ethylenediaminetetraacetic acid
K	potassium
KDIGO	Kidney Disease: Improving Global Outcomes
Ki67	antigen Ki67
L	liter
LBBB	left bundle branch block
LDL	low-density lipoprotein
LFT	liver function test
LHRH	luteinizing hormone-releasing hormone
LLN	lower limit of normal
mBC	metastatic breast cancer
MCF7	an epithelial cell line isolated from the breast tissue of a patient with metastatic adenocarcinoma and used in breast cancer research
MedDRA	Medical Dictionary for Regulatory Activities
mESR1	ESR1 gene mutation
mg	milligram

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Abbreviation	Term
Mg	magnesium
MID	minimally important difference
min	minute
mIU	milli-international unit
ml	milliliter
mm	millimeter
MMRM	mixed model repeated measures
mos	months
mOS	median overall survival
mPFS	median progression free survival
MQI	medically qualified individual
MRI	magnetic resonance imaging
mTOR	mammalian target of rapamycin
mTORi	mammalian target of rapamycin inhibitors
n or N	number
Na	sodium
N/A	not applicable/not available
NCCN	National Comprehensive Cancer Network®
NCI	National Cancer Institute
NE	not evaluable
NIMP	Non-Investigational Medicinal Product
NR	not reached
NTI	Narrow Therapeutic Window
OR	objective response
ORR	objective response rate
os	orally
OS	overall survival
PACL	Protocol Administrative Change Letter
PARP	poly-ADP ribose polymerase
PARPi	poly-ADP ribose polymerase inhibitors
PD	pharmacodynamic(s)/progressive disease
PE	physical examination
PFS	progression-free survival
P-gp	P-glycoprotein
PI	Principal Investigator
PI3Ki	phosphoinositide 3-kinase inhibitor
PIK3CA	phosphatidylinositol-4,5-bisphosphate 3-kinase catalytic subunit alpha
PK	pharmacokinetic(s)
PLT	platelet
PPI	proton pump inhibitor
PR	partial response/pulse rate/progesterone receptor
PRO	patient reported outcome

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Abbreviation	Term
PROTAC	proteolysis targeting chimeric
PSSA	Pfizer's Serious Adverse Event Submission Assistant
PT	prothrombin time
PVC	premature ventricular contraction
QD	once daily
QLQ-BR23	EORTC Quality of Life Questionnaire Breast 23
QLQ-C30	EORTC Quality of Life Cancer Questionnaire 30
QoL	quality of life
QRS	combination of the Q wave, R wave and S wave
QT	time interval from the start of the Q wave to the end of the T wave, ie, time taken for ventricular depolarization and repolarization
QTc	corrected QT interval
QTcF	QTc calculated using Fridericia method
QTL	quality tolerance Limit
RANKL	receptor activator of nuclear factor kappa beta
RBC	red blood cell
RCI	repeated CI
RECIST	Response Evaluation Criteria in Solid Tumors
RMST	restricted mean survival time
RNA	ribonucleic acid
RP2D	recommended phase 2 dose
RP3D	recommended phase 3 dose
RPSFT	rank-preserving-structural-failure-time model
RT	radiotherapy
SABCS	San Antonio Breast Cancer Symposium
SAE	serious adverse event
SAP	Statistical Analysis Plan
SARS-CoV-2	severe acute respiratory syndrome coronavirus 2
SAS	Safety Analysis Set
Screat	serum creatinine
Scys	serum cystatin C
SD	stable disease
SERCA	selective estrogen receptor covalent antagonists
SERD	selective estrogen receptor degraders
SERM	selective estrogen receptor modulator
SLI	Study Lead-in
SmPC	Summary of Product Characteristics
SoA	Schedule of Activities
SoC	standard of care
SOP	Standard Operating Procedure
SRSD	single reference safety document
ST-T	ST segment or T-wave abnormalities or both

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Abbreviation	Term
SULT	sulfotransferase
SUSAR	Suspected Unexpected Serious Adverse Reaction
$t_{1/2}$	terminal elimination half-life
T bili	total bilirubin
TdP	Torsades de Pointes
TEAE	Treatment emergent adverse event
TGI	tumor growth inhibition
TK	toxicokinetic
TK1	Thymidine Kinase 1
TNM	tumor, nodes, metastases
TOC	Table of Content
TRAE	treatment-related AE
TTD	time to deterioration
UK	United Kingdom
ULN	upper limit of normal
US	United States
USPI	United States Prescribing Information
UW	University of Washington
VAS	Visual Analog Scale
VE	venous embolism
WBC	white blood cell
wt.	weight
wt	wild type

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